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FUSED AMINO PYRIMIDINE COMPOUNDS FOR TREATMENT OF SYNUCLEINOPATHIES AND TDP-43 PROTEINOPATHIES

Abstract

Compositions and methods for treating one or more synucleinopathies or one or more TDP-43 proteinopathies or dementia with a compound of Formula (I), or pharmaceutically acceptable salts thereof, are provided. The compositions and methods may be used to improve one or more symptoms of one or more synucleinopathies or one or more TDP-43 proteinopathies. Formula (I):
##STR00001##

or pharmaceutically acceptable salts thereof, wherein R.sup.1, R.sup.2, R.sup.7 and ring A have any of the meanings herein defined in the description.

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Background/Summary

CROSS REFERENCE TO RELATED APPLICATION [0001] This application claims benefit of and priority to the U.S. Provisional Application No. 63/561,914, filed on Mar. 6, 2024, which is incorporated herein by reference in its entirety.

TECHNICAL FIELD

[0002] Methods and compositions for treating synucleinopathies and TDP-43 proteinopathies with fused amino pyrimidine compounds are provided.

BACKGROUND

[0003] The term synucleinopathies describes a group of neurodegenerative disorders characterized by fibrillary aggregates of alpha-synuclein protein in the cytoplasm of selective populations of neurons and glia. See, e.g., Martí M J, Tolosa E, Campdelacreu, Clinical overview of the synucleinopathies. *Mov Disord.* 2003 September; 18 Suppl 6:S21-7. These disorders include Parkinson's disease (PD), dementia with Lewy bodies (DLB) (otherwise known as Lewy body dementia (LBD)), pure autonomic failure (PAF), and multiple system atrophy (MSA). *Id.* Clinically, they are characterized by a chronic and progressive decline in motor, cognitive, behavioral, and autonomic functions, depending on the distribution of the lesions. *Id.*

[0004] According to the National Institutes of Health, Parkinson's disease is a chronic and progressive neurological disease that is marked by tremors in the resting muscles, rigidity, slowness of movement, impaired balance and a shuffling gait. In addition, many patients with PD develop non-motor symptoms such as behavioral changes and cognitive impairment. Because the worldwide population is living longer, PD represents a significant and increasing threat to public health. In the United States alone it has been estimated that up to 1 million people may have PD. Though it is a complex neurodegenerative disorder, the two primary hallmarks of PD are the progressive loss of dopamine-containing neurons in a specific area of the brain called the substantia nigra and the buildup within neurons of a protein, α -synuclein, into clusters called Lewy bodies. Dementia is a frequent complication of PD with a yearly incidence of around 10% of patients with PD. Lewy body pathology may be the most important factor in the development of Parkinson disease dementia (PDD). Treatment for Parkinson's disease is symptomatic. There is currently no cure.

[0005] According to the National Institutes of Health, National Institute on Aging, Lewy body dementia (LBD) is a disease associated with abnormal deposits of alpha-synuclein in the brain. These deposits, or Lewy bodies, affect chemicals in the brain whose changes, in turn, can lead to problems with thinking, movement, behavior, and mood. Lewy body dementia is one of the most common causes of dementia. LBD affects more than 1 million individuals in the United States. People typically show symptoms at age 50 or older, although sometimes younger people have LBD. LBD appears to affect slightly more men than women. LBD is a progressive disease, i.e., symptoms start slowly and worsen over time. The disease lasts an average of five to eight years from the time of diagnosis to death, but can range from two to 20 years for some people. In the early stages of LBD, symptoms can be mild, and people can function fairly normally. As the disease advances, patients with LBD require more assistance due to a decline in thinking and movement abilities. In the later stages of the disease, they often depend entirely on others for assistance and care. Some LBD symptoms may respond to treatment for a period of time. Currently, there is no cure for the disease.

[0006] According to the National Institutes of Health, Genetic and Rare diseases Information Center, pure autonomic failure (PAF) is a neurodegenerative disease of the autonomic nervous system, which regulates body processes like blood pressure and breathing rate. PAF has been known as idiopathic orthostatic hypotension or Bradbury-Eggleston syndrome. PAF usually affects

only the peripheral autonomic nervous system, i.e., it does not usually involve the brain and spinal cord (the central nervous system). Symptoms begin in midlife, although they can begin earlier. The main symptom of PAF is orthostatic hypotension. This can cause dizziness, lightheadedness, blurry vision, and weakness. Other symptoms can include fatigue, bladder problems, constipation, abnormal sweating, and sleep disorders. The symptoms of PAF tend to get worse over time, and sometimes, PAF can lead to other conditions such as Parkinson's disease, dementia with Lewy bodies, or multiple system atrophy. The cause of PAF is not known. The symptoms are caused by Lewy bodies in the cells of autonomic nerves. The Lewy bodies restrict the production and release of norepinephrine from nerve cells, which in turn causes hypotension. Treatment for PAF is symptomatic. There is currently no cure.

[0007] According to the National Institutes of Health, Genetic and Rare Diseases Information Center (<https://rarediseases.info.nih.gov/diseases/7079/multiple-system-atrophy>), multiple system atrophy (MSA) causes the progressive loss of nerve cells in the brain. MSA affects several areas of the brain, including the cerebellum, which is involved in controlling movement and some emotions, as well as certain types of learning and memory, and the autonomic nervous system, which controls automatic, or regulating functions, such as blood pressure, digestion and temperature. The initial symptoms of MSA usually start around age 50 and are very similar to the initial symptoms of Parkinson's disease. These symptoms may include slowness of movement, tremor, or rigidity (stiffness), clumsiness or coordination problems, difficulties with speech, orthostatic hypotension, and bladder control problems. Other symptoms of MSA may include muscle contractures, abnormal posture, bending of the neck, involuntary sighing, trouble sleeping and emotional problems. As MSA progresses, breathing problems while sleeping (sleep apnea) and irregular heart rhythms may develop. The parkinsonian type (MSA-P) has Parkinson disease-like symptoms, such as moving slowly, stiffness, and tremor, along with problems of balance, coordination, and autonomic nervous system dysfunction; the cerebellar type (MSA-C) has primary symptoms of cerebellar ataxia such as problems with balance and coordination, difficulty swallowing and speaking, and abnormal eye movements. The cause of MSA is unknown. There is no cure for MSA. Currently, there are no treatments to delay the progress of neurodegeneration associated with MSA in the brain. But there are treatments available to help people cope with some of the more disabling symptoms of MSA. In some individuals, levodopa may improve motor function, but the benefit may not continue as the disease progresses.

[0008] TDP-43 proteinopathies involve inclusions of pathogenic deposits containing TAR DNA-binding protein 43 (TDP-43) that are evident in the brain and spinal cord of patients in certain neurodegenerative diseases. For instance, the majority of patients with sporadic amyotrophic lateral sclerosis (ALS) (up to 97%) and a substantial proportion of patients with frontotemporal lobar degeneration, also known as frontotemporal dementia (FTD), (~45%) exhibit TDP-43 positive neuronal inclusions, suggesting a role for this protein in disease pathogenesis. See, de Boer E M J, Orie V K, Williams T, et al., TDP-43 proteinopathies: a new wave of neurodegenerative diseases *Journal of Neurology, Neurosurgery & Psychiatry* 2021; 92:86-95.

[0009] According to the National Institutes of Health, National Institute of Neurological Disorders and Stroke (<https://www.ninds.nih.gov/health-information/disorders/amyotrophic-lateral-sclerosis-als#:>) amyotrophic lateral sclerosis (ALS) is a neurological disorder that affects motor neurons, the nerve cells in the brain and spinal cord that control voluntary muscle movement and breathing. As motor neurons degenerate and die, they stop sending messages to the muscles, which causes the muscles to weaken, start to twitch (fasciculations), and waste away (atrophy). Eventually, in people with ALS, the brain loses its ability to initiate and control voluntary movements such as walking, talking, chewing and other functions, as well as breathing. ALS is progressive, meaning the symptoms get worse over time. ALS is an ultimately fatal disorder that is characterized by rapid and progressive loss of the upper and lower motor neurons (LMNs) at the spinal or bulbar level that control voluntary muscle movement. Voluntary muscles produce movements like walking,

breathing, chewing, and talking. See, Zarei et al., A comprehensive review of amyotrophic lateral sclerosis. *Surg Neurol Int.* 2015 Nov. 16; 6:171. ALS is categorized in two forms. The most common form is sporadic (90-95%) which has no obvious genetically inherited component. The remaining 5-10% of the cases are familial-type ALS (FALS) due to their associated genetic dominant inheritance factor. The first onset of symptoms is usually between the ages of 50 and 65. The most common symptoms that appear in both types of ALS are muscle weakness, twitching, and cramping, which eventually can lead to the impairment of muscles. The motor neurons degenerate, die, and stop communicating with muscles. The muscles gradually weaken, waste away, and twitch, and the brain cannot start and control voluntary movement. Symptoms are usually first noticed in the arms and hands, legs, or swallowing muscles. Patients with ALS lose their strength and become unable to move their arms and legs, and to hold the body upright. In the most advanced stages, ALS patients will develop symptoms of dyspnea and dysphagia. No cure has yet been found for ALS. However, the drugs riluzole and edaravone have been approved by the Food and Drug Administration (FDA) to treat ALS. Riluzole prolongs life by 2-3 months but does not relieve symptoms. Edaravone can slow the clinical decline in daily functioning of people with ALS.

[0010] According to the National Institutes of Health, National Institute of Neurological Disorders and Stroke, frontotemporal dementia (FTD) describes a clinical syndrome associated with shrinking of the frontal and temporal anterior lobes of the brain and was originally known as Pick's disease. Symptoms of FTD fall into two clinical patterns that involve either (1) changes in behavior, or (2) problems with language. The first type features behavior that can be either impulsive (disinhibited) or bored and listless (apathetic) and includes inappropriate social behavior; lack of social tact; lack of empathy; distractibility; loss of insight into the behaviors of oneself and others; an increased interest in sex; changes in food preferences; agitation or, conversely, blunted emotions; neglect of personal hygiene; repetitive or compulsive behavior, and decreased energy and motivation. The second type primarily features symptoms of language disturbance, including difficulty making or understanding speech, often in conjunction with the behavioral type symptoms. Spatial skills and memory remain intact. There is a strong genetic component to the disease; FTD often runs in families. The outcome for patients with FTD is poor. The disease progresses steadily and often rapidly, ranging from less than 2 years in some individuals to more than 10 years in others. Eventually some individuals with FTD will need 24-hour care and monitoring at home or in an institutionalized care setting. There is no cure for FTD. No treatment has been shown to slow the progression of FTD. Treatment is symptomatic. Behavior modification may help control unacceptable or dangerous behaviors. Aggressive, agitated, or dangerous behaviors could require medication. Anti-depressants have been shown to improve some symptoms.

[0011] There is a need for effective treatments to ease the symptoms of the foregoing synucleinopathies and TDP-43 proteinopathies.

[0012] Fused amino pyridine compounds and pharmaceutically acceptable salts thereof are disclosed in WO2021/180952, incorporated herein by reference. As described in WO2021/180952, these compounds and their pharmaceutically acceptable salts selectively modulate KCC2 and are used to treat or prevent KCC2 mediated disease, including neurological disorders. As described in the OMIM database, the LC12A5 gene encodes the neuronal KCC2 channel that is the major extruder of intracellular chloride in mature neurons. In the presence of low intraneuronal chloride, the binding of GABA and glycine to their ionotropic receptors results in chloride influx with subsequent hyperpolarization contributing to neuronal inhibition. Hyperpolarizing GABA.sub.AR currents are critically dependent upon efficient Cl.sup.- extrusion, which is facilitated by KCC2.

[0013] Conditional inactivation of KCC2 in the hippocampus leads to neuronal Cl.sup.- accumulation and depolarizing GABA.sub.AR currents. These deficits in inhibition parallel the development of spontaneous seizures, neuronal apoptosis and reactive astrogliosis (Kelley et al.,

EBioMedicine 32, 62-71 (2018)). GABA.sub.A signaling is a major inhibitory neurotransmitter mechanism in the adult brain and consequently KCC2 has a key role in normal neurodevelopment and various neurological disorders. Decreased activity of KCC2 has been implicated in the pathogenesis of neurological disorders including epilepsy (Galanopoulou et al, Epilepsia 2007; 48:14-18; Huberfield et al, The Journal of Neuroscience (2007) 27, 9866-9873), neuropathic pain (Price et al, Curr Top Med Chem 2005; 5:547-555), Rett's syndrome (Tang et al, 2019, Translational Medicine, 11(503)), autism (Tyzio et al, Science 343, 675-679, Merner et al, Frontiers in cellular neuroscience 9, 2015), mental disorders, spinal cord injury (Boulenguez et al, Nature Medicine 2010, 16, 302-307) and conditions in which there is neuronal hyperexcitability such as ALS (Fuchs et al, Journal of Neuropathology & Experimental Neurology, Volume 69, Issue 10, October 2010, Pages 1057-1070).

[0014] Direct modulation of KCC2 by interaction with small molecules has been reported. Delpire et al (Proc Natl Acad Sci USA. 2009 Mar. 31; 106(13): 5383-5388) describe an assay to identify small molecule inhibitors of KCC2 and Zhang et al (Journal of Biomolecular Screening 15(2): 2010) describe an assay used to identify positive modulators of KCC2.

SUMMARY

[0015] Methods and compositions for treating synucleinopathies or TDP-43 proteinopathies are provided and, in embodiments, include administering to a subject diagnosed with a synucleinopathy or a proteinopathy an effective amount of a compound according to Formula (I):
##STR00002##

or a pharmaceutically acceptable salt thereof, wherein: [0016] R.sup.1 is selected from C.sub.2-6alkyl; C.sub.2-6alkenyl; C.sub.2-6alkynyl; C.sub.2-6alkoxy; C.sub.2-6alkenyloxy; C.sub.2-6alkynyloxy; C.sub.3-7cycloalkyl; —O—C.sub.3-7cycloalkyl; C.sub.6-10aryl; —O—(CH.sub.2).sub.m—C.sub.6-10aryl; 6 membered heteroaryl; and thiophenyl; wherein alkyl, alkenyl, alkynyl, alkoxy, alkenyloxy, alkynyloxy and cycloalkyl are optionally substituted with 1, 2 or 3 substituents selected from —F and —CF.sub.3 and wherein aryl and heteroaryl are optionally substituted with 1 or 2 substituents selected from -halo, —C.sub.1-3alkyl, —C.sub.1-8alkoxy and —C.sub.2-8alkynyloxy wherein —C.sub.1-3alkyl, —C.sub.1-8alkoxy and —C.sub.2-8alkynyloxy are optionally substituted with 1, 2, or 3 substituents selected from —F, —CF.sub.3, —NHC(O)O—C.sub.1-6alkyl or two substituents together with the carbon to which they are attached form diazirinyl; [0017] R.sup.2 is selected from —H; -halo; and —C.sub.1-3alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and —CF.sub.3; [0018] A is selected from ##STR00003## or a N-oxide thereof; [0019] R.sup.3 is selected from —H; —C.sub.1-6alkyl; —C.sub.2-6alkenyl; —C.sub.2-6alkynyl; C.sub.3-7cycloalkyl; and a 5 or 6 membered heterocycloalkyl; wherein the alkyl, alkenyl, alkynyl, cycloalkyl or heterocycloalkyl are optionally substituted by 1, 2 or 3 groups selected from —F, —CF.sub.3, —C.sub.1-3alkyl optionally substituted by 1 or 2 substituents selected from —F, —CF.sub.3, —C(O)NR.sup.8R.sup.9 and —NR.sup.8R.sup.9; [0020] R.sup.4a and R.sup.4b are each independently selected from —H and —C.sub.1-3 alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and CF.sub.3; [0021] R.sup.4c and R.sup.4d are each independently selected from —H and —C.sub.1-3 alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and CF.sub.3, or R.sup.4c and R.sup.4d together with the carbon to which they are attached represent carbonyl; [0022] R.sup.5a, R.sup.5b, R.sup.5c and R.sup.5d are each independently selected from —H and —C.sub.1-3 alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and CF.sub.3; [0023] R.sup.6 is selected from —H; -halo; —NH.sub.2; —CN; —C.sub.1-3alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and CF.sub.3; —C.sub.1-3alkoxy optionally substituted with 1, 2 or 3 substituents selected from —F and —CF.sub.3; —C(O)O—C.sub.1-3alkyl; —C(O)NR.sup.8R.sup.9; —C(O)OH; and —NHC(O)—C.sub.1-3alkyl; [0024] R.sup.7 is selected from NR.sup.10R.sup.11; a 5 to 7 membered monocyclic heterocycloalkyl; and a 5 or 6 membered monocyclic heteroaryl; wherein the heterocycloalkyl and heteroaryl are optionally substituted with

1, 2 or 3 groups selected from —CN; —C.sub.1-6alkyl optionally substituted with 1, 2 or 3 substituents selected from —F, —CF.sub.3 and —OH; —C.sub.1-3alkoxy optionally substituted with 1, 2 or 3 substituents selected from —F and CF.sub.3; —C(O)OH; —C.sub.1-3alkylene-NHC(O)C.sub.1-6alkyl; —C.sub.1-3alkylene-NHC(O)OC.sub.1-6alkyl; C.sub.3-5cycloalkyl; or the heterocycloalkyl is optionally substituted with two substituents on the same ring carbon which together with the carbon atom to which they are attached form a 5 to 7 membered monocyclic heterocycloalkyl; and wherein when R.sup.7 is morpholinyl and R.sup.1 is unsubstituted phenyl, R.sup.2 is not —H; [0025] R.sup.8 and R.sup.9 are each independently selected from —H and —C.sub.1-6alkyl; [0026] R.sup.10 is —C.sub.1-6alkyl; [0027] R.sup.11 is selected from —C.sub.1-6alkyl optionally substituted with 1 or 2 substituents selected from —F and —C.sub.1-3alkoxy; and —(CH.sub.2).sub.nR.sup.12; [0028] R.sup.12 is a 5 or 6 membered heteroaryl, a 3 to 5 membered cycloalkyl or a 3 to 6 membered heterocycloalkyl; [0029] m is 0 or 1; and [0030] n is 1, 2 or 3.

[0031] In embodiments, methods of treating one or more synucleinopathies include administering a compound according to Formula (I), or a pharmaceutically acceptable salt thereof, to a subject in need thereof to provide improvement in one or more symptoms of the one or more synucleinopathies. In embodiments, methods of treating one or more synucleinopathies include administering a compound according to Formula (I), or a pharmaceutically acceptable salt thereof, to a subject in need thereof to provide improvement in two or more symptoms of the one or more synucleinopathies. In embodiments, methods of treating one or more synucleinopathies include administering a compound according to Formula (I), or a pharmaceutically acceptable salt thereof, to a subject in need thereof to provide improvement in three or more symptoms of the one or more synucleinopathies. In embodiments, methods of treating one or more synucleinopathies include administering a compound according to Formula (I), or a pharmaceutically acceptable salt thereof, to a subject in need thereof to provide improvement in next day functioning of the subject. In embodiments, provided herein is a compound of Formula (I), or a pharmaceutically acceptable salt thereof, for use in treating one or more synucleinopathies in a subject. In embodiments, provided herein is a compound of Formula (I), or a pharmaceutically acceptable salt thereof, for use in providing improvement in next day functioning of a subject with one or more synucleinopathies. In embodiments, provided herein is a compound of Formula (I), or a pharmaceutically acceptable salt thereof, for the manufacture of a medicament for treatment of one or more synucleinopathies.

[0032] In embodiments, methods of treating one or more TDP-43 proteinopathies include administering a compound according to Formula (I), or a pharmaceutically acceptable salt thereof, to a subject in need thereof to provide improvement in one or more symptoms of the one or more TDP-43 proteinopathies. In embodiments, methods of treating one or more TDP-43 proteinopathies include administering a compound according to Formula (I), or a pharmaceutically acceptable salt thereof, to a subject in need thereof to provide improvement in two or more symptoms of the one or more TDP-43 proteinopathies. In embodiments, methods of treating one or more TDP-43 proteinopathies include administering a compound according to Formula (I), or a pharmaceutically acceptable salt thereof, to a subject in need thereof to provide improvement in three or more symptoms of the one or more TDP-43 proteinopathies. In embodiments, methods of treating one or more TDP-43 proteinopathies include administering a compound according to Formula (I), or a pharmaceutically acceptable salt thereof, to a subject in need thereof to provide improvement in next day functioning of the subject. In embodiments, provided herein is a compound of Formula (I), or a pharmaceutically acceptable salt thereof, for use in treating one or more TDP-43 proteinopathies in a subject. In embodiments, provided herein is a compound of Formula (I), or a pharmaceutically acceptable salt thereof, for use in providing improvement in next day functioning of a subject with one or more TDP-43 proteinopathies. In embodiments, provided herein is a compound of Formula (I), or a pharmaceutically acceptable salt thereof, for the manufacture of a medicament for treatment of one or more TDP-43 proteinopathies.

[0033] In embodiments, synucleinopathies include Parkinson's disease, Parkinson's disease with dementia, dementia, Lewy body dementia, pure autonomic failure, and multiple system atrophy. In embodiments, TDP-43 proteinopathies include amyotrophic lateral sclerosis and frontotemporal dementia.

[0034] In embodiments, a compound of Formula (I) is Compound A:

##STR00004##

[0035] In embodiments, a compound of Formula (I) is Compound B:

##STR00005##

[0036] In embodiments, a compound of Formula (I) is Compound C:

##STR00006##

[0037] In embodiments, a compound of Formula (I) is Compound D:

##STR00007##

[0038] In embodiments, a compound of Formula (I) is Compound E:

##STR00008##

[0039] In embodiments, a compound of Formula (I) is Compound F:

##STR00009##

[0040] In embodiments, a compound of Formula (I) is Compound G:

##STR00010##

[0041] In embodiments, a compound of Formula (I) is Compound H:

##STR00011##

Description

BRIEF DESCRIPTION OF THE FIGURES

[0042] FIG. 1A is a drug screening workflow chart illustrating the steps taken to identify KCC2 direct activators, a multi-tiered program starting from a compound library of 1.3 million compounds. Identification of a subseries of fused pyrimidine ring compounds, denoted as subseries A (SSA) were identified as lead-hits from the compound library screen.

[0043] FIG. 1B depicts immunoblotting results for HEK-293 cells expressing KCC2 exposed to vehicle (–) or 30 μ M SSA1 (+) for 90 min and then heated to 37-58° C. Soluble fractions were subsequently immunoblotted with KCC2 and GAPDH antibodies.

[0044] FIG. 1C is a graph depicting thermal aggregation curves for KCC2 under control conditions and in the presence of SSA-0126. *=significantly different to control $p < 0.001$; t-test; $n = 4$ transfections. The level of remaining soluble native KCC2 at each temperature was normalized to that seen at 37° C. ($I \times I_{37^\circ \text{C.}}$).

[0045] FIG. 1D is a graph depicting isothermal dose response curves constructed for destabilization of KCC2 by SSA1 at 50° C., $n = 4$ transfections

[0046] FIG. 1E shows the structure of Compound A (also referred to as CmpA).

[0047] FIG. 1F is a graph showing the effects of Compound A on KCC2 activity expressed in HEK-293 cells as measured using TI flux. Data is normalized to values seen with vehicle (100%), $n = 3$.

[0048] FIG. 1G is a chart showing the EC₅₀ of Compound A for KCC2, KCC2-S940A, KCC3, KCC4 and NKCC1 as measured in expressing HEK-293 cells, $n = 4$ transfections. In all panels p values were determined using unpaired t-tests with Welch's correction.

[0049] FIG. 2A depicts three graphs showing E_{sub}Gly values versus time as measured from HEK-293 cells expressing KCC2 together with GlyR α 1 using perforated patch clamp recordings. Individual shifts in E_{sub}Gly are shown for cells incubated with Compound A (0.3 and 3 μ M) or vehicle (V) for 15 min.

[0050] FIG. 2B depicts a first bar graph showing Δ E_{sub}Gly (mM) by concentration of vehicle and

Compound A and a second bar graph showing reduction in [Cl.sup.-] (mM) by concentration of vehicle and Compound A. Mean shifts in E.sub.Gly were determined for cells treated with vehicle, $p=0.1227$, Compound A $0.3\ \mu\text{M}$ ($*p=0.0051$), and $3\ \mu\text{M}$ ($*p=0.0350$), respectively, $n=5$ transfections.

[0051] FIG. 2C depicts immunoblotting results and a bar graph showing surface/total KCC2 (% conc.) by concentration of Compound A and Compound A induced changes in [Cl.sup.-] as calculated from E.sub.Gly using the Nernst equation for vehicle (V), ($p=0.1227$), $300\ \text{nM}$, ($*p=0.0051$), or $3\ \mu\text{M}$ ($*p=0.0350$) CmpA, $n=5$ transfections.

[0052] FIG. 2D depicts immunoblotting results obtained by exposing untransfected HEK-293 cells (U) or those expressing KCC2 to vehicle (V), 0.3 , or $3\ \mu\text{M}$, Compound A, for $15\ \text{min}$ and biotinylated with NHS-Biotin. Surface and total extracts were subsequently immunoblotted with KCC2 and actin antibodies. The ratio of surface/total KCC2 immunoreactivity was determined and normalized to V (100%) for 0.3 ($*p=0.659$) or $3\ \mu\text{M}$ Compound A ($*p=0.739$), $n=3$ transfections.

[0053] FIG. 2E depicts a first bar graph showing pS940/KCC2 (% concentration) of Compound A (μM) and a second bar graph showing pT1007/KCC2 (% concentration) of Compound A (μM). HEK-293 cells were treated with V (-) or $3\ \mu\text{M}$ Compound A (+) for $15\ \text{min}$. Cell lysates were immunoblotted with KCC2, pS940, KCC2 and actin antibodies. The ratios of pS940/KCC2 ($p=0.491$) and pT1007/KCC2 ($p=0.721$) were then compared to V (100%), $n=3$ transfections. In all panels p values were determined using unpaired t-tests with Welch's correction.

[0054] FIG. 3A depicts a graph showing current (pA) versus holding potential (mV) for vehicle and Compound A. 18-21 Div hippocampal neurons were subjected to gramicidin perforated patch clamp recordings in the presence of bumetanide ($10\ \mu\text{M}$) and TTX ($500\ \text{nM}$). After attaining perforation, cultures were exposed to $300\ \text{nM}$ Compound A, or V (1% BCD) for $15\ \text{min}$. E.sub.GABA was then determine using voltage ramps protocols and representative current-voltage (I-V) plots are shown for neurons at 0 (dark gray) and 15 (light gray) min treatment for V and Compound A treated neurons.

[0055] FIG. 3B depicts a first bar graph showing neuronal E.sub.GABA values by exposure to vehicle (V) over time and a second bar graph defining neuronal E.sub.GABA values by exposure to Compound A over time. E.sub.GABA values were measured at 0 and $15\ \text{min}$ following treatment with V ($p=0.2190$) or $300\ \text{nM}$ Compound A ($*p=0.0104$), $n=5$ cultures.

[0056] FIG. 3C depicts a bar graph showing neuronal [Cl.sup.-] values by exposure to Compound A over time. [Cl.sup.-] values were calculated from E.sub.GABA values for V ($p=0.1646$) or Compound A ($*p=0.0153$) treated neurons, $n=5$ cultures.

[0057] FIG. 3D depicts a first bar graph showing neuronal Basal E.sub.GABA (mV) upon exposure to vehicle (V) and Compound A, and a second bar graph showing neuronal $11\text{K}\ \Delta$ E.sub.GABA upon exposure to vehicle (V) and Compound A. 18-21 Div hippocampal neurons were incubated with $300\ \text{nM}$ Compound A or vehicle for $1\ \text{h}$. Neurons were subject to whole cell recording using an intracellular solution containing $30\ \text{mM}$ Cl.sup.-. $5\ \text{min}$ later basal E.sub.GABA values were determined and compared between treatments, $*p=0.0253$, $n=4$ cultures. Neurons were subsequently exposed to $10\ \mu\text{M}$ VU0463271 and the magnitude of the shift in E.sub.GABA (VU0463271 Δ E.sub.GABA) was then determined and compared between groups, $*p=0.040$ $n=4$ cultures.

[0058] FIG. 4A depicts a bar graph showing accumulation of Compound A in the mouse brain within 30 minutes after intravenous injection of $25\ \text{mg/kg}$ Compound A in 5% β -cyclodextrin (BCD). 15 and 60 minutes later drug levels were measured via LC-MS/MS, $n=3$ mice.

[0059] FIG. 4B depicts a bar graph showing accumulation of Compound A in the mouse brain after intravenous injection of $50\ \text{mg/kg}$ Compound A in 5% β -cyclodextrin (BCD). 1 and 2 h later drug levels were measured via LC-MS/MS, $n=3$ mice.

[0060] FIG. 4C is a graph showing accumulation of Compound A in the mouse brain within 30 minutes after subcutaneous injection of $50\ \text{mg/Kg}$ Compound A in 5% β -cyclodextrin (BCD). Drug

accumulation was then measured over a time course of 8 h via LC-MS/MS, n=3-4 mice.

[0061] FIG. 4D depicts a first bar graph showing the effect of Compound A and vehicle (V) on distance traveled after mice were dosed with vehicle (V) or 50 mg/kg Compound A and a second bar graph showing the effect of Compound A and vehicle (V) on time spent in a center zone after mice were dosed with vehicle or 50 mg/kg Compound A. 2-3 h after injection, the mice were placed in the center of a 60 cm×60 cm open field and allowed to explore for 10 min. The total distance traveled (p=0.365) and time in the center of the area (p=0.425) were then quantified and compared between treatments using t-tests; n=9 mice.

DETAILED DESCRIPTION

[0062] Compositions and methods of treating one or more synucleinopathies or one or more TDP-43 proteinopathies are provided and, in embodiments, include administering to a subject in need thereof an effective amount of a compound according to Formula (I):

##STR00012##

or a pharmaceutically acceptable salt thereof, wherein: [0063] R^{sup.1} is selected from C_{sub.2-6}alkyl; C_{sub.2-6}alkenyl; C_{sub.2-6}alkynyl; C_{sub.2-6}alkoxy; C_{sub.2-6}alkenyloxy; C_{sub.2-6}alkynyloxy; C_{sub.3-7}cycloalkyl; —O—C_{sub.3-7}cycloalkyl; C_{sub.6-10}aryl; —O—(CH_{sub.2})_{sub.m}—C_{sub.6-10}aryl; 6 membered heteroaryl; and thiophenyl; wherein alkyl, alkenyl, alkynyl, alkoxy, alkenyloxy, alkynyloxy and cycloalkyl are optionally substituted with 1, 2 or 3 substituents selected from —F and —CF_{sub.3} and wherein aryl and heteroaryl are optionally substituted with 1 or 2 substituents selected from -halo, —C_{sub.1-3}alkyl, —C_{sub.1-8}alkoxy and —C_{sub.2-8}alkynyloxy wherein —C_{sub.1-3}alkyl, —C_{sub.1-8}alkoxy and —C_{sub.2-8}alkynyloxy are optionally substituted with 1, 2, or 3 substituents selected from —F, —CF_{sub.3}, —NHC(O)O—C_{sub.1-6}alkyl or two substituents together with the carbon to which they are attached form diazirinyl; [0064] R^{sup.2} is selected from —H; -halo; and —C_{sub.1-3}alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and —CF_{sub.3}; [0065] A is selected from

##STR00013## [0066] R^{sup.3} is selected from —H; —C_{sub.1-6}alkyl; —C_{sub.2-6}alkenyl; —C_{sub.2-6}alkynyl; C_{sub.3-7}cycloalkyl; and a 5 or 6 membered heterocycloalkyl; wherein the alkyl, alkenyl, alkynyl, cycloalkyl or heterocycloalkyl are optionally substituted by 1, 2 or 3 groups selected from —F, —CF_{sub.3}, —C_{sub.1-3}alkyl optionally substituted by 1 or 2 substituents selected from —F, —CF_{sub.3}, —C(O)NR^{sup.8}R^{sup.9} and —NR^{sup.8}R^{sup.9}; [0067] R^{sup.4a} and R^{sup.4b} are each independently selected from —H and —C_{sub.1-3}alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and CF_{sub.3}; [0068] R^{sup.4c} and R^{sup.4d} are each independently selected from —H and —C_{sub.1-3}alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and CF_{sub.3}, or R^{sup.4c} and R^{sup.4d} together with the carbon to which they are attached represent carbonyl; [0069] R^{sup.5a}, R^{sup.5b}, R^{sup.5c} and R^{sup.5d} are each independently selected from —H and —C_{sub.1-3}alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and CF_{sub.3}; [0070] R^{sup.6} is selected from —H; -halo; —NH_{sub.2}; —CN; —C_{sub.1-3}alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and CF_{sub.3}; —C_{sub.1-3}alkoxy optionally substituted with 1, 2 or 3 substituents selected from —F and —CF_{sub.3}; —C(O)O—C_{sub.1-3}alkyl; —C(O)NR^{sup.8}R^{sup.9}; —C(O)OH; and —NHC(O)—C_{sub.1-3}alkyl; [0071] R^{sup.7} is selected from NR^{sup.10}R^{sup.11}; a 5 to 7 membered monocyclic heterocycloalkyl; and a 5 or 6 membered monocyclic heteroaryl; wherein the heterocycloalkyl and heteroaryl are optionally substituted with 1, 2 or 3 groups selected from —CN; —C_{sub.1-6}alkyl optionally substituted with 1, 2 or 3 substituents selected from —F, —CF_{sub.3} and —OH; —C_{sub.1-3}alkoxy optionally substituted with 1, 2 or 3 substituents selected from —F and CF_{sub.3}; —C(O)OH; —C_{sub.1-3}alkylene-NHC(O)C_{sub.1-6}alkyl; —C_{sub.1-3}alkylene-NHC(O)OC_{sub.1-6}alkyl; C_{sub.3-5}cycloalkyl; or the heterocycloalkyl is optionally substituted with two substituents on the same ring carbon which together with the carbon atom to which they are attached form a 5 to 7 membered monocyclic heterocycloalkyl; and wherein when R^{sup.7} is morpholinyl and R^{sup.1} is unsubstituted phenyl,

R.sup.2 is not —H; [0072] R.sup.8 and R.sup.9 are each independently selected from —H and —C.sub.1-6 alkyl; [0073] R.sup.10 is —C.sub.1-6alkyl; [0074] R.sup.11 is selected from —C.sub.1-6alkyl optionally substituted with 1 or 2 substituents selected from —F and —C.sub.1-3alkoxy; and —(CH.sub.2).sub.nR.sup.12; [0075] R.sup.12 is a 5 or 6 membered heteroaryl, a 3 to 5 membered cycloalkyl or a 3 to 6 membered heterocycloalkyl; [0076] m is 0 or 1; and [0077] n is 1, 2 or 3.

[0078] In embodiments, methods of treating one or more synucleinopathies include administering a compound according to Formula (I), or a pharmaceutically acceptable salt thereof, to a subject in need thereof to provide improvement in one or more symptoms of the one or more synucleinopathies. In embodiments, methods of treating one or more synucleinopathies include administering a compound according to Formula (I), or a pharmaceutically acceptable salt thereof, to a subject in need thereof to provide improvement in two or more symptoms of the one or more synucleinopathies. In embodiments, methods of treating one or more synucleinopathies include administering a compound according to Formula (I), or a pharmaceutically acceptable salt thereof, to a subject in need thereof to provide improvement in three or more symptoms of the one or more synucleinopathies. In embodiments, methods of treating one or more synucleinopathies include administering a compound according to Formula (I), or a pharmaceutically acceptable salt thereof, to a subject in need thereof to provide improvement in next day functioning of the subject. In embodiments, provided herein is a compound of Formula (I), or a pharmaceutically acceptable salt thereof, for use in treating one or more synucleinopathies in a subject. In embodiments, provided herein is a compound of Formula (I), or a pharmaceutically acceptable salt thereof, for use in providing improvement in next day functioning of a subject with one or more synucleinopathies. In embodiments, provided herein is a compound of Formula (I), or a pharmaceutically acceptable salt thereof, for the manufacture of a medicament for treatment of one or more synucleinopathies.

[0079] In embodiments, methods of treating one or more TDP-43 proteinopathies include administering a compound according to Formula (I), or a pharmaceutically acceptable salt thereof, to a subject in need thereof to provide improvement in one or more symptoms of the one or more TDP-43 proteinopathies. In embodiments, methods of treating one or more TDP-43 proteinopathies include administering a compound according to Formula (I), or a pharmaceutically acceptable salt thereof, to a subject in need thereof to provide improvement in two or more symptoms of the one or more TDP-43 proteinopathies. In embodiments, methods of treating one or more TDP-43 proteinopathies include administering a compound according to Formula (I), or a pharmaceutically acceptable salt thereof, to a subject in need thereof to provide improvement in three or more symptoms of the one or more TDP-43 proteinopathies. In embodiments, methods of treating one or more TDP-43 proteinopathies include administering a compound according to Formula (I), or a pharmaceutically acceptable salt thereof, to a subject in need thereof to provide improvement in next day functioning of the subject. In embodiments, provided herein is a compound of Formula (I), or a pharmaceutically acceptable salt thereof, for use in treating one or more TDP-43 proteinopathies in a subject. In embodiments, provided herein is a compound of Formula (I), or a pharmaceutically acceptable salt thereof, for use in providing improvement in next day functioning of a subject with one or more TDP-43 proteinopathies. In embodiments, provided herein is a compound of Formula (I), or a pharmaceutically acceptable salt thereof, for the manufacture of a medicament for treatment of one or more TDP-43 proteinopathies.

[0080] In embodiments, synucleinopathies being treated include Parkinson's disease, Parkinson's disease with dementia, dementia, Lewy body dementia, pure autonomic failure, and multiple system atrophy. In embodiments, TDP-43 proteinopathies being treated include amyotrophic lateral sclerosis and frontotemporal dementia.

[0081] In embodiments, the synucleinopathy being treated is Parkinson's disease. Parkinson's disease is a chronic and progressive neurological disease that is marked by tremors in the resting muscles, rigidity, slowness of movement, impaired balance and a shuffling gait. In addition, many

patients with PD develop non-motor symptoms such as behavioral changes and cognitive impairment. PD symptoms include, tremor in hands, arms, legs, jaw, or head, muscle stiffness, where muscle remains contracted for a long time, slowness of movement, impaired balance and coordination, sometimes leading to falls, depression and other emotional changes, difficulty swallowing, chewing, and speaking, urinary problems, constipation, reduced or lack of facial expression and animation, parkinsonian gait including a tendency to lean forward, taking small, quick steps, and reduced arm swinging. Patients with PD may have trouble initiating or continuing movement. In embodiments, administration of a compound of Formula (I), or a pharmaceutically acceptable salt thereof to a subject in need thereof provides improvement in one or more symptoms of PD. In embodiments, administration of a compound of Formula (I), or a pharmaceutically acceptable salt thereof to a subject in need thereof provides improvement in two or more symptoms of PD. In embodiments, administration of a compound of Formula (I), or a pharmaceutically acceptable salt thereof to a subject in need thereof provides improvement in three or more symptoms of PD. In embodiments, methods of treating PD include administering a compound according to Formula (I), or a pharmaceutically acceptable salt thereof, to a subject in need thereof to provide improvement in next day functioning of the subject in that there is improvement in one or more symptoms of PD a day after administration.

[0082] In embodiments, the synucleinopathy being treated is Parkinson's disease with dementia. Dementia is the loss of cognitive functioning—the ability to think, remember, problem solve or reason—to such an extent that it interferes with a person's daily life and activities. Patients having Parkinson's disease with dementia may experience one or more symptoms described above along with dementia. Dementia can range in severity from the mildest stage, when it is just beginning to affect a person's functioning, to the most severe stage, when the person must depend completely on others for basic activities of daily living. Symptoms of dementia include loss of or reduced memory, loss of or reduced language skills, loss of or reduced visual perception, diminished problem solving, inability to self-manage, and loss of or reduced ability to focus and pay attention. Some patients with dementia cannot control their emotions (emotional lability), and their personalities may change. In some instances, patients with dementia experience delusions and/or hallucinations. In embodiments, administration of a compound of Formula (I), or a pharmaceutically acceptable salt thereof to a subject in need thereof provides improvement in one or more symptoms of PD with dementia. In embodiments, administration of a compound of Formula (I), or a pharmaceutically acceptable salt thereof to a subject in need thereof provides improvement in two or more symptoms of PD with dementia. In embodiments, administration of a compound of Formula (I), or a pharmaceutically acceptable salt thereof to a subject in need thereof provides improvement in three or more symptoms of PD with dementia. In embodiments, methods of treating PD with dementia include administering a compound according to Formula (I), or a pharmaceutically acceptable salt thereof, to a subject in need thereof to provide improvement in next day functioning of the subject in that there is improvement in one or more symptoms of PD with dementia a day after administration.

[0083] In embodiments, the condition being treated is dementia from any cause. In embodiments, administration of a compound of Formula (I), or a pharmaceutically acceptable salt thereof to a subject in need thereof provides improvement in one or more symptoms of dementia. In embodiments, administration of a compound of Formula (I), or a pharmaceutically acceptable salt thereof to a subject in need thereof provides improvement in two or more symptoms of dementia. In embodiments, administration of a compound of Formula (I), or a pharmaceutically acceptable salt thereof to a subject in need thereof provides improvement in three or more symptoms of dementia. In embodiments, methods of treating dementia include administering a compound according to Formula (I), or a pharmaceutically acceptable salt thereof, to a subject in need thereof to provide improvement in next day functioning of the subject in that there is improvement in one or more symptoms of dementia a day after administration.

[0084] In embodiments, the synucleinopathy being treated is Lewy body dementia (LBD).

Symptoms of LBD include visual hallucinations, or seeing things that are not present, nonvisual hallucinations, such as hearing or smelling things that are not present, cognitive problems such as unpredictable changes in concentration, attention, alertness, and wakefulness from day to day and sometimes throughout the day, ideas may be disorganized, unclear, or illogical, severe loss of thinking abilities that interfere with daily activities. Unlike in Alzheimer's dementia, memory problems may not be evident at first but often arise as LBD progresses. Other symptoms related to cognition may include poor judgment, confusion about time and place, and difficulty with language and numbers. Movement symptoms may include ataxia, muscle rigidity or stiffness, shuffling walk, slow movement, or frozen stance, tremor or shaking, most commonly at rest, balance problems and repeated falls, stooped posture, loss of coordination, smaller handwriting than was usual for the person, reduced facial expression, difficulty swallowing, weak voice. Other symptoms of LBD include sleep disorders such as insomnia, REM sleep behavior disorder, restless leg syndrome, depression, apathy, anxiety, agitation, aggression, and paranoia. Autonomic symptoms include diminished regulation of body temperature, diminished regulation of blood pressure, dizziness, syncope, sensitivity to heat and cold, sexual dysfunction, urinary incontinence, constipation and poor sense of smell. In embodiments, administration of a compound of Formula (I), or a pharmaceutically acceptable salt thereof to a subject in need thereof provides improvement in one or more symptoms of LBD. In embodiments, administration of a compound of Formula (I), or a pharmaceutically acceptable salt thereof to a subject in need thereof provides improvement in two or more symptoms of LBD. In embodiments, administration of a compound of Formula (I), or a pharmaceutically acceptable salt thereof to a subject in need thereof provides improvement in three or more symptoms of LBD. In embodiments, methods of treating LBD include administering a compound according to Formula (I), or a pharmaceutically acceptable salt thereof, to a subject in need thereof to provide improvement in next day functioning of the subject in that there is improvement in one or more symptoms of LBD a day after administration.

[0085] In embodiments, the synucleinopathy being treated is pure autonomic failure (PAF).

Symptoms of PAF include orthostatic hypotension, e.g., a fall in blood pressure by more than 20 mm systolic or 10 mm diastolic after at least 1 minute of standing, dizziness or faintness including syncope, light-headedness, visual disturbances such as blurry vision, neck pain, breathing difficulty, abnormal sweating, decrease in sweating (anhidrosis/hypohidrosis) which can lead to heat intolerance, night sweats, poor gastric motility, fatigue, weakness, sexual dysfunction, impotence and sleep disorders. Constipation and urinary problems such as frequent urination, incontinence and frequent urinary infections may also occur. In embodiments, administration of a compound of Formula (I), or a pharmaceutically acceptable salt thereof to a subject in need thereof provides improvement in one or more symptoms of PAF. In embodiments, administration of a compound of Formula (I), or a pharmaceutically acceptable salt thereof to a subject in need thereof provides improvement in two or more symptoms of PAF. In embodiments, administration of a compound of Formula (I), or a pharmaceutically acceptable salt thereof to a subject in need thereof provides improvement in three or more symptoms of PAF. In embodiments, methods of treating PAF include administering a compound according to Formula (I), or a pharmaceutically acceptable salt thereof, to a subject in need thereof to provide improvement in next day functioning of the subject in that there is improvement in one or more symptoms of PAF a day after administration.

[0086] In embodiments, the synucleinopathy being treated is multiple system atrophy (MSA).

Symptoms include slowness of movement, tremor, or rigidity (stiffness), clumsiness or coordination problems, impaired speech, hoarseness, orthostatic hypotension, bladder control problems, incontinence, urinary retention, muscle contractures, abnormal posture, bending of the neck, involuntary sighing, insomnia, emotional problems, breathing problems while sleeping (sleep apnea), irregular heart rhythms, tremor, problems of balance, coordination, and autonomic nervous system dysfunction. Cerebellar type (MSA-C) has primary symptoms of cerebellar ataxia such as

problems with balance and coordination, difficulty swallowing and speaking, and abnormal eye movements. In embodiments, administration of a compound of Formula (I), or a pharmaceutically acceptable salt thereof to a subject in need thereof provides improvement in one or more symptoms of MSA. In embodiments, administration of a compound of Formula (I), or a pharmaceutically acceptable salt thereof to a subject in need thereof provides improvement in two or more symptoms of MSA. In embodiments, administration of a compound of Formula (I), or a pharmaceutically acceptable salt thereof to a subject in need thereof provides improvement in three or more symptoms of MSA. In embodiments, methods of treating MSA include administering a compound according to Formula (I), or a pharmaceutically acceptable salt thereof, to a subject in need thereof to provide improvement in next day functioning of the subject in that there is improvement in one or more symptoms of MSA a day after administration.

[0087] In embodiments, the TD-43 proteinopathy being treated is amyotrophic lateral sclerosis (ALS). Symptoms of ALS include muscle twitches in the arm, leg, shoulder, or tongue, muscle cramps, tight and stiff muscles, spasticity, muscle weakness affecting an arm, a leg, the neck, or diaphragm, difficulty moving, slurred speech, nasal speech, difficulty chewing or swallowing (dysphagia), difficulty forming words (dysarthria), difficulty breathing (dyspnea), anxiety, depression, dementia and neuropathy. In embodiments, administration of a compound of Formula (I), or a pharmaceutically acceptable salt thereof to a subject in need thereof provides improvement in one or more symptoms of ALS. In embodiments, administration of a compound of Formula (I), or a pharmaceutically acceptable salt thereof to a subject in need thereof provides improvement in two or more symptoms of ALS. In embodiments, administration of a compound of Formula (I), or a pharmaceutically acceptable salt thereof to a subject in need thereof provides improvement in three or more symptoms of ALS. In embodiments, methods of treating ALS include administering a compound according to Formula (I), or a pharmaceutically acceptable salt thereof, to a subject in need thereof to provide improvement in next day functioning of the subject in that there is improvement in one or more symptoms of ALS a day after administration.

[0088] In embodiments, the TD-43 proteinopathy being treated is frontotemporal dementia (FTD). Symptoms of FTD include changes in behavior and personality, such as increasingly inappropriate social behavior, loss of empathy, lack of judgment, loss of inhibition, lack of interest (apathy), repetitive compulsive behavior, such as tapping, clapping or smacking lips, a decline in personal hygiene, changes in eating habits, usually overeating or developing a preference for sweets and carbohydrates, eating inedible objects, compulsively wanting to put things in the mouth, problems or impairment or loss of speech, primary progressive aphasia, semantic dementia and progressive agrammatic (nonfluent) aphasia, e.g., difficulty in using and understanding written and spoken language, such as having trouble finding the right word to use in speech or naming objects, trouble naming things, possibly replacing a specific word with a more general word such as “it” for pen, no longer knowing word meanings, having hesitant speech that may sound telegraphic, and making mistakes in sentence construction. Motor-related problems may include tremor, rigidity, muscle spasms or twitches, poor coordination, difficulty swallowing, muscle weakness, inappropriate laughing or crying, and falls or walking problems. In embodiments, administration of a compound of Formula (I), or a pharmaceutically acceptable salt thereof to a subject in need thereof provides improvement in one or more symptoms of FTD. In embodiments, administration of a compound of Formula (I), or a pharmaceutically acceptable salt thereof to a subject in need thereof provides improvement in two or more symptoms of FTD. In embodiments, administration of a compound of Formula (I), or a pharmaceutically acceptable salt thereof to a subject in need thereof provides improvement in three or more symptoms of FTD. In embodiments, methods of treating FTD include administering a compound according to Formula (I), or a pharmaceutically acceptable salt thereof, to a subject in need thereof to provide improvement in next day functioning of the subject in that there is improvement in one or more symptoms of FTD a day after administration.

[0089] In embodiments, the terms “effective amount” or “therapeutically effective amount” as

applied to a treating one or more synucleinopathies or one or more TDP-43 proteinopathies refer to an amount of a compound, material, composition, medicament, or other material that is effective to achieve a particular pharmacological and/or physiologic effect in connection with symptoms of one or more synucleinopathies or symptoms of one or more TDP-43 proteinopathies described above.

[0090] In embodiments, effective amount refers to an amount which may be suitable to prevent a decline in any one or more of the above symptoms, or, in embodiments, to improve any one or more of the above symptoms. In embodiments, effective amount refers to an amount which may be suitable to prevent a decline in two or more of the above symptoms, or, in embodiments, to improve two or more of the above symptoms. In embodiments, effective amount refers to an amount which may be suitable to prevent a decline in three or more of the above symptoms, or, in embodiments, to improve three or more of the above symptoms. In embodiments, an effective amount may be suitable to reduce either the extent or rate of decline in a subject's cognitive skills or functioning, and/or the effective amount may be suitable to delay the onset of such decline. Such effectiveness may be achieved, for example, by administering compositions described herein to an individual or to a population. In embodiments, the reduction, or delay of such a decline, or the improvement in an individual or population can be relative to a cohort, e.g., a control subject or a cohort population that has not received the treatment or been administered the composition or medicament. The terms “effective amount” and “therapeutically effective amount” are used interchangeably herein.

[0091] The dosage amount can vary according to a variety of factors such as subject-dependent variables (e.g., age, immune system, health, etc.), the disease or disorder being treated, as well as the route of administration and the pharmacokinetics of the agent being administered.

[0092] Many pharmaceutical products are administered as a fixed dose, at regular intervals, to achieve therapeutic efficacy. Duration of action is typically reflected by a drug's plasma half-life. Since efficacy is often dependent on sufficient exposure within the central nervous system administration of CNS drugs with a short half-life may require frequent maintenance dosing. CNS drugs with a long half-life may require less frequent maintenance dosing.

[0093] It should be understood that in the context of Formula (I) and other Formulas disclosed herein that unless otherwise indicated, the term “alkyl” includes both linear and branched chain alkyl groups. The prefix C.sub.p-q in C.sub.p-q alkyl and other terms (where p and q are integers) indicates the range of carbon atoms that are present in the group, for example C.sub.1-3alkyl includes C.sub.1alkyl (methyl), C.sub.2alkyl (ethyl) and C.sub.3alkyl (propyl as n-propyl and isopropyl).

[0094] The term “C.sub.p-q alkoxy” comprises —O—C.sub.p-q alkyl groups and —C.sub.p-q alkyl groups where the O atom is within the alkyl chain, for example, —CH.sub.2—O—CH.sub.3.

[0095] The term “C.sub.p-q alkenyl” includes both linear and branched chain alkyl groups containing at least two carbon atoms and at least one double carbon-carbon bond.

[0096] The term “C.sub.p-q alkenyloxy” comprises —O—C.sub.p-q alkenyl groups and —C.sub.p-q alkenyl groups where the O atom is within the alkenyl chain.

[0097] The term “C.sub.p-q alkynyl” includes both linear and branched chain alkyl groups containing at least two carbon atoms and at least one triple carbon-carbon bond.

[0098] The term “C.sub.p-q alkynyloxy” comprises —O—C.sub.p-q alkynyl groups and —C.sub.p-q alkynyl groups where the O atoms is within the alkynyl chain.

[0099] C.sub.p-q cycloalkyl refers to a cyclic non-aromatic group of p-q carbon atoms and no heteroatoms. For example, a 3 to 7 membered cycloalkyl refers to a ring containing 3 to 7 carbon atoms. Suitable C.sub.3-7cycloalkyl rings include cyclopropyl, cyclobutyl, cyclopentyl and cyclohexyl.

[0100] Aryl is a 6 to 10 membered monocyclic or bicyclic aromatic ring containing no heteroatoms. Aryl includes phenyl.

[0101] Heterocycloalkyl is a monocyclic saturated or partially unsaturated, non-aromatic ring

having, for example, 3 to 7 members, such as 3 to 6 members, 5 to 7 members such as 5 or 6 members, where at least one member and up to 4 members, particularly 1, 2 or 3 members of the ring are heteroatoms selected from N, O and S, and the remaining ring atoms are carbon atoms, in stable combinations known to those of skill in the art. Heterocycloalkyl ring nitrogen and sulphur atoms are optionally oxidised. Suitable heterocycloalkyl rings include morpholinyl, thiazolidinyl, homomorpholine, tetrahydropyranyl, pyrrolyl, thiomorpholinyl and tetrahydrofuranyl. In embodiments, when R^{sup.7} is heterocycloalkyl, optionally two substituents on the same ring carbon together with the carbon to which they are attached form a 5 to 7 membered heterocycloalkyl ring, thereby creating a spirocyclic ring system. For example, in embodiments, R^{sup.7} is morpholinyl and two substituents on the same ring carbon together form a tetrahydropyran.

[0102] Heteroaryl is a polyunsaturated, monocyclic 5 or 6 membered aromatic ring containing at least one and up to 3 heteroatoms, particularly, 1 or 2 heteroatoms selected from N, O and S, and the remaining ring atoms are carbon atoms. Heteroaryl ring nitrogen and sulphur atoms are optionally oxidised. Suitable heteroaryl rings include pyridinyl, isoxazolyl, oxadiazolyl, imidazolyl, pyrazinyl, oxazolyl, thiophenyl and thiazolyl.

[0103] The term “halo” is fluorine, chlorine or bromine.

[0104] The use of the dashed bond “-----” in rings A of Formula (I) represents the fusion of the pyrimidine ring.

[0105] Where the term “optionally” is used, it is intended that the subsequent feature may or may not occur. As such, use of the term “optionally” includes instances where the feature is present, and also instances where the feature is not present. For example, a group “optionally substituted with 1, 2 or 3 —F substituents” includes group with and without an —F substituent.

[0106] The term “substituted” means that one or more hydrogens (for example 1 or 2 hydrogens, or alternatively 1 hydrogen) on the designated group is replaced by the indicated substituent(s) (for example 1, 2 or 3 substituents, or alternatively 1 or 2 substituents, or alternatively 1 substituent), provided that any atom(s) bearing a substituent maintains a permitted valency. Substituent combinations encompass only stable compounds and stable synthetic intermediates. “Stable” means that the relevant compound or intermediate is sufficiently robust to be isolated and have utility either as a synthetic intermediate or as an agent having potential therapeutic utility. If a group is not described as “substituted”, or “optionally substituted”, it is to be regarded as unsubstituted (i.e. that none of the hydrogens on the designated group have been replaced).

[0107] The term “pharmaceutically acceptable” is used to specify that an object (for example a salt, dosage form or excipient) is suitable for use in patients. An example list of pharmaceutically acceptable salts can be found in the *Handbook of Pharmaceutical Salts: Properties, Selection and Use*, P. H. Stahl and C. G. Wermuth, editors, Weinheim/Zürich: Wiley-VCH/VHCA, 2002. In embodiments, a suitable pharmaceutically acceptable salt of a compound of the Formula (I) is, for example, a salt formed within the human or animal body after administration of a compound of the Formula (I), to said human or animal body.

[0108] In embodiments, a suitable pharmaceutically acceptable salt of a compound of Formula (I) is, for example, an acid addition salt. An acid addition salt of a compound of Formula (I) may be formed by bringing the compound into contact with a suitable inorganic or organic acid under conditions known to the skilled person. In embodiments, compounds described herein may form base addition salts. A base-addition salt of a compound of Formula (I) may be formed by bringing the compound into contact with a suitable inorganic or organic base under conditions known to the skilled person.

[0109] In embodiments, Formula (I) may be provided as an acid addition salt, a zwitter ion hydrate, zwitter ion anhydrate, hydrochloride or hydrobromide salt, or in the form of the zwitter ion monohydrate. Acid addition salts, include but are not limited to, maleic, fumaric, benzoic, ascorbic, succinic, oxalic, bis-methylenesalicylic, methanesulfonic, ethane-disulfonic, acetic, propionic,

tartaric, salicylic, citric, gluconic, lactic, malic, mandelic, cinnamic, citraconic, aspartic, stearic, palmitic, itaconic, glycolic, pantothenic, p-amino-benzoic, glutamic, benzene sulfonic or theophylline acetic acid addition salts, as well as the 8-halotheophyllines, for example 8-bromotheophylline. In embodiments, inorganic acid addition salts, including but not limited to, hydrochloric, hydrobromic, hydroiodic, sulfuric, sulfamic, phosphoric or nitric acid addition salts may be used.

[0110] In embodiments, there is provided a compound of Formula (I), or a pharmaceutically acceptable salt thereof. In embodiments, there is provided a compound of Formula (I). In embodiments there is provided a pharmaceutically acceptable salt of a compound of Formula (I).

[0111] Compounds and salts described herein may exist in solvated forms and unsolvated forms. For example, a solvated form may be a hydrated form, such as a hemi-hydrate, a monohydrate, a dihydrate, a trihydrate or an alternative quantity thereof. The description herein encompasses all such solvated and unsolvated forms of compounds of Formula (I), particularly to the extent that such forms possess KCC2 modulating activity, as for example measured using the tests described herein.

[0112] The following embodiments of moiety A may be applied to the description of the compounds of Formula (I), provided herein:

[0113] Moiety A is selected from:

##STR00014##

or a N-oxide thereof.

[0114] In embodiments, moiety A is

##STR00015##

[0115] In embodiments, moiety A is

##STR00016##

[0116] In embodiments, moiety A is

##STR00017##

[0117] In embodiments, moiety A is

##STR00018##

[0118] In embodiments, moiety A is

##STR00019##

[0119] In embodiments, there is provided a compound of Formula (II):

##STR00020##

or a pharmaceutically acceptable salt thereof, wherein R^{sup.1}, R^{sup.2}, R^{sup.3}, R^{sup.4a}, R^{sup.4b} and R^{sup.7} are as defined for Formula (I).

[0120] In embodiments, there is provided a compound of Formula (II) or a pharmaceutically acceptable salt thereof, wherein R^{sup.1}, R^{sup.2}, R^{sup.3}, R^{sup.4a}, R^{sup.4b} and R^{sup.7} are as defined for Formula (I) and when R^{sup.7} is morpholinyl, either:

[0121] R^{sup.1} is selected from C_{sub.2-6}alkyl; C_{sub.2-6}alkenyl; C_{sub.2-6}alkynyl; C_{sub.2-6}alkoxy; C_{sub.2-6}alkenyloxy; C_{sub.2-6}alkynyloxy; C_{sub.3-7}cycloalkyl; —O—C_{sub.3-7}cycloalkyl; C_{sub.6-10}aryl; —O—(CH_{sub.2})_{sub.m}—C_{sub.6-10}aryl; 6 membered heteroaryl; and thiophenyl; wherein alkyl, alkenyl, alkynyl, alkoxy, alkenyloxy, alkynyloxy and cycloalkyl are optionally substituted with 1, 2 or 3 substituents selected from —F and —CF_{sub.3}; heteroaryl is optionally substituted with 1 or 2 substituents selected from -halo, —C_{sub.1-3}alkyl, —C_{sub.1-8}alkoxy and —C_{sub.2-8}alkynyloxy, wherein —C_{sub.1-3}alkyl, —C_{sub.1-8}alkoxy and —C_{sub.2-8}alkynyloxy are optionally substituted with 1, 2, or 3 substituents selected from —F, —CF_{sub.3}, —NHC(O)O—C_{sub.1-6}alkyl or two substituents together with the carbon to which they are attached form diazirinyl; and aryl is substituted with 1 or 2 substituents selected from -halo, —C_{sub.1-3}alkyl, —C_{sub.1-8}alkoxy and —C_{sub.2-8}alkynyloxy, wherein —C_{sub.1-3}alkyl, —C_{sub.1-8}alkoxy and —C_{sub.2-8}alkynyloxy are optionally substituted with 1, 2, or 3 substituents selected from —F, —CF_{sub.3}, —NHC(O)O—C_{sub.1-6}alkyl or two substituents together with the

carbon to which they are attached form diazirinyl; and R.sup.2 is selected from —H; -halo; and —C.sub.1-3alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and —CF.sub.3; or [0122] R.sup.1 is selected from C.sub.2-6alkyl; C.sub.2-6alkenyl; C.sub.2-6alkynyl; C.sub.2-6alkoxy; C.sub.2-6alkenyloxy; C.sub.2-6alkynyloxy; C.sub.3-7cycloalkyl; —O—C.sub.3-7cycloalkyl; C.sub.6-10aryl; —O—(CH.sub.2).sub.m—C.sub.6-10aryl; 6 membered heteroaryl; and thiophenyl; wherein alkyl, alkenyl, alkynyl, alkoxy, alkenyloxy, alkynyloxy and cycloalkyl are optionally substituted with 1, 2 or 3 substituents selected from —F and —CF.sub.3; and heteroaryl is optionally substituted with 1 or 2 substituents selected from -halo, —C.sub.1-3alkyl, —C.sub.1-8alkoxy and —C.sub.2-8alkynyloxy, wherein —C.sub.1-3alkyl, —C.sub.1-8alkoxy and —C.sub.2-8alkynyloxy are optionally substituted with 1, 2, or 3 substituents selected from —F, —CF.sub.3, —NHC(O)O—C.sub.1-6alkyl or two substituents together with the carbon to which they are attached form diazirinyl; and R.sup.2 is selected from -halo and —C.sub.1-3alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and —CF.sub.3.

[0123] In embodiments, there is provided a compound of Formula (II) or a pharmaceutically acceptable salt thereof, wherein: [0124] R.sup.1 is selected from —C.sub.2-6alkyl; —C.sub.2-6alkoxy; C.sub.3-7cycloalkyl; —O—C.sub.3-7cycloalkyl; phenyl optionally substituted with 1 or 2 substituents selected from -halo, —C.sub.1-3alkyl, —C.sub.1-8alkoxy and —C.sub.2-8alkynyloxy wherein —C.sub.1-3alkyl, —C.sub.1-8alkoxy and —C.sub.2-8alkynyloxy are optionally substituted with 1, 2, or 3 substituents selected from —F, —CF.sub.3, —NHC(O)O—C.sub.1-6alkyl or two substituents together with the carbon to which they are attached form diazirinyl; —O-phenyl optionally substituted with 1 or 2 -halo substituents; —O—CH.sub.2-phenyl; and thiophenyl; wherein —C.sub.2-6alkyl and —C.sub.2-6alkoxy are optionally substituted with 1, 2 or 3 substituents selected from —F and —CF.sub.3; [0125] R.sup.2 is selected from —H, —F and —CH.sub.3; [0126] R.sup.3 is selected from —C.sub.2-4alkynyl and —C.sub.1-3alkyl optionally substituted with —NR.sup.8R.sup.9; [0127] R.sup.4a and R.sup.4b are both —H; [0128] R.sup.7 is selected from —NR.sup.10R.sup.11; a 5 to 7 membered monocyclic heterocycloalkyl; and a 5 or 6 membered monocyclic heteroaryl; wherein the heterocycloalkyl and heteroaryl are optionally substituted with 1 or 2 substituents selected from —CN; —C.sub.1-3alkyl optionally substituted with 1, 2 or 3 substituents selected from —F, —CF.sub.3 and —OH; —C.sub.1-3alkoxy; cyclopropyl; —C(O)OH; —C.sub.1-3alkylene-NHC(O)C.sub.1-6alkyl; —C.sub.1-3alkylene-NHC(O)OC.sub.1-6alkyl; or the heterocycloalkyl is optionally substituted with two substituents on the same ring carbon which together with the carbon atom to which they are attached form a 6 membered monocyclic heterocycloalkyl; [0129] R.sup.8 and R.sup.9 are each independently selected from —C.sub.1-6alkyl; [0130] R.sup.10 is selected from —C.sub.1-3alkyl; [0131] R.sup.11 is selected from —C.sub.1-3alkyl optionally substituted with 1 or 2 substituents selected from —F and —C.sub.1-3alkoxy; and —(CH.sub.2).sub.nR.sup.12; [0132] R.sup.12 is selected from a 5 or 6 membered heteroaryl, a 3 to 5 membered cycloalkyl or a 3 to 6 membered heterocycloalkyl; [0133] n is 1 or 2.

[0134] In embodiments, there is provided a compound of Formula (II) or a pharmaceutically acceptable salt thereof, wherein: [0135] R.sup.1 is selected from —C.sub.2-6alkyl; —C.sub.2-6alkoxy; C.sub.3-7cycloalkyl; —O—C.sub.3-7cycloalkyl; phenyl substituted with 1 or 2 substituents selected from -halo, —C.sub.1-3alkyl, —C.sub.1-8alkoxy and —C.sub.2-8alkynyloxy wherein —C.sub.1-3alkyl, —C.sub.1-8alkoxy and —C.sub.2-8alkynyloxy are optionally substituted with 1, 2, or 3 substituents selected from —F, —CF.sub.3, —NHC(O)O—C.sub.1-6alkyl or two substituents together with the carbon to which they are attached form diazirinyl; —O-phenyl optionally substituted with 1 or 2 -halo substituents; —O—CH.sub.2-phenyl; and thiophenyl; wherein —C.sub.2alkyl and —C.sub.2alkoxy are optionally substituted with 1, 2 or 3 substituents selected from —F and —CF.sub.3; and R.sup.2 is selected from —H, —F and —CH.sub.3; or

[0136] R.sup.1 is selected from —C.sub.2-6alkyl; —C.sub.2-6alkoxy; C.sub.3-7cycloalkyl; —O—

C.sub.3-7cycloalkyl; unsubstituted phenyl; —O-phenyl optionally substituted with 1 or 2 -halo substituents; —O—CH.sub.2-phenyl; and thiophenyl; wherein —C.sub.2-6alkyl and —C.sub.2-6alkoxy are optionally substituted with 1, 2 or 3 substituents selected from —F and —CF.sub.3; and R.sup.2 is selected from —F and —CH.sub.3; [0137] R.sup.3 is selected from —C.sub.2-4alkynyl and —C.sub.1-3alkyl optionally substituted with —NR.sup.8R.sup.9; [0138] R.sup.4a and R.sup.4b are both —H; [0139] R.sup.7 is selected from —NR.sup.10R.sup.11; a 5 to 7 membered monocyclic heterocycloalkyl; and a 5 or 6 membered monocyclic heteroaryl; wherein the heterocycloalkyl and heteroaryl are optionally substituted with 1 or 2 substituents selected from —CN; —C.sub.1-3alkyl optionally substituted with 1, 2 or 3 substituents selected from —F, —CF.sub.3 and —OH; —C.sub.1-3alkoxy; cyclopropyl; —C(O)OH; —C.sub.1-3alkylene-NHC(O)C.sub.1-6alkyl; —C.sub.1-3alkylene-NHC(O)OC.sub.1-6alkyl; or the heterocycloalkyl is optionally substituted with two substituents on the same ring carbon which together with the carbon atom to which they are attached form a 6 membered monocyclic heterocycloalkyl; [0140] R.sup.8 and R.sup.9 are each independently selected from —C.sub.1-6alkyl; [0141] R.sup.10 is selected from —C.sub.1-3alkyl; [0142] R.sup.11 is selected from —C.sub.1-3alkyl optionally substituted with 1 or 2 substituents selected from —F and —C.sub.1-3alkoxy; and —(CH.sub.2).sub.nR.sup.12; [0143] R.sup.12 is selected from a 5 or 6 membered heteroaryl, a 3 to 5 membered cycloalkyl or a 3 to 6 membered heterocycloalkyl; and [0144] n is 1 or 2.

[0145] In embodiments, there is provided a compound of Formula (III):

##STR00021##

or a pharmaceutically acceptable salt thereof, wherein R.sup.1, R.sup.2, R.sup.5a, R.sup.5b, R.sup.5c, R.sup.5d and R.sup.7 are as defined for Formula (I).

[0146] In embodiments, there is provided a compound of Formula (III) or a pharmaceutically acceptable salt thereof, wherein:

[0147] R.sup.1 is selected from C.sub.3-7cycloalkyl and C.sub.6-10aryl, wherein the aryl is optionally substituted with a —C.sub.2-8alkoxy substituent wherein the alkoxy is optionally substituted with 1 or 2 —CF.sub.3 substituents; [0148] R.sup.2 is —H; [0149] R.sup.5a, R.sup.5b, R.sup.5c and R.sup.5d are each —H; [0150] R.sup.7 is selected from —NR.sup.10R.sup.11; a 5 to 7 membered monocyclic heterocycloalkyl; and a 5 or 6 membered monocyclic heteroaryl; wherein the heterocycloalkyl and heteroaryl are optionally substituted with 1, 2 or 3 groups selected from —CN; —C.sub.1-6alkyl optionally substituted with 1, 2 or 3 substituents selected from —F, —CF.sub.3 and —OH; —C.sub.1-3alkoxy optionally substituted with 1, 2 or 3 substituents selected from —F and —CF.sub.3; —C(O)OH; —C.sub.1-3alkylene-NHC(O)C.sub.1-6alkyl; —C.sub.1-3alkylene-NHC(O)OC.sub.1-6alkyl; and C.sub.3-5cycloalkyl; or the heterocycloalkyl is optionally substituted with two substituents on the same ring carbon which together with the carbon atom to which they are attached form a 5 to 7 membered monocyclic heterocycloalkyl; and wherein when R.sup.7 is morpholinyl and R.sup.1 is unsubstituted phenyl, R.sup.2 is not —H.

[0151] In embodiments, there is provided a compound of Formula (III) or a pharmaceutically acceptable salt thereof, wherein: [0152] R.sup.1 is selected from C.sub.3-7cycloalkyl and C.sub.6-10aryl, wherein the aryl is optionally substituted with a —C.sub.2-8alkoxy substituent wherein the alkoxy is optionally substituted with 1 or 2 —CF.sub.3 substituents; [0153] R.sup.2 is —H; [0154] R.sup.5a, R.sup.5b, R.sup.5c and R.sup.5d are each —H; [0155] R.sup.7 is selected from a 5 to 7 membered monocyclic heterocycloalkyl and a 5 or 6 membered monocyclic heteroaryl, wherein the heterocycloalkyl and heteroaryl are optionally substituted with a substituent selected from —C.sub.1-3alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and —OH; and cyclopropyl.

[0156] In embodiments, there is provided a compound of Formula (IV):

##STR00022##

or a N-oxide or pharmaceutically acceptable salt thereof, wherein R.sup.1, R.sup.2, R.sup.6 and R.sup.7 are as defined for Formula (I).

[0157] In embodiments, there is provided a compound of Formula (IV), or a N-oxide or pharmaceutically acceptable salt thereof, wherein: [0158] R^{sup.1} selected from C_{sub.3-7}cycloalkyl optionally substituted with 1, 2 or 3 substituents selected from —F and —CF_{sub.3}; [0159] R^{sup.2} is selected from —H; -halo; and —C_{sub.1-3}alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and —CF_{sub.3}; [0160] R^{sup.6} is selected from —H; -halo; —NH_{sub.2}; —CN; —C_{sub.1-3}alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and CF_{sub.3}; —C_{sub.1-3}alkoxy optionally substituted with 1, 2 or 3 substituents selected from —F and —CF_{sub.3}; —C(O)O—C_{sub.1-3}alkyl; —C(O)NR^{sup.8}R^{sup.9}; —C(O)OH; and —NHC(O)—C_{sub.1-3}alkyl; [0161] R^{sup.7} is selected from —NR^{sup.10}R^{sup.11}; a 5 to 7 membered monocyclic heterocycloalkyl; and a 5 or 6 membered monocyclic heteroaryl; wherein the heterocycloalkyl and heteroaryl are optionally substituted with 1, 2 or 3 groups selected from —CN; —C_{sub.1-6}alkyl optionally substituted with 1, 2 or 3 substituents selected from —F, —CF_{sub.3} and —OH; —C_{sub.1-3}alkoxy optionally substituted with 1, 2 or 3 substituents selected from —F and —CF_{sub.3}; —C(O)OH; —C_{sub.1-3}alkylene-NHC(O)C_{sub.1-6}alkyl; —C_{sub.1-3}alkylene-NHC(O)OC_{sub.1-6}alkyl; and C_{sub.3-5}cycloalkyl; or the heterocycloalkyl is optionally substituted with two substituents on the same ring carbon which together with the carbon atom to which they are attached form a 5 to 7 membered monocyclic heterocycloalkyl; [0162] R^{sup.8} and R^{sup.9} are each independently selected from —H and —C_{sub.1-6}alkyl; [0163] R^{sup.10} is —C_{sub.1-6}alkyl; [0164] R^{sup.11} is selected from —C_{sub.1-6}alkyl optionally substituted with 1 or 2 substituents selected from —F and —C_{sub.1-3}alkoxy; and —(CH_{sub.2})_{sub.n}R^{sup.12}; [0165] R^{sup.12} is a 5 or 6 membered heteroaryl, a 3 to 5 membered cycloalkyl or a 3 to 6 membered heterocycloalkyl; [0166] n is 1, 2 or 3.

[0167] In embodiments, there is provided a compound of Formula (IV), or a N-oxide or pharmaceutically acceptable salt thereof, wherein: [0168] R^{sup.1} selected from C_{sub.3-7}cycloalkyl optionally substituted with 1, 2 or 3 substituents selected from —F and —CF_{sub.3}; [0169] R^{sup.2} is —H; [0170] R^{sup.6} is selected from —H; -halo; —NH_{sub.2}; —CN; —C_{sub.1-3}alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and CF_{sub.3}; —C_{sub.1-3}alkoxy optionally substituted with 1, 2 or 3 substituents selected from —F and —CF_{sub.3}; —C(O)O—C_{sub.1-3}alkyl; —C(O)NR^{sup.8}R^{sup.9}; —C(O)OH; and —NHC(O)—C_{sub.1-3}alkyl; [0171] R^{sup.7} is selected from a 5 to 7 membered monocyclic heterocycloalkyl optionally substituted with 1, 2 or 3 groups selected from —C_{sub.1-6}alkyl optionally substituted with 1, 2 or 3 substituents selected from —F, —CF_{sub.3} and —OH; and C_{sub.3-5}cycloalkyl; [0172] R^{sup.8} and R^{sup.9} are each independently selected from —H and —C_{sub.1-6}alkyl.

[0173] The following embodiments of moieties R^{sup.1}, R^{sup.2}, R^{sup.3}, R^{sup.4a}, R^{sup.4b}, R^{sup.4c}, R^{sup.4d}, R^{sup.5}, R^{sup.6a}, R^{sup.6b}, R^{sup.7}, R^{sup.8}, R^{sup.9}, R^{sup.10}, R^{sup.11}, R^{sup.12}, m and n may be applied, alone or in combination, to the description of the compounds of Formula (I) provided herein. The following embodiments of moieties R^{sup.1}, R^{sup.2}, R^{sup.3}, R^{sup.4a}, R^{sup.4b}, R^{sup.7}, R^{sup.8}, R^{sup.9}, R^{sup.10}, R^{sup.11}, R^{sup.12}, m and n may be applied, alone or in combination, to the description of the compounds of Formula (II) provided herein. The following embodiments of moieties R^{sup.1}, R^{sup.2}, R^{sup.5a}, R^{sup.5b}, R^{sup.5c}, R^{sup.5d}, R^{sup.7}, R^{sup.8}, R^{sup.9}, R^{sup.10}, R^{sup.11}, R^{sup.12}, m and n may be applied, alone or in combination, to the description of the compounds of Formula (III) provided herein. The following embodiments of moieties R^{sup.1}, R^{sup.2}, R^{sup.6}, R^{sup.7}, R^{sup.8}, R^{sup.9}, R^{sup.10}, R^{sup.11}, R^{sup.12}, m and n may be applied, alone or in combination, to the descriptions of the compounds of Formula (IV) provided herein.

[0174] R^{sup.1} is selected from C_{sub.2-6}alkyl; C_{sub.2-6}alkenyl; C_{sub.2-6}alkynyl; C_{sub.2-6}alkoxy; C_{sub.2-6}alkenyloxy; C_{sub.2-6}alkynyloxy; C_{sub.3-7}cycloalkyl; —O—C_{sub.3-7}cycloalkyl; C_{sub.6-10}aryl; —O—(CH_{sub.2})_{sub.m}—C_{sub.6-10}aryl; 6 membered heteroaryl; and thiophenyl; wherein alkyl, alkenyl, alkynyl, alkoxy, alkenyloxy, alkynyloxy and cycloalkyl are optionally substituted with 1, 2 or 3 substituents selected from —F and —CF_{sub.3} and wherein

aryl and heteroaryl are optionally substituted with 1 or 2 substituents selected from -halo, —C.sub.1-3alkyl, —C.sub.1-8alkoxy and —C.sub.2-8alkynyloxy wherein —C.sub.1-3alkyl, —C.sub.1-8alkoxy and —C.sub.2-8alkynyloxy are optionally substituted with 1, 2, or 3 substituents selected from —F, —CF.sub.3 and —NHC(O)O—C.sub.1-6alkyl or two substituents together with the carbon to which they are attached form diazirinyl.

[0175] In embodiments, R.sup.1 is selected from C.sub.2-6alkyl; C.sub.2-6alkenyl; C.sub.2-6alkynyl; C.sub.2-6alkoxy; C.sub.2-6alkenyloxy; C.sub.2-6alkynyloxy; C.sub.3-7cycloalkyl; —O—C.sub.3-7cycloalkyl; C.sub.6-10aryl; —O—(CH.sub.2).sub.m—C.sub.6-10aryl; 6 membered heteroaryl; and thiophenyl; wherein alkyl, alkenyl, alkynyl, alkoxy, alkenyloxy, alkynyloxy and cycloalkyl are optionally substituted with 1, 2 or 3 substituents selected from —F and —CF.sub.3 and wherein —O—(CH.sub.2).sub.m—C.sub.6-10aryl and heteroaryl are optionally substituted with 1 or 2 substituents selected from -halo, —C.sub.1-3alkyl, —C.sub.1-8alkoxy and —C.sub.2-8alkynyloxy wherein —C.sub.1-3alkyl, —C.sub.1-8alkoxy and —C.sub.2-8alkynyloxy are optionally substituted with 1, 2, or 3 substituents selected from —F, —CF.sub.3 and —NHC(O)O—C.sub.1-6alkyl or two substituents together with the carbon to which they are attached form diazirinyl; and C.sub.6-10aryl is substituted with 1 or 2 substituents selected from -halo, —C.sub.1-3alkyl, —C.sub.1-8alkoxy and —C.sub.2-8alkynyloxy wherein —C.sub.1-3alkyl, —C.sub.1-8alkoxy and —C.sub.2-8alkynyloxy are optionally substituted with 1, 2, or 3 substituents selected from —F, —CF.sub.3 and —NHC(O)O—C.sub.1-6alkyl or two substituents together with the carbon to which they are attached form diazirinyl

[0176] In embodiments, R.sup.1 is selected from C.sub.2-6alkyl; C.sub.2-6alkoxy; C.sub.3-7cycloalkyl; —O—C.sub.3-7cycloalkyl; C.sub.6-10aryl; —O—(CH.sub.2).sub.m—C.sub.6-10aryl and thiophenyl; wherein alkyl, alkoxy and cycloalkyl are optionally substituted with 1, 2 or 3 substituents selected from —F and —CF.sub.3 and wherein aryl is optionally substituted with 1 or 2 substituents selected from -halo, —C.sub.1-3alkyl, —C.sub.1-8alkoxy and —C.sub.2-8alkynyloxy wherein —C.sub.1-3alkyl, —C.sub.1-8alkoxy and —C.sub.2-8alkynyloxy are optionally substituted with 1, 2, or 3 substituents selected from —F, —CF.sub.3 and —NHC(O)O—C.sub.1-6alkyl or two substituents together with the carbon to which they are attached form diazirinyl.

[0177] In embodiments, R.sup.1 is selected from C.sub.2-6alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and —CF.sub.3; C.sub.2-4alkoxy; C.sub.4-6cycloalkyl; —O—C.sub.4-6cycloalkyl; phenyl; —O—(CH.sub.2).sub.m-phenyl; and thiophenyl; wherein phenyl is optionally substituted with 1 or 2 substituents selected from -halo, —C.sub.1-3alkyl, —C.sub.1-8alkoxy and —C.sub.2-8alkynyloxy wherein —C.sub.1-3alkyl, —C.sub.1-8alkoxy and —C.sub.2-8alkynyloxy are optionally substituted with 1, 2, or 3 substituents selected from —F, —CF.sub.3 and —NHC(O)O—C.sub.1-6alkyl or two substituents together with the carbon to which they are attached form diazirinyl.

[0178] In embodiments, R.sup.1 is selected from C.sub.2-6alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and —CF.sub.3; C.sub.2-4alkoxy; C.sub.4-6cycloalkyl; —O—C.sub.4-6cycloalkyl; phenyl; —O—(CH.sub.2).sub.m-phenyl; and thiophenyl; wherein —O—(CH.sub.2).sub.m-phenyl is optionally substituted with 1 or 2 substituents selected from -halo, —C.sub.1-3alkyl, —C.sub.1-8alkoxy and —C.sub.2-8alkynyloxy wherein —C.sub.1-3alkyl, —C.sub.1-8alkoxy and —C.sub.2-8alkynyloxy are optionally substituted with 1, 2, or 3 substituents selected from —F, —CF.sub.3 and —NHC(O)O—C.sub.1-6alkyl or two substituents together with the carbon to which they are attached form diazirinyl; and phenyl is substituted with 1 or 2 substituents selected from -halo, —C.sub.1-3alkyl, —C.sub.1-8alkoxy and —C.sub.2-8alkynyloxy wherein —C.sub.1-3alkyl, —C.sub.1-8alkoxy and —C.sub.2-8alkynyloxy are optionally substituted with 1, 2, or 3 substituents selected from —F, —CF.sub.3 and —NHC(O)O—C.sub.1-6alkyl or two substituents together with the carbon to which they are attached form diazirinyl.

[0179] In embodiments, R.sup.1 is selected from —CF.sub.2CF.sub.3; propyl; butyl; pentyl;

propoxy; cyclobutyl; cyclohexyl; —O-cyclopentyl; thiophenyl; phenyl; —O-phenyl; —O—CH.sub.2-phenyl; wherein phenyl is optionally substituted with 1 or 2 substituents selected from —F, —Cl, —CH.sub.3, —O—(CH.sub.2).sub.5C≡CH, —O—(CH.sub.2).sub.7, —O—(CH.sub.2).sub.2C(N=N)(CH.sub.2).sub.2C≡CH, —O—(CH.sub.2).sub.2NHC(O)OC(CH.sub.3).sub.3, —O—CH.sub.2C≡CH, —O—(CH.sub.2).sub.5CF.sub.3 and —O—(CH.sub.2).sub.7.

[0180] In embodiments, R.sup.1 is selected from —CF.sub.2CF.sub.3; propyl; butyl; pentyl; propoxy; cyclobutyl; cyclohexyl; —O-cyclopentyl; thiophenyl; phenyl substituted with 1 or 2 substituents selected from —F, —Cl, —CH.sub.3, —O—(CH.sub.2).sub.5C≡CH, —O—(CH.sub.2).sub.7, —O—(CH.sub.2).sub.2C(N=N)(CH.sub.2).sub.2C≡CH, —O—(CH.sub.2).sub.2NHC(O)OC(CH.sub.3).sub.3, —O—CH.sub.2C≡CH, —O—(CH.sub.2).sub.5CF.sub.3 and —O—(CH.sub.2).sub.7; —O-phenyl; —O—CH.sub.2-phenyl; wherein —O-phenyl and —O—CH.sub.2-phenyl is optionally substituted with 1 or 2 substituents selected from —F, —Cl, —CH.sub.3, —O—(CH.sub.2).sub.5C≡CH, —O—(CH.sub.2).sub.7, —O—(CH.sub.2).sub.2C(N=N)(CH.sub.2).sub.2C≡CH, —O—(CH.sub.2).sub.2NHC(O)OC(CH.sub.3).sub.3, —O—CH.sub.2C≡CH, —O—(CH.sub.2).sub.5CF.sub.3 and —O—(CH.sub.2).sub.7.

[0181] In embodiments, R.sup.1 is cyclohexyl. In another embodiment, R.sup.1 is phenyl substituted with —F, —Cl, —CH.sub.3, —O—(CH.sub.2).sub.5C≡CH, —O—(CH.sub.2).sub.7, —O—(CH.sub.2).sub.2C(N=N)(CH.sub.2).sub.2C≡CH, —O—(CH.sub.2).sub.2NHC(O)OC(CH.sub.3).sub.3, —O—CH.sub.2C≡CH, —O—(CH.sub.2).sub.5CF.sub.3 and —O—(CH.sub.2).sub.7. In embodiments, R.sup.1 is phenyl.

[0182] R.sup.2 is selected from —H, -halo and —C.sub.1-3alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and —CF.sub.3. In embodiments, R.sup.2 is —H. In embodiments, R.sup.2 is -halo. In embodiments, R.sup.2 is —F. In embodiments, R.sup.2 is —C.sub.1-3alkyl. In embodiments, R.sup.2 is methyl.

[0183] R.sup.3 is selected from —H; —C.sub.1-6alkyl; —C.sub.2-6alkenyl; —C.sub.2-6alkynyl; —C.sub.3-7cycloalkyl; and a 5 or 6 membered heterocycloalkyl; wherein the alkyl, alkenyl, alkynyl, cycloalkyl or heterocycloalkyl are optionally substituted by 1, 2 or 3 groups, for example 1 or 2 groups, selected from —C.sub.1-3alkyl optionally substituted with 1, 2 or 3 substituents selected from —F, —CF.sub.3, —C(O)NR.sup.8R.sup.9 and —NR.sup.8R.sup.9.

[0184] In embodiments, R.sup.3 is selected from —H; —C.sub.2-4alkynyl; —C.sub.1-3alkyl optionally substituted with —C(O)NR.sup.8R.sup.9 or —NR.sup.8R.sup.9; and a 5 or 6 membered heterocycloalkyl optionally substituted with C.sub.1-3alkyl.

[0185] In embodiments, R.sup.3 is selected from —H; —C.sub.2-4alkynyl; —C.sub.1-3alkyl optionally substituted with —C(O)NR.sup.8R.sup.9 or —NR.sup.8R.sup.9; and a 5 or 6 membered nitrogen containing heterocycloalkyl optionally substituted with C.sub.1-3alkyl.

[0186] In embodiments, R.sup.3 is selected from —H; —C.sub.2-4alkynyl; —C.sub.1-3alkyl optionally substituted with —C(O)NR.sup.8R.sup.9 or —NR.sup.8R.sup.9; and piperidinyl optionally substituted with C.sub.1-3alkyl.

[0187] In embodiments, R.sup.3 is selected from methyl, ethyl, i-propyl, —(CH.sub.2).sub.2N(CH.sub.3).sub.2, —(CH.sub.2).sub.3N(CH.sub.3).sub.2, —CH.sub.2C≡CH, —CH.sub.2C(O)N(CH.sub.3).sub.2 and N-methylpiperidine. In embodiments, R.sup.3 is selected from ethyl, i-propyl, —(CH.sub.2).sub.2N(CH.sub.3).sub.2, —(CH.sub.2).sub.3N(CH.sub.3).sub.2, —CH.sub.2C≡CH, —CH.sub.2C(O)N(CH.sub.3).sub.2 and N-methylpiperidine.

[0188] In embodiments, R.sup.3 is selected from —C.sub.2-4alkynyl and —C.sub.1-3alkyl optionally substituted with —NR.sup.8R.sup.9.

[0189] In embodiments, R.sup.3 is selected from ethyl, i-propyl, —(CH.sub.2).sub.2N(CH.sub.3).sub.2, —(CH.sub.2).sub.3N(CH.sub.3).sub.2 and —CH.sub.2C≡CH.

[0190] In embodiments, R.sup.3 is i-propyl.

[0191] R.sup.4a and R.sup.4b are each independently selected from —H and —C.sub.1-3alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and —CF.sub.3. In embodiments, R.sup.4a is methyl and R.sup.4b is —H. In embodiments, R.sup.4a and R.sup.4b are both —H.

[0192] R.sup.4c and R.sup.4d are each independently selected from —H and C.sub.1-3 alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and —CF.sub.3; or R.sup.4c and R.sup.4d together with the carbon to which they are attached represent carbonyl. In embodiments, R.sup.4c and R.sup.4d together with the carbon to which they are attached represent carbonyl. In embodiments, R.sup.4c and R.sup.4d are each independently selected from —H and C.sub.1-3 alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and —CF.sub.3. In embodiments, R.sup.4c and R.sup.4d are both —H or together with the carbon to which they are attached represent carbonyl. In embodiments, R.sup.4c and R.sup.4d are both —H.

[0193] R.sup.5a, R.sup.5b, R.sup.5c and R.sup.5d are each independently selected from —H and —C.sub.1-3 alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and —CF.sub.3. In embodiments, R.sup.5, R.sup.5b, R.sup.5c and R.sup.5d are each independently selected from —H and —C.sub.1-3 alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and —CF.sub.3. In embodiments, R.sup.5a, R.sup.5b, R.sup.5c and R.sup.5d are each independently selected from —H and —C.sub.1-3 alkyl. In embodiments, R.sup.5a is methyl and R.sup.5b, R.sup.5c and R.sup.5d are each —H. In embodiments, R.sup.5a, R.sup.5b and R.sup.5c are each —H and R.sup.5d is methyl. In embodiments, R.sup.5a, R.sup.5b, R.sup.5c and R.sup.5d each represent —H.

[0194] R.sup.6 is selected from —H; -halo; —NH.sub.2; —CN; —C.sub.1-3alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and CF.sub.3; —C.sub.1-3alkoxy optionally substituted with 1, 2 or 3 substituents selected from —F and —CF.sub.3; —C(O)O—C.sub.1-3alkyl; —C(O)NR.sup.8R.sup.9; —C(O)OH; and —NHC(O)—C.sub.1-3alkyl. In embodiments, R.sup.6 is selected from —H; —Br; —NH.sub.2; —CN; methoxy; ethyl; —C(O)OCH.sub.3; —C(O)NH.sub.2; —C(O)OH; and —NHC(O)CH.sub.3.

[0195] R.sup.7 is selected from —NR.sup.10R.sup.11; a 5 to 7 membered monocyclic heterocycloalkyl; and a 5 or 6 membered monocyclic heteroaryl; wherein the heterocycloalkyl and heteroaryl are optionally substituted with 1, 2 or 3 (for example, 1 or 2) groups selected from —CN; —C.sub.1-6alkyl optionally substituted with 1, 2 or 3 substituents selected from —F, —CF.sub.3 and —OH; —C.sub.1-3alkoxy optionally substituted with 1, 2 or 3 substituents selected from —F and —CF.sub.3; —C(O)OH; —C.sub.1-3alkylene-NHC(O)C.sub.1-6alkyl; —C.sub.1-3alkylene-NHC(O)OC.sub.1-6alkyl; and C.sub.3-5cycloalkyl; or the heterocycloalkyl is optionally substituted with two substituents on the same ring carbon which together with the carbon atom to which they are attached form a 5 to 7 membered monocyclic heterocycloalkyl.

[0196] In embodiments, R.sup.7 is selected from NR.sup.10R.sup.11; a 5 to 7 membered monocyclic heterocycloalkyl selected from morpholinyl, thiazolidinyl, tetrahydropyranyl, pyrrolyl, thiomorpholinyl and 3,4-dihydro-2H-pyranyl; a 5 or 6 membered monocyclic heteroaryl selected from pyridinyl, dihydropyranyl, oxazolyl, imidazolyl and thiazolyl; wherein the heterocycloalkyl and heteroaryl are optionally substituted with 1, 2 or 3 (for example, 1 or 2) groups selected from —CN; —C.sub.1-6alkyl optionally substituted with 1, 2 or 3 substituents selected from —F, —CF.sub.3 and —OH; —C.sub.1-3alkoxy optionally substituted with 1, 2 or 3 substituents selected from —F and —CF.sub.3; —C(O)OH; —C.sub.1-3alkylene-NHC(O)C.sub.1-6alkyl; —C.sub.1-3alkylene-NHC(O)OC.sub.1-6alkyl; and C.sub.3-5cycloalkyl; or the heterocycloalkyl is optionally substituted with two substituents on the same ring carbon which together with the carbon atom to which they are attached form a 5 to 7 membered monocyclic heterocycloalkyl.

[0197] In embodiments, R.sup.7 is selected from NR.sup.10R.sup.11; a 5 to 7 membered monocyclic heterocycloalkyl selected from morpholinyl, thiazolidinyl, tetrahydropyranyl, pyrrolyl,

thiomorpholinyl and 3,4-dihydro-2H-pyranyl; a 5 or 6 membered monocyclic heteroaryl selected from pyridinyl, dihydropyranyl, oxazolyl, imidazolyl and thiazolyl; wherein the heterocycloalkyl and heteroaryl are optionally substituted with 1, 2 or 3 (for example, 1 or 2) groups selected from —CN, methyl, ethyl, propyl, cyclopropyl, methoxy, —CH₂CF₃, —CH₂OH, —CH₂CH₂OH, —C(O)OH, —(CH₂)₂NHC(O)CH₃ and —CH₂NHC(O)OC(CH₃)₃; or the heterocycloalkyl is optionally substituted with two substituents on the same ring carbon which together with the carbon atom to which they are attached form a 6 membered monocyclic heterocycloalkyl.

[0198] In embodiments, R⁷ is selected from NR¹⁰R¹¹ wherein R¹⁰ is selected from methyl, ethyl or propyl and R¹¹ is selected from ethyl, propyl, CH₂CHF₂, CH₂CH₂OCH₂CH₃ and —(CH₂)_pR¹²; a 5 to 7 membered monocyclic heterocycloalkyl selected from morpholinyl, thiazolidinyl, tetrahydropyranyl, pyrrolyl, thiomorpholinyl and 3,4-dihydro-2H-pyranyl; a 5 or 6 membered monocyclic heteroaryl selected from pyridinyl, dihydropyranyl, oxazolyl, imidazolyl and thiazolyl; wherein the heterocycloalkyl and heteroaryl are optionally substituted with 1 or 2 groups selected from —CN, methyl, ethyl, propyl, cyclopropyl, methoxy, —CH₂CF₃, —CH₂OH, —CH₂CH₂OH, —C(O)OH, —(CH₂)₂NHC(O)CH₃ and —CH₂NHC(O)OC(CH₃)₃; or the heterocycloalkyl is optionally substituted with two substituents on the same ring carbon which together with the carbon atom to which they are attached form tetrahydropyranyl.

[0199] In embodiments, R⁷ is selected from NR¹⁰R¹¹ wherein R¹⁰ is selected from methyl, ethyl or propyl and R¹¹ is selected from ethyl, propyl, CH₂CHF₂, CH₂CH₂OCH₂CH₃ and —(CH₂)_nR¹².

[0200] In embodiments, R⁷ is selected from NR¹⁰R¹¹ wherein R¹⁰ is selected from methyl, ethyl or propyl; R¹¹ is selected from ethyl, propyl, CH₂CHF₂, CH₂CH₂OCH₂CH₃ and —(CH₂)_nR¹²; n is 1 or 2; and R¹² is selected from isoxazolyl, oxadiazolyl, cyclopropyl, pyrazinyl, tetrahydrofuranlyl and pyridinyl.

[0201] In embodiments, R⁷ is selected from a 5 to 7 membered monocyclic heterocycloalkyl optionally substituted with 1, 2 or 3 (for example, 1 or 2) groups selected from —CN; —C₁₋₆alkyl optionally substituted with 1, 2 or 3 substituents selected from —F, —CF₃ and —OH; —C₁₋₃alkoxy optionally substituted with 1, 2 or 3 substituents selected from —F and —CF₃; —C(O)OH; —C₁₋₃alkylene-NHC(O)C₁₋₆alkyl; —C₁₋₃alkylene-NHC(O)OC₁₋₆alkyl and C₃₋₅cycloalkyl; or the heterocycloalkyl is optionally substituted with two substituents on the same ring carbon which together with the carbon atom to which they are attached form a 5 to 7 membered monocyclic heterocycloalkyl.

[0202] In embodiments, R⁷ is a 5 to 7 membered monocyclic heterocycloalkyl selected from morpholinyl, thiazolidinyl, tetrahydropyranyl, pyrrolyl, thiomorpholinyl and 3,4-dihydro-2H-pyranyl wherein the heterocycloalkyl is optionally substituted with 1 or 2 groups selected from —CN; —C₁₋₆alkyl optionally substituted with 1, 2 or 3 substituents selected from —F, —CF₃ and —OH; —C₁₋₃alkoxy optionally substituted with 1, 2 or 3 substituents selected from —F and —CF₃; —C(O)OH; —CH₂NHC(O)CH₃; —CH₂NHC(O)OC(CH₃)₃; and C₃₋₅cycloalkyl; or the heterocycloalkyl is optionally substituted with two substituents on the same ring carbon which together with the carbon atom to which they are attached form a 6 membered monocyclic heterocycloalkyl.

[0203] In embodiments, R⁷ is a 5 to 7 membered monocyclic heterocycloalkyl optionally substituted with 1 or 2 substituents selected from methyl, ethyl, propyl, cyclopropyl, —CH₂CH₂OH, —CH₂OH, —C(O)OH, —CH₂CF₃, and —CH₂NHC(O)OC(CH₃)₃; or the heterocycloalkyl is optionally substituted with two substituents on the same ring carbon which together with the carbon atom to which they are attached form tetrahydropyran.

[0204] In embodiments, R⁷ is morpholinyl optionally substituted with 1 or 2 substituents

selected from methyl, ethyl, propyl, cyclopropyl, —CH.sub.2CH.sub.2OH, —CH.sub.2OH, —C(O)OH, —CH.sub.2CF.sub.3, and —CH.sub.2NHC(O)OC(CH.sub.3).sub.3; or optionally substituted with two substituents on the same ring carbon which together with the carbon atom to which they are attached form tetrahydropyran (i.e. R.sup.7 becomes a spirocyclic group).

[0205] In embodiments, R.sup.7 is 2-methylmorpholin-4-yl.

[0206] R.sup.8 is selected from —H and —C.sub.1-6alkyl. In embodiments, R.sup.8 is selected from —H and —C.sub.1-3alkyl. In embodiments, R.sup.8 is —H. In embodiments, R.sup.8 is —C.sub.1-3alkyl. In embodiments, R.sup.8 is methyl.

[0207] R.sup.9 is selected from —H and —C.sub.1-6alkyl. In embodiments, R.sup.9 is selected from —H and —C.sub.1-3alkyl. In embodiments, R.sup.9 is —H. In embodiments, R.sup.9 is —C.sub.1-3alkyl. In embodiments, R.sup.9 is methyl.

[0208] R.sup.10 is —C.sub.1-6alkyl. In embodiments, R.sup.10 is —C.sub.1-3alkyl. In embodiments, R.sup.10 is methyl. In another embodiment, R.sup.10 is ethyl. In embodiments, R.sup.10 is propyl.

[0209] R.sup.11 is selected from —C.sub.1-6alkyl optionally substituted with 1 or 2 substituents selected from —F and —C.sub.1-3alkoxy; or —(CH.sub.2).sub.nR.sup.12. In embodiments, R.sup.11 is selected from —C.sub.1-6alkyl optionally substituted with 1 or 2 substituents selected from —F and ethoxy. In embodiments, R.sup.11 is selected from ethyl, propyl, CH.sub.2CHF.sub.2, CH.sub.2CH.sub.2OCH.sub.2CH.sub.3 and —(CH.sub.2).sub.nR.sup.12. In embodiments, R.sup.11 is selected from —(CH.sub.2).sub.nR.sup.12.

[0210] R.sup.12 is selected from a 5 or 6 membered heteroaryl, a 3 to 5 membered cycloalkyl or a 3 to 6 membered heterocycloalkyl. In embodiments, R.sup.12 is selected from isoxazolyl, oxadiazolyl, cyclopropyl, pyrazinyl, tetrahydrofuranyl and pyridinyl.

[0211] m is 0 or 1. In embodiments, m is 0. In embodiments, m is 1.

[0212] n is 1, 2 or 3. In embodiments, n is 1 or 2. In embodiments, n is 1. In embodiments, n is 2. In embodiments, n is 3.

[0213] In embodiments, the compound of Formula (I) is selected from: [0214] 2-(diethylamino)-6-(propan-2-yl)-4-{{4-(propan-2-yl)phenyl}amino}-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0215] 4-[(4-cyclohexylphenyl)amino]-2-(2-cyclopropylmorpholin-4-yl)-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0216] 6-(propan-2-yl)-4-{{4-(propan-2-yl)phenyl}amino}-2-(1,3-thiazolidin-3-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0217] 2-[(2R,6S)-2,6-dimethylmorpholin-4-yl]-6-(propan-2-yl)-4-{{4-(propan-2-yl)phenyl}amino}-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0218] 6-(propan-2-yl)-4-{{4-(propan-2-yl)phenyl}amino}-2-(thiomorpholin-4-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0219] 2-[(2S)-2-methylmorpholin-4-yl]-6-(propan-2-yl)-4-{{4-(propan-2-yl)phenyl}amino}-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0220] 2-[(2R)-2-methylmorpholin-4-yl]-6-(propan-2-yl)-4-{{4-(propan-2-yl)phenyl}amino}-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0221] 2-[(2S,6S)-2,6-dimethylmorpholin-4-yl]-6-(propan-2-yl)-4-{{4-(propan-2-yl)phenyl}amino}-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0222] 2-(3-methylmorpholin-4-yl)-6-(propan-2-yl)-4-{{4-(propan-2-yl)phenyl}amino}-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0223] 2-(2-cyclopropylmorpholin-4-yl)-6-(propan-2-yl)-4-{{4-(propan-2-yl)phenyl}amino}-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0224] 4-[(4-cyclohexylphenyl)amino]-2-(morpholin-4-yl)-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0225] 4-[(4-cyclohexylphenyl)amino]-2-(2-methylmorpholin-4-yl)-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0226] 4-[(4-cyclohexylphenyl)amino]-2-[(2R)-2-methylmorpholin-4-yl]-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0227] 4-[(4-cyclohexylphenyl)amino]-2-[(2R)-cyclopropylmorpholin-4-yl]-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0228] 4-[(4-cyclohexylphenyl)amino]-2-[(2S)-cyclopropylmorpholin-4-yl]-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0229] 4-[(4-cyclohexylphenyl)amino]-6-(propan-2-yl)-2-[2-(2,2,2-trifluoroethyl)morpholin-4-

yl]-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0230] tert-butyl {[(2R)-4-{4-[(4-cyclohexylphenyl)amino]-7-oxo-6-(propan-2-yl)-6,7-dihydro-5H-pyrrolo[3,4-d]pyrimidin-2-yl}morpholin-2-yl)methyl}carbamate; [0231] 4-[(4-cyclohexylphenyl)amino]-6-(propan-2-yl)-2-[2-(propan-2-yl)morpholin-4-yl]-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0232] 4-[(4-cyclohexylphenyl)amino]-6-(propan-2-yl)-2-(1,3-thiazolidin-3-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0233] 4-[(4-cyclohexylphenyl)amino]-2-[(2-ethoxyethyl)(methyl)amino]-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0234] 4-[(4-cyclohexylphenyl)amino]-2-(2-ethylmorpholin-4-yl)-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0235] 4-[(4-cyclohexylphenyl)amino]-2-{methyl[(1,2-oxazol-3-yl)methyl]amino}-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0236] 4-[(4-cyclohexylphenyl)amino]-2-{methyl[2-(1,2,4-oxadiazol-3-yl)ethyl]amino}-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0237] 4-[(4-cyclohexylphenyl)amino]-2-(1,4-oxazepan-4-yl)-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0238] 4-[(4-cyclohexylphenyl)amino]-2-(1,9-dioxa-4-azaspiro[5.5]undecan-4-yl)-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0239] 4-[(4-cyclohexylphenyl)amino]-2-(3-methoxypyrrolidin-1-yl)-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0240] 4-[(4-cyclohexylphenyl)amino]-2-[2-(2-hydroxyethyl)morpholin-4-yl]-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0241] 4-[(4-cyclohexylphenyl)amino]-2-(dipropylamino)-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0242] 4-[(4-cyclohexylphenyl)amino]-2-[(cyclopropylmethyl)(methyl)amino]-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0243] 4-[(4-cyclohexylphenyl)amino]-2-[2-(hydroxymethyl)morpholin-4-yl]-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0244] 4-[(4-cyclohexylphenyl)amino]-2-[3-(hydroxymethyl)morpholin-4-yl]-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0245] 4-[(4-cyclohexylphenyl)amino]-2-{methyl[(pyrazin-2-yl)methyl]amino}-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0246] 4-[(4-cyclohexylphenyl)amino]-2-(diethylamino)-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0247] 4-[(4-cyclohexylphenyl)amino]-2-{methyl[(oxolan-2-yl)methyl]amino}-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0248] 4-[(4-cyclohexylphenyl)amino]-2-[(2,2-difluoroethyl)(methyl)amino]-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0249] 4-[(4-cyclohexylphenyl)amino]-2-{methyl[2-(pyridin-2-yl)ethyl]amino}-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0250] (3S)-4-{4-[(4-cyclohexylphenyl)amino]-7-oxo-6-(propan-2-yl)-6,7-dihydro-5H-pyrrolo[3,4-d]pyrimidin-2-yl}morpholine-3-carboxylic acid; [0251] N-[2-(4-{4-[(4-cyclohexylphenyl)amino]-7-oxo-6-(propan-2-yl)-6,7-dihydro-5H-pyrrolo[3,4-d]pyrimidin-2-yl}morpholin-2-yl)ethyl]acetamide; [0252] 6-(propan-2-yl)-4-{[4-(propan-2-yl)phenyl]amino}-2-(pyridin-4-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0253] 6-isopropyl-4-((4-isopropylphenyl)amino)-2-(pyridin-4-yl)-5,6-dihydro-7H-pyrrolo [3,4-d]pyrimidin-7-one (Compound B); [0254] 4-{4-[(4-cyclohexylphenyl)amino]-7-oxo-6-(propan-2-yl)-6,7-dihydro-5H-pyrrolo[3,4-d]pyrimidin-2-yl}pyridine-2-carbonitrile; [0255] 4-[(4-cyclohexylphenyl)amino]-2-(2-cyclopropylpyridin-4-yl)-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0256] 4-[(4-cyclohexylphenyl)amino]-2-(2-methoxypyridin-4-yl)-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0257] 4-[(4-cyclohexylphenyl)amino]-2-(2-methylpyridin-4-yl)-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one (Compound C); [0258] 4-[(4-cyclohexylphenyl)amino]-2-(3,6-dihydro-2H-pyran-4-yl)-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0259] 4-[(4-cyclohexylphenyl)amino]-6-(propan-2-yl)-2-(pyridin-4-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0260] 4-[(4-cyclohexylphenyl)amino]-2-(1-methyl-1H-pyrazol-4-yl)-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0261] 4-[(4-cyclohexylphenyl)amino]-2-(1,3-oxazol-5-yl)-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0262] 4-[(4-cyclohexylphenyl)amino]-6-(propan-2-yl)-2-(1,3-thiazol-5-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0263] 2-(3,6-dihydro-2H-pyran-4-yl)-6-

(propan-2-yl)-4-{{[4-(propan-2-yl)phenyl]amino}}-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0264] 4-{{[4-(4-fluorophenoxy)phenyl]amino}}-2-[(2R)-2-methylmorpholin-4-yl]-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0265] 2-(2-cyclopropylmorpholin-4-yl)-4-({4'-[(hept-6-yn-1-yl)oxy][1,1'-biphenyl]-4-yl}amino)-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0266] 2-(2-cyclopropylmorpholin-4-yl)-4-{{[4'-(heptyloxy)[1,1'-biphenyl]-4-yl]amino}}-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0267] 4-[(4'-{2-[3-(but-3-yn-1-yl)-3H-diaziren-3-yl]ethoxy}[1,1'-biphenyl]-4-yl)amino]-2-(2-cyclopropylmorpholin-4-yl)-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0268] 2-[(2R)-2-methylmorpholin-4-yl]-4-[(4-pentylphenyl)amino]-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0269] 4-{{[4-(butan-2-yl)phenyl]amino}}-2-[(2R)-2-methylmorpholin-4-yl]-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0270] 4-{{[4-(benzyloxy)phenyl]amino}}-2-[(2R)-2-methylmorpholin-4-yl]-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0271] 2-(2-cyclopropylmorpholin-4-yl)-4-{{[4-(pentafluoroethyl)phenyl]amino}}-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0272] 2-(2-cyclopropylmorpholin-4-yl)-6-(propan-2-yl)-4-[(4-propylphenyl)amino]-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0273] 2-[(2R)-2-methylmorpholin-4-yl]-6-(propan-2-yl)-4-[(4-propylphenyl)amino]-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0274] 2-[(2R)-2-methylmorpholin-4-yl]-4-{{[4-(pentafluoroethyl)phenyl]amino}}-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0275] 2-(2-cyclopropylmorpholin-4-yl)-6-(propan-2-yl)-4-({4-[(propan-2-yl)oxy]phenyl}amino)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0276] 4-[(4-cyclobutylphenyl)amino]-2-(morpholin-4-yl)-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0277] 4-{{[4-(cyclopentyloxy)phenyl]amino}}-2-[(2R)-2-methylmorpholin-4-yl]-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0278] 2-[(2R)-2-methylmorpholin-4-yl]-6-(propan-2-yl)-4-{{[4-(2,2,2-trifluoroethyl)phenyl]amino}}-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0279] tert-butyl {2-[(4'-{2-(2-cyclopropylmorpholin-4-yl)-7-oxo-6-(propan-2-yl)-6,7-dihydro-5H-pyrrolo[3,4-d]pyrimidin-4-yl]amino}[1,1'-biphenyl]-4-yl)oxy}ethyl} carbamate; [0280] 6-ethyl-2-[(2R)-2-methylmorpholin-4-yl]-4-{{[4-(propan-2-yl)phenyl]amino}}-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0281] 4-[(4-cyclohexylphenyl)amino]-6-ethyl-2-[(2R)-2-methylmorpholin-4-yl]-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0282] tert-butyl {2-[(4'-{2-(morpholin-4-yl)-7-oxo-6-(propan-2-yl)-6,7-dihydro-5H-pyrrolo[3,4-d]pyrimidin-4-yl]amino}[1,1'-biphenyl]-4-yl)oxy}ethyl} carbamate; [0283] 4-[(4'-{2-[3-(but-3-yn-1-yl)-3H-diaziren-3-yl]ethoxy}[1,1'-biphenyl]-4-yl)amino]-2-(morpholin-4-yl)-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0284] 2-(morpholin-4-yl)-6-(propan-2-yl)-4-({4'-[(prop-2-yn-1-yl)oxy][1,1'-biphenyl]-4-yl}amino)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0285] 4-[(4-cyclohexylphenyl)amino]-6-[3-(dimethylamino)propyl]-2-[(2R)-2-methylmorpholin-4-yl]-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0286] 4-[(4-cyclohexylphenyl)amino]-6-[2-(dimethylamino)ethyl]-2-[(2R)-2-methylmorpholin-4-yl]-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0287] 4-[(4-cyclobutylphenyl)amino]-6-[3-(dimethylamino)propyl]-2-[(2R)-2-methylmorpholin-4-yl]-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0288] 4-[(4-cyclobutylphenyl)amino]-6-[2-(dimethylamino)ethyl]-2-[(2R)-2-methylmorpholin-4-yl]-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0289] 2-(morpholin-4-yl)-4-{{[4-(propan-2-yl)phenyl]amino}}-6-(prop-2-yn-1-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0290] 4-[(4-cyclohexylphenyl)amino]-2-(oxan-4-yl)-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0291] 4-[(4-cyclohexylphenyl)amino]-2-(1H-imidazol-1-yl)-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0292] 2-(3,6-dihydro-2H-pyran-4-yl)-4-[(2'-methyl[1,1'-biphenyl]-4-yl)amino]-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0293] 4-[(4'-{2-[3-(but-3-yn-1-yl)-3H-diaziren-3-yl]ethoxy}[1,1'-biphenyl]-4-yl)amino]-2-(3,6-dihydro-2H-pyran-4-yl)-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0294] 2-(3,6-dihydro-2H-pyran-4-yl)-4-[(2-fluoro[1,1'-biphenyl]-4-yl)amino]-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-

d]pyrimidin-7-one; [0295] 2-(morpholin-4-yl)-4-{{[4-(pentafluoroethyl)phenyl]amino}-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0296] 4-[(2-fluoro[1,1'-biphenyl]-4-yl)amino]-2-(morpholin-4-yl)-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0297] 4-[(3',4'-dichloro[1,1'-biphenyl]-4-yl)amino]-2-(morpholin-4-yl)-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0298] 2-(morpholin-4-yl)-6-(propan-2-yl)-4-{{[4-(propan-2-yl)phenyl]amino}-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0299] 4-[(4-tert-butylphenyl)amino]-2-(morpholin-4-yl)-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0300] 4-[(2-methyl[1,1'-biphenyl]-4-yl)amino]-2-(morpholin-4-yl)-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0301] 4-[(4'-chloro[1,1'-biphenyl]-4-yl)amino]-2-(morpholin-4-yl)-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0302] N-(4-cyclohexylphenyl)-2-[(2R)-2-methylmorpholin-4-yl]-5,7-dihydrofuro[3,4-d]pyrimidin-4-amine; [0303] N-(4-cyclobutylphenyl)-2-(3,6-dihydro-2H-pyran-4-yl)-5,7-dihydrofuro[3,4-d]pyrimidin-4-amine; [0304] N-(4-cyclohexylphenyl)-2-(2-cyclopropylmorpholin-4-yl)-5,7-dihydrofuro[3,4-d]pyrimidin-4-amine; [0305] 2-(2-cyclopropylmorpholin-4-yl)-N-[4'-(heptyloxy)[1,1'-biphenyl]-4-yl]-5,7-dihydrofuro[3,4-d]pyrimidin-4-amine; [0306] 2-[(2R)-2-methylmorpholin-4-yl]-N-{4'-[(6,6,6-trifluorohexyl)oxy][1,1'-biphenyl]-4-yl}-5,7-dihydrofuro[3,4-d]pyrimidin-4-amine; [0307] N-(4-cyclohexylphenyl)-2-(2-methylpyridin-4-yl)-5,7-dihydrofuro[3,4-d]pyrimidin-4-amine; [0308] N-(4-cyclohexylphenyl)-2-[(2R)-2-methylmorpholin-4-yl]pyrido[2,3-d]pyrimidin-4-amine (Compound D); [0309] 6-bromo-N-(4-cyclohexylphenyl)-2-[(2R)-2-methylmorpholin-4-yl]pyrido[2,3-d]pyrimidin-4-amine; [0310] N-(4-cyclohexylphenyl)-2-(3,6-dihydro-2H-pyran-4-yl)pyrido[2,3-d]pyrimidin-4-amine; [0311] N-(4-cyclohexylphenyl)-2-[(2R)-2-methylmorpholin-4-yl]-8-oxo-8lambda--5--pyrido[2,3-d]pyrimidin-4-amine; [0312] N-(4-cyclohexylphenyl)-6-ethyl-2-[(2R)-2-methylmorpholin-4-yl]pyrido[2,3-d]pyrimidin-4-amine; [0313] 4-[(4-cyclohexylphenyl)amino]-2-[(2R)-2-methylmorpholin-4-yl]pyrido[2,3-d]pyrimidine-6-carbonitrile (Compound H); [0314] methyl 4-[(4-cyclohexylphenyl)amino]-2-[(2R)-2-methylmorpholin-4-yl]pyrido[2,3-d]pyrimidine-6-carboxylate; [0315] 4-[(4-cyclohexylphenyl)amino]-2-[(2R)-2-methylmorpholin-4-yl]pyrido[2,3-d]pyrimidine-6-carboxylic acid; [0316] methyl (R)-4-[(4-cyclohexylphenyl)amino]-2-(2-methylmorpholino)pyrido[2,3-d]pyrimidine-6-carboxylate (Compound E); [0317] 4-[(4-cyclohexylphenyl)amino]-2-[(2R)-2-methylmorpholin-4-yl]pyrido[2,3-d]pyrimidine-6-carboxamide; [0318] 4-[(4-cyclohexylphenyl)amino]-2-(2-cyclopropylmorpholin-4-yl)pyrido[2,3-d]pyrimidine-6-carboxamide; [0319] N-(4-cyclohexylphenyl)-6-methoxy-2-[(2R)-2-methylmorpholin-4-yl]pyrido[2,3-d]pyrimidin-4-amine; [0320] (R)—N-(4-cyclohexylphenyl)-6-methoxy-2-(2-methylmorpholino)pyrido[2,3-d]pyrimidin-4-amine (Compound F); [0321] N-{4-[(4-cyclohexylphenyl)amino]-2-[(2R)-2-methylmorpholin-4-yl]pyrido[2,3-d]pyrimidin-6-yl}acetamide; [0322] N-4-(4-cyclohexylphenyl)-2-[(2R)-2-methylmorpholin-4-yl]pyrido[2,3-d]pyrimidine-4,6-diamine; [0323] N-(4-cyclohexylphenyl)-2-(morpholin-4-yl)-6-(propan-2-yl)-6,7-dihydro-5H-pyrrolo[3,4-d]pyrimidin-4-amine; [0324] N-(4-cyclohexylphenyl)-2-[(2R)-2-methylmorpholin-4-yl]-6-(propan-2-yl)-6,7-dihydro-5H-pyrrolo[3,4-d]pyrimidin-4-amine; [0325] 2-{4-[(4-cyclohexylphenyl)amino]-2-(3,6-dihydro-2H-pyran-4-yl)-5,7-dihydro-6H-pyrrolo[3,4-d]pyrimidin-6-yl}-N,N-dimethylacetamide (Compound G); [0326] N-(4-cyclohexylphenyl)-2-(2-cyclopropylmorpholin-4-yl)-6-(1-methylpiperidin-4-yl)-6,7-dihydro-5H-pyrrolo[3,4-d]pyrimidin-4-amine; [0327] 2-(morpholin-4-yl)-6-(propan-2-yl)-4-{{[4-(thiophen-2-yl)phenyl]amino}-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0328] 2-(morpholin-4-yl)-6-(propan-2-yl)-4-{{[4-(thiophen-3-yl)phenyl]amino}-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one; [0329] and pharmaceutically acceptable salts thereof.

[0330] In embodiments, a compound of Formula (I) is:

##STR00023##

known as (R)—N-(4-cyclohexylphenyl)-2-(2-methylmorpholino)-5,7-dihydrofuro[3,4-d]pyrimidin-4-amine (also referred herein to as N-(4-cyclohexylphenyl)-2-[(2R)-2-methylmorpholin-4-yl]-5,7-dihydrofuro[3,4-d]pyrimidin-4-amine, or also referred to herein as Compound A or CmpA), or a

pharmaceutically acceptable salt thereof.

[0331] In embodiments, the compound of Formula I is Compound B:

##STR00024##

6-isopropyl-4-((4-isopropylphenyl)amino)-2-(pyridin-4-yl)-5,6-dihydro-7H-pyrrolo [3,4-d]pyrimidin-7-one

Compound B

[0332] In embodiments, the compound of Formula (I) is Compound C:

##STR00025##

4-[(4-cyclohexylphenyl)amino]-2-(2-methylpyridin-4-yl)-6-(propan-2-yl)-5,6-dihydro-7H-pyrrolo[3,4-d]pyrimidin-7-one

Compound C

[0333] In embodiments, the compound of Formula (I) is Compound D:

##STR00026##

N-(4-cyclohexylphenyl)-2-[(2R)-2-methylmorpholin-4-yl]pyrido[2,3-d]pyrimidin-4-amine

Compound D

[0334] In embodiments, the compound of Formula (I) is Compound E:

##STR00027##

methyl (R)-4-((4-cyclohexylphenyl)amino)-2-(2-methylmorpholino) pyrido[2,3-d]pyrimidine-6-carboxylate

Compound E

[0335] In embodiments, the compound of Formula (I) is Compound F:

##STR00028##

(R)—N-(4-cyclohexylphenyl)-6-methoxy-2-(2-methylmorpholino) pyrido[2,3-d]pyrimidin-4-amine

Compound F

[0336] In embodiments, the compound of Formula (I) is Compound G:

##STR00029##

2-(4-((4-cyclohexylphenyl)amino)-2-(3,6-dihydro-2H-pyran-4-yl)-5,7-dihydro-6H-pyrrolo[3,4-d]pyrimidin-6-yl)-N,N-dimethylacetamide

Compound G

[0337] In embodiments, the compound according to Formula (I) is:

##STR00030##

known as (R)-4-((4-cyclohexylphenyl)amino)-2-(2-methylmorpholino)pyrido [2,3-d]pyrimidine-6-carbonitrile (also referred to as 4-[(4-cyclohexylphenyl)amino]-2-[(2R)-2-methylmorpholin-4-yl]pyrido[2,3-d]pyrimidine-6-carbonitrile or Compound H), or a pharmaceutically acceptable salt thereof.

[0338] Atoms of the compounds and salts described herein may exist as their isotopes. All compounds of Formula (I) where an atom is replaced by one or more of its isotopes (for example a compound of Formula (I) where one or more carbon atom is an ¹¹C or ¹³C carbon isotope, or where one or more hydrogen atoms is a ²H or ³H isotope, or where one or more nitrogen atoms is a ¹⁵N isotope or where one of more oxygen atoms is an ¹⁷O or ¹⁸O isotope) are encompassed herein.

[0339] Compounds herein may exist in one or more geometrical, optical, enantiomeric, and diastereomeric forms, including, but not limited to, cis- and trans-forms, E- and Z-forms, and R-, S- and meso-forms. Unless otherwise stated a reference to a particular compound includes all such isomeric forms, including racemic and other mixtures thereof. Where appropriate such isomers can be separated from their mixtures by the application or adaptation of known methods (e.g., chromatographic techniques and recrystallisation techniques). Where appropriate such isomers can be prepared by the application or adaptation of known methods. In embodiments, a single stereoisomer is obtained by isolating it from a mixture of isomers (e.g., a racemate) using, for example, chiral chromatographic separation. In embodiments, a single stereoisomer is obtained

through direct synthesis from, for example, a chiral starting material.

[0340] In embodiments, there is provided a compound of Formula (I), or a pharmaceutically acceptable salt thereof, which is a single optical isomer being in an enantiomeric excess (% e.e.) of $\geq 95\%$, $\geq 98\%$ or $\geq 99\%$. In embodiments, the single optical isomer is present in an enantiomeric excess (% e.e.) of $\geq 99\%$.

[0341] In embodiments, there is provided an N-oxide of a compound of Formula (I) as herein defined, or a pharmaceutically acceptable salt thereof.

[0342] Compounds of Formula (I), where R^{sup.7} is —NR^{sup.10}R^{sup.11} (i.e. R^{sup.7} is linked by an aliphatic N atom), may for example be prepared as described in WO2021/180952 by the reaction of a compound of Formula (V):

##STR00031##

or a salt thereof, where R^{sup.1}, R^{sup.2} and A are as defined herein, with an amine. The reaction is conveniently performed in a suitable solvent and at a suitable temperature, for example, di-isopropylethylamine in dimethylsulfoxide at a temperature of 20-100° C., or TsOH in butanol at 80° C.

[0343] When R^{sup.7} is attached via a carbon atom, the compound of Formula (I) can be made by the reaction of a compound of Formula (V) with a boronic acid or ester of the Formula (VI), where R^{sup.7} is as defined herein and each R is the same or different and represents —H, an aliphatic chain, or where together the two R groups form a ring with the boron and two oxygen atoms. The reaction is conveniently performed with a suitable base in the presence of a palladium catalyst and a solvent at a suitable temperature. For example, cesium carbonate or sodium carbonate and a palladium catalyst such as Pd(PPh_{sub.3})_{sub.4}, in aqueous dioxane at a temperature in the range of 80-100° C.

##STR00032##

[0344] When R^{sup.7} is linked via an aromatic N atom, the compound of Formula (I) can be made by reaction of a compound of Formula (V) with the anion of R^{sup.7}. For example, by reaction of the anion of imidazole, generated by treatment with a suitable base (for example, sodium hydride), in a suitable solvent (for example dimethylformamide), with a compound of Formula (V).

[0345] A compound of the Formula (V) may be prepared from a compound of Formula (VII), or a salt thereof, where A is as defined herein, and a compound of Formula (VIII), or a salt thereof, where R^{sup.1} and R^{sup.2} are as defined herein, in the presence of a base in a suitable solvent (for example, di-isopropylethylamine in tert butanol or dimethylsulfoxide) and at a suitable temperature (for example 20-100° C.).

##STR00033##

[0346] A compound of Formula (I) may also be made in one pot from the reaction between a compound of Formula (VII) with the stepwise addition of a compound of Formula (VIII) and an amine R^{sup.7}. The reaction is conveniently performed in the presence of a base (for example, di-isopropylethylamine) in a suitable solvent (dimethylsulfoxide) at a suitable temperature (for example, a temperature of 20-100° C.).

[0347] A compound of Formula (VII) may be made, for example, from a compound of Formula (IX). Suitable conditions for this transformation are heating at a temperature of about 80° C. in POCl_{sub.3} in the presence of an amine base such as diethylphenylamine.

##STR00034##

[0348] A compound of the Formula (IX) may, for example, be prepared from a compound of the Formula (X) by reaction with propan-2-amine and formaldehyde in a suitable solvent (for example, ethanol) at a suitable temperature (for example, a temperature of 0-80° C.).

##STR00035##

[0349] A compound of the Formula (I), when R^{sup.5a} and R^{sup.5b} are both H, may also be made from reaction of a compound of the Formula (XI), or a salt thereof, where R^{sup.1}, R^{sup.2} and R^{sup.7} are as defined herein, with a suitable amine, for example N, N-dimethylpropane-1,3-

diamine. Suitable conditions for this reaction are HCl in ethanol at a temperature of about 190° C. in a sealed tube.

##STR00036##

[0350] It will be appreciated that certain of the various ring substituents in the compounds herein may be introduced by standard aromatic substitution reactions or generated by conventional functional group modifications either prior to or immediately following the processes mentioned above. For example, compounds of Formula (I) may be converted into further compounds of Formula (I) by conventional functional group modifications. Such reactions and modifications include, for example, introduction of a substituent by means of an aromatic substitution reaction, C—H activation reaction, reduction of substituents, alkylation of substituents and oxidation of substituents. The reagents and reaction conditions for such procedures are well known in the chemical art. Particular examples of aromatic substitution reactions include the introduction of a halogen group.

[0351] It will also be appreciated that in some of the reactions mentioned herein it may be necessary/desirable to protect any sensitive groups in the compounds. The instances where protection is necessary or desirable and suitable methods for protection are known to those skilled in the art. Conventional protecting groups may be used in accordance with standard practice (for illustration see T. W. Green, *Protective Groups in Organic Synthesis*, John Wiley and Sons, 1991). Thus, if reactants include groups such as amino, carboxy or hydroxy it may be desirable to protect the group in some of the reactions mentioned herein.

[0352] As a result of their KCC2 activation activity, the compounds of Formula (I), or pharmaceutically acceptable salts thereof are useful in therapy, for example, in the treatment of one or more synucleinopathies or one or more TDP-43 proteinopathies mediated at least in part by KCC2.

[0353] Without wishing to be bound by any particular theory, in embodiments, the compounds of Formula (I), or pharmaceutically acceptable salts thereof, increase KCC2 activity, thus restoring GABAergic function in patients and diminishing neuronal hyperexcitability, resulting in beneficial therapeutic effects on one or more synucleinopathies or one or more TDP-43 proteinopathies whose symptoms are related to neuronal hyperexcitability. See, e.g., Fuchs et al. *J. Neuropathol. Exp. Neurol.*, Vol 69, No. 10, 1057-170 October 2010 and Lam et al., *Molecules* 2023, 28(3), 1344; <https://doi.org/10.3390/molecules28031344>.

[0354] The term “therapy” is intended to have its normal meaning of dealing with a disease in order to entirely or partially relieve one, some or all of its symptoms, or to correct or compensate for the underlying pathology. In embodiments, the term “therapy” may also include “prophylaxis” if “prophylaxis” is specifically referred to. The term “prophylaxis” is intended to have its normal meaning and includes primary prophylaxis to prevent the development of the disease and secondary prophylaxis whereby the disease has already developed and the patient is temporarily or permanently protected against exacerbation or worsening of the disease or the development of new symptoms associated with the disease. Nonetheless, prophylactic (preventive) and therapeutic (curative) treatment are two separate embodiments of the disclosure herein. In embodiments, the term “treatment” can be used synonymously with “therapy”. Similarly, the term “treat” can be regarded as “applying therapy” where “therapy” is as defined herein. As used herein, the terms “treat”, “treatment” or “treating” as applied to one or more synucleinopathies or one or more TDP-43 proteinopathies encompass any manner in which the symptoms or pathology of a condition, disorder or disease associated with one or more synucleinopathies or one or more TDP-43 proteinopathies are ameliorated or otherwise beneficially altered. In embodiments, “treat”, “treatment” or “treating” can refer to inhibiting a disease or condition, e.g., arresting or reducing its development or at least one clinical or subclinical symptom thereof. In embodiments, “treat”, “treatment” or “treating” can refer to relieving the disease or condition, e.g., causing regression of the disease or condition or at least one of its clinical or subclinical symptoms. The terms “treat”,

“treatment” or “treating” may be used interchangeably herein. In embodiments, treating “dementia” means ameliorating, beneficially altering and/or providing relief from one or more of the symptoms of dementia. In embodiments, treating “dementia” means ameliorating, beneficially altering and/or providing relief from two or more of the symptoms of dementia. In embodiments, treating “dementia” means ameliorating, beneficially altering and/or providing relief from three or more of the symptoms of dementia. The benefit to a subject being treated may be statistically significant, mathematically significant, or at least perceptible to the subject and/or the physician.

[0355] In embodiments, there is provided a compound of Formula (I), or a pharmaceutically acceptable salt thereof, for use in therapy for one or more synucleinopathies or one or more TDP-43 proteinopathies. For example, in embodiments, Compound A, Compound B, Compound C, Compound D, Compound E, Compound F, Compound G, or Compound H, or a pharmaceutically acceptable salt of any of the foregoing, is for use in therapy for one or more synucleinopathies or one or more TDP-43 proteinopathies. For example, in embodiments, Compound B, or a pharmaceutically acceptable salt thereof, is for use in therapy for one or more synucleinopathies or one or more TDP-43 proteinopathies. In embodiments, there is provided the use of a compound of Formula (I), or a pharmaceutically acceptable salt thereof, e.g., Compound A, Compound B, Compound C, Compound D, Compound E, Compound F, Compound G, or Compound H, or a pharmaceutically acceptable salt of any of the foregoing, for the manufacture of a medicament for treating one or more synucleinopathies or one or more TDP-43 proteinopathies.

[0356] The term “therapeutically effective amount” refers to an amount of a compound of Formula (I) as described herein which is effective to provide “therapy” in a subject, or to “treat” a disease or disorder in a subject. In the case of one or more synucleinopathies or one or more TDP-43 proteinopathies, the therapeutically effective amount may cause any of the changes observable or measurable in a subject as described in the definition of “therapy”, “treatment” and “prophylaxis” above. As recognized by those skilled in the art, effective amounts may vary depending on route of administration, excipient usage, and co-usage with other agents. For example, where a combination therapy is used, the amount of the compound of Formula (I) or pharmaceutically acceptable salt described in this specification and the amount of the other pharmaceutically active agent(s) are, when combined, jointly effective to treat a targeted disorder in the patient. In this context, the combined amounts are in a “therapeutically effective amount” if they are, when combined, sufficient to decrease the symptoms of one or more synucleinopathies or one or more TDP-43 proteinopathies responsive to activation of KCC2 as described herein. Typically, such amounts may be determined by one skilled in the art by, for example, starting with the dosage range described in this specification for the compound of Formula (I) or pharmaceutically acceptable salt thereof and an approved or otherwise published dosage range(s) of the other pharmaceutically active compound(s). “Subjects” or “patients” include mammals, for example, humans. The terms “subjects” and “patients” may be used interchangeably herein.

[0357] The compounds of Formula (I), and pharmaceutically acceptable salts thereof, may be administered as pharmaceutical compositions, including one or more pharmaceutically acceptable excipients. Therefore, in embodiments, there is provided a pharmaceutical composition including a compound of Formula (I), or a pharmaceutically acceptable salt thereof, and at least one pharmaceutically acceptable excipient.

[0358] The excipient(s) selected for inclusion in a particular composition will depend on factors such as the mode of administration and the form of the composition provided. Suitable pharmaceutically acceptable excipients are well known to persons skilled in the art and are described, for example, in the *Handbook of Pharmaceutical Excipients*, Sixth edition, Pharmaceutical Press, edited by Rowe, Ray C; Sheskey, Paul J; Quinn, Marian. Pharmaceutically acceptable excipients may function as, for example, adjuvants, diluents, carriers, stabilizers, flavorings, colorants, fillers, binders, disintegrants, lubricants, glidants, thickening agents and coating agents. As persons skilled in the art will appreciate, certain pharmaceutically acceptable

excipients may serve more than one function and may serve alternative functions depending on how much of the excipient is present in the composition and what other excipients are present in the composition.

[0359] The pharmaceutical compositions may be in a form suitable for oral use (for example as tablets, lozenges, hard or soft capsules, films, dragees, aqueous or oily suspensions, emulsions, dispersible powders or granules, syrups or elixirs), for topical use (for example as creams, ointments, gels, lotions, transdermal patches, or aqueous or oily solutions or suspensions), for administration by inhalation (for example as a finely divided powder or a liquid aerosol), for administration by insufflation (for example as a finely divided powder) or for parenteral administration (for example as a sterile aqueous or oily solution or suspension for intravenous, subcutaneous or intramuscular dosing), or as a suppository for rectal dosing. The compositions may be obtained by conventional procedures using conventional pharmaceutical excipients well known in the art. Thus, compositions intended for oral use may contain, for example, one or more coloring, sweetening, flavoring and/or preservative agents.

[0360] In embodiments, a pharmaceutical composition herein may contain a compound according to Formula (I) in any of the amounts set forth herein and a diluent in an amount from 30% to 90% by weight. Examples of diluents include lactose, microcrystalline cellulose, starch, calcium phosphate, calcium carbonate, sucrose, mannitol, maltodextrin and sorbitol. In embodiments, a pharmaceutical composition herein may contain a compound according to Formula (I) and a lubricant in an amount, e.g., from 0.25% to 5.0% by weight. Examples of lubricants include magnesium stearate, stearic acid, sodium stearyl fumarate, talc, polyethylene glycols and silicon dioxide. Examples of water-soluble lubricants include sodium benzoate, polyethylene glycol, and adipic acid. In embodiments, a pharmaceutical composition herein may contain a compound according to Formula (I) and a disintegrant in an amount, e.g., from 1.0% to 10.0% by weight. There are two classes of disintegrants: traditional disintegrants, such as starch, and super disintegrants, which include croscarmellose sodium, crospovidone, and sodium starch glycolate.

[0361] The compound of Formula (I) will normally be administered to a subject, e.g., a warm-blooded animal at a unit dose within the range 2.5-5000 mg/m² body area of the animal, or approximately 0.05-100 mg/kg, and this normally provides a therapeutically-effective dose. A unit dose form such as a tablet, capsule, film, patch, vial will can contain, for example 0.1-500 mg of active ingredient. The daily dose will necessarily be varied depending upon the host treated, the particular route of administration, any therapies being co-administered, and the severity of the illness being treated.

[0362] The pharmaceutical compositions described herein include compounds of Formula (I), or a pharmaceutically acceptable salt thereof, for use in therapy for one or more synucleinopathies or one or more TDP-43 proteinopathies.

[0363] As such, in embodiments, there is provided a pharmaceutical composition for use in therapy for one or more synucleinopathies or one or more TDP-43 proteinopathies, including a compound of Formula (I), or a pharmaceutically acceptable salt thereof, and at least one pharmaceutically acceptable excipient.

[0364] In embodiments, there is provided a pharmaceutical composition for use in the treatment of one or more synucleinopathies or one or more TDP-43 proteinopathies in which activation of KCC2 is beneficial, including a compound of Formula (I), or a pharmaceutically acceptable salt thereof, and at least one pharmaceutically acceptable excipient.

[0365] In embodiments, treatment of one or more synucleinopathies or one or more TDP-43 proteinopathies is implemented by administering to a subject in need thereof about 0.01 mg to about 1500 mg of Formula (I), or a pharmaceutically acceptable salt thereof, such as Compound A, Compound B, Compound C, Compound D, Compound E, Compound F, Compound G, or Compound H, or a pharmaceutically acceptable salt of any of the foregoing. In embodiments, methods include treating one or more synucleinopathies or one or more TDP-43 proteinopathies by

administering to a subject in need thereof about 0.01 mg to about 1500 mg of Formula (I), or a pharmaceutically acceptable salt thereof. In embodiments, the amount Formula (I), or a pharmaceutically acceptable salt thereof, can be, e.g., between 0.1 and 1500 mg/day, or 0.01 mg/kg/day to 15 mg/kg/day, for treatment of one or more synucleinopathies or one or more TDP-43 proteinopathies. For example, the daily dosage can be, e.g., in the range of about 0.01 to 1500 mg, 0.1 to 1250 mg, 0.1 to 1000 mg, 0.1 to 750 mg, 0.1 to 500 mg, 0.1 to 450 mg, 0.1 to 300 mg, 0.1 to 250 mg, 0.1 to 200 mg, 0.1 to 175 mg, 0.1 to 150 mg, 0.1 to 125 mg, 0.1 to 100 mg, 0.1 to 75 mg, 0.1 to 50 mg, 0.1 to 30 mg, 0.1 to 25 mg, 0.1 to 20 mg, 0.1 to 15 mg, 0.1 to 10 mg, 0.1 to 5 mg, 0.1 to 1 mg, 1 to 1500 mg, 1 to 1000 mg, 1 to 500 mg, 1 to 300 mg, 1 to 250 mg, 1 to 200 mg, 1 to 175 mg, 1 to 150 mg, 1 to 125 mg, 1 to 100 mg, 1 to 75 mg, 1 to 50 mg, 1 to 30 mg, 1 to 25 mg, 1 to 20 mg, 1 to 15 mg, 1 to 10 mg, 1 to 5 mg, 5 to 1500 mg, 5 to 1000 mg, 5 to 500 mg, 5 to 300 mg, 5 to 250 mg, 5 to 200 mg, 5 to 175 mg, 5 to 150 mg, 5 to 125 mg, 5 to 100 mg, 5 to 75 mg, 5 to 50 mg, 5 to 30 mg, 5 to 25 mg, 5 to 20 mg, 5 to 15 mg, 5 to 10 mg, 10 to 1500 mg, 10 to 1000 mg, 10 to 500 mg, 10 to 300 mg, 10 to 250 mg, 10 to 200 mg, 10 to 175 mg, 10 to 150 mg, 10 to 125 mg, 10 to 100 mg, 10 to 75 mg, 10 to 50 mg, 10 to 30 mg, 10 to 25 mg, 10 to 20 mg, 0 to 5 mg, 15 to 1500 mg, 15 to 1000 mg, 15 to 500 mg, 15 to 300 mg, 15 to 250 mg, 15 to 200 mg, 15 to 175 mg, 15 to 150 mg, 15 to 125 mg, 15 to 100 mg, 15 to 75 mg, 15 to 50 mg, 15 to 30 mg, 15 to 25 mg, 15 to 20 mg, 20 to 1500 mg, 20 to 1000 mg, 20 to 500 mg, 20 to 300 mg, 20 to 250 mg, 20 to 200 mg, 20 to 175 mg, 20 to 150 mg, 20 to 125 mg, 20 to 100 mg, 20 to 75 mg, 20 to 50 mg, 20 to 30 mg, 20 to 25 mg, 25 to 1500 mg, 25 to 1000 mg, 25 to 500 mg, 25 to 300 mg, 25 to 250 mg, 25 to 200 mg, 25 to 175 mg, 25 to 150 mg, 25 to 125 mg, 100 mg, 25 to 75 mg, 25 to 50 mg, 25 to 30 mg, 30 to 1500 mg, 30 to 1000 mg, 30 to 500 mg, 30 to 300 mg, 30 to 250 mg, 30 to 200 mg, 30 to 175 mg, 30 to 150 mg, 30 to 125 mg, 100 mg, 30±75 mg, 30 to 50 mg, 35 to 1500 mg, 35 to 1000 mg, 35 to 500 mg, 35 to 300 mg, 35 to 250 mg, 35 to 200 mg, 35 to 175 mg, 35 to 150 mg, 35 to 125 mg, 35 to 100 mg, 5 mg, 35 to 50 mg, 40 to 1500 mg, 40 to 1000 mg, 40 to 500 mg, 40 to 300 mg, 40 to 250 mg, 40 to 200 mg, 40 to 175 mg, 40 to 150 mg, 40 to 125 mg, 40 to 100 mg, 40 to 75 mg, 40 to 50 mg, 50 to 1500 mg, 50 to 1000 mg, 50 to 500 mg, 50 to 300 mg, 50 to 250 mg, 50 to 200 mg, 50 to 175 mg, 50 to 150 mg, 50 to 125 mg, 50 to 100 mg, 50 to 75 mg, 75 to 1500 mg, 75 to 1000 mg, 75 to 500 mg, 75 to 300 mg, 75 to 250 mg, 75 to 200 mg, 75 to 175 mg, 75 to 150 mg, 75 to 125 mg, 75 to 100 mg, 100 to 1500 mg, 100 to 1000 mg, 100 to 500 mg, 100 to 300 mg, 100 to 250 mg, 100 to 200 mg, 100 to 175 mg, 100 to 150 mg, 100 to 125 mg, 125 to 1500 mg, 125 to 1000 mg, 125 to 500 mg, 125 to 300 mg, 125 to 250 mg, 125 to 200 mg, 125 to 175 mg, 125 to 150 mg, 150 to 1500 mg, 150 to 1000 mg, 150 to 500 mg, 150 to 300 mg, 150 to 250 mg, 150 to 200 mg, 150 to 175 mg, 175 to 1500 mg, 175 to 1000 mg, 175 to 500 mg, 175 to 300 mg, 175 to 250 mg, 175 to 200 mg, 200 to 1500 mg, 200 to 1000 mg, 200 to 500 mg, 200 to 300 mg, 200 to 250 mg, 250 to 1500 mg, 250 to 1000 mg, 250 to 500 mg, 250 to 300 mg, 7.5 to 15 mg, 2.5 to 5 mg, 1 to 5 mg, with doses of, e.g., about 0.25 mg, 0.5 mg, 0.75 mg, 1 mg, 1.25 mg, 1.5 mg, 1.75 mg, 2.0 mg, 2.5 mg, 3.0 mg, 3.5 mg, 4.0 mg, 4.5 mg, 5 mg, 7.5 mg, 10 mg, 12.5 mg, 15 mg, 17.5 mg, 20 mg, 22.5 mg, 25 mg, 27.5 mg, 30 mg, 35 mg, 40 mg, 45 mg, 50 mg, 75 mg, 100 mg, 125 mg, 150 mg, 175 mg, 200 mg, 225 mg, 250 mg, 275 mg, 300 mg, 400 mg and 500 mg being examples.

[0366] In embodiments, pharmaceutical compositions for treating one or more synucleinopathies or one or more TDP-43 proteinopathies may include Formula (I), or a pharmaceutically acceptable salt thereof, in an amount of, e.g., about 0.01 to 1500 mg, 0.01 to 1250 mg, 0.01 to 1000 mg, 0.01 to 750 mg, 0.01 to 500 mg, 0.01 to 250 mg, 0.01 to 100 mg, 0.01 to 50 mg, 0.01 to 25 mg, 0.01 to 10 mg, 0.01 to 5 mg, 0.01 to 1 mg, 0.1 to 500 mg, 0.1 to 450 mg, 0.1 to 300 mg, 0.1 to 250 mg, 0.1 to 200 mg, 0.1 to 175 mg, 0.1 to 150 mg, 0.1 to 125 mg, 0.1 to 100 mg, 0.1 to 75 mg, 0.1 to 50 mg, 0.1 to 30 mg, 0.1 to 25 mg, 0.1 to 20 mg, 0.1 to 15 mg, 0.1 to 10 mg, 0.1 to 5 mg, 0.1 to 1 mg, 0.5 to 500 mg, 0.5 to 450 mg, 0.5 to 300 mg, 0.5 to 250 mg, 0.5 to 200 mg, 0.5 to 175 mg, 0.5 to 150 mg, 0.5 to 125 mg, 0.5 to 100 mg, 0.5 to 75 mg, 0.5 to 50 mg, 0.5 to 30 mg, 0.5 to 25 mg, 0.5 to 20 mg, 0.5 to 15 mg, 0.5 to 10 mg, 0.5 to 5 mg, 0.5 to 1 mg, 1 to 500 mg, 1 to 450 mg, 1 to 300 mg, 1

to 250 mg, 1 to 200 mg, 1 to 175 mg, 1 to 150 mg, 1 to 125 mg, 1 to 100 mg, 1 to 75 mg, 1 to 50 mg, 1 to 30 mg, 1 to 25 mg, 1 to 20 mg, 1 to 15 mg, 1 to 10 mg, 1 to 5 mg, 5 to 500 mg, 5 to 450 mg, 5 to 300 mg, 5 to 250 mg, 5 to 200 mg, 5 to 175 mg, 5 to 150 mg, 5 to 125 mg, 5 to 100 mg, 5 to 75 mg, 5 to 50 mg, 5 to 30 mg, 5 to 25 mg, 5 to 20 mg, 5 to 15 mg, 5 to 10 mg, 10 to 500 mg, 10 to 450 mg, 10 to 300 mg, 10 to 250 mg, 10 to 200 mg, 10 to 175 mg, 10 to 150 mg, 10 to 125 mg, 10 to 100 mg, 10 to 75 mg, 10 to 50 mg, 10 to 30 mg, 10 to 25 mg, 10 to 20 mg, 10 to 15 mg, 15 to 500 mg, 15 to 450 mg, 15 to 300 mg, 15 to 250 mg, 15 to 200 mg, 15 to 175 mg, 15 to 150 mg, 15 to 125 mg, 15 to 100 mg, 15 to 75 mg, 15 to 50 mg, 15 to 30 mg, 15 to 25 mg, 15 to 20 mg, 20 to 500 mg, 20 to 450 mg, 20 to 300 mg, 20 to 250 mg, 20 to 200 mg, 20 to 175 mg, 20 to 150 mg, 20 to 125 mg, 20 to 100 mg, 20 to 75 mg, 20 to 50 mg, 20 to 30 mg, 20 to 25 mg, 25 to 500 mg, 25 to 450 mg, 25 to 300 mg, 25 to 250 mg, 25 to 200 mg, 25 to 175 mg, 25 to 150 mg, 25 to 125 mg, 25 to 100 mg, 25 to 80 mg, 25 to 75 mg, 25 to 50 mg, 25 to 30 mg, 30 to 500 mg, 30 to 450 mg, 30 to 300 mg, 30 to 250 mg, 30 to 200 mg, 30 to 175 mg, 30 to 150 mg, 30 to 125 mg, 30 to 100 mg, 30 to 75 mg, 30 to 50 mg, 40 to 500 mg, 40 to 450 mg, 40 to 400 mg, 40 to 250 mg, 40 to 200 mg, 40 to 175 mg, 40 to 150 mg, 40 to 125 mg, 40 to 100 mg, 40 to 75 mg, 40 to 50 mg, 50 to 500 mg, 50 to 450 mg, 50 to 300 mg, 50 to 250 mg, 50 to 200 mg, 50 to 175 mg, 50 to 150 mg, 50 to 125 mg, 50 to 100 mg, 50 to 75 mg, 75 to 500 mg, 75 to 450 mg, 75 to 300 mg, 75 to 250 mg, 75 to 200 mg, 75 to 175 mg, 75 to 150 mg, 75 to 125 mg, 75 to 100 mg, 100 to 500 mg, 100 to 450 mg, 100 to 300 mg, 100 to 250 mg, 100 to 200 mg, 100 to 175 mg, 100 to 150 mg, 100 to 125 mg, 125 to 500 mg, 125 to 450 mg, 125 to 300 mg, 125 to 250 mg, 125 to 200 mg, 125 to 175 mg, 125 to 150 mg, 150 to 500 mg, 150 to 450 mg, 150 to 300 mg, 150 to 250 mg, 150 to 200 mg, 200 to 500 mg, 200 to 450 mg, 200 to 300 mg, 200 to 250 mg, 250 to 500 mg, 250 to 450 mg, 250 to 300 mg, 300 to 500 mg, 300 to 450 mg, 300 to 400 mg, 300 to 350 mg, 350 to 500 mg, 350 to 450 mg, 350 to 400 mg, 400 to 500 mg, 400 to 450 mg, with 0.1 mg, 0.25 mg, 0.5 mg, 0.75 mg, 1 mg, 2.5 mg, 5 mg, 7.5 mg, 10 mg, 12.5 mg, 15 mg, 17.5 mg, 20 mg, 22.5 mg, 25 mg, 30 mg, 35 mg, 40 mg, 45 mg, 50 mg, 55 mg, 60 mg, 65 mg, 70 mg, 75 mg, 80 mg, 85 mg, 90 mg, 95 mg, 100 mg, 125 mg, 150 mg, 175 mg, 200 mg, 225 mg, 250 mg, 275 mg, 300 mg, 325 mg, 350 mg, 375 mg, 400 mg, 425 mg, 450 mg, 475 mg, and 500 mg being examples.

[0367] Typically, dosages may be administered to a subject having one or more synucleinopathies or one or more TDP-43 proteinopathies once, twice, three or four times daily, every other day, once weekly, or once a month. In embodiments, Formula (I), or a pharmaceutically acceptable salt thereof, is administered to a subject having one or more synucleinopathies or one or more TDP-43 proteinopathies once a day (e.g., morning or night), or twice a day, (e.g., morning and evening), three times a day (e.g., at breakfast, lunch, and dinner, or every 8 hours), or four times a day (e.g., breakfast, lunch, dinner and at bedtime) at a dose of 0.01-1000 mg/administration. In embodiments, the pharmaceutical compositions described herein may be administered by continuous infusion.

[0368] In embodiments, Formula (I), or a pharmaceutically acceptable salt thereof, is administered to a subject having one or more synucleinopathies or one or more TDP-43 proteinopathies 1500 mg/per day, 1400 mg/per day, 1300 mg/per day, 1200 mg/per day, 1000 mg/per day, 900 mg/per day, 800 mg/per day, 700 mg/per day, 600 mg/per day, 500 mg/per day, 400 mg/per day, 300 mg/per day, 200 mg/per day, 100 mg/per day, 95 mg/per day, 90 mg/per day, 85 mg/per day, 80 mg/per day, 75 mg/per day, 70 mg/per day, 65 mg/per day, 60 mg/per day, 55 mg/per day, 50 mg/per day, 45 mg/per day, 40 mg/per day, 35 mg/per day, 30 mg/per day, 25 mg/per day, 20 mg/per day, 15 mg/per day, 10 mg/per day, 5 mg/per day, 4 mg/per day, 3 mg/per day, 3 mg/per day, 2 mg/per day, 1 mg/per day, in one or more doses. Dosages can be lower for infants and children than for adults. In embodiments, an infant or pediatric dose can be about 0.1 to 1500 mg per day once or in 2, 3 or 4 divided doses. In embodiments, a pediatric dose can be 0.05 mg/kg/day to 1500 mg/kg/day. In embodiments, the subject may be started at a low dose and the dosage is escalated over time. It should be understood that the above amounts are exemplary and doses of Formula (I), or a pharmaceutically acceptable salt thereof, can include different amounts and

varying ranges within a continuum between the minimum or maximum amounts described above in connection with Formula (I), or a pharmaceutically acceptable salt thereof.

[0369] In embodiments, methods of treating one or more synucleinopathies or one or more TDP-43 proteinopathies are provided which include administering to a subject in need thereof a pharmaceutical composition including Formula (I), or a pharmaceutically acceptable salt thereof, wherein the composition provides improvement in one or more symptoms of the one or more synucleinopathies or one or more TDP-43 proteinopathies for more than 1 hour after administration to the subject. In embodiments, methods of treating one or more synucleinopathies or one or more TDP-43 proteinopathies are provided which include administering to a subject in need thereof a pharmaceutical composition including (Formula (I), or a pharmaceutically acceptable salt thereof, wherein the composition provides improvement in one or more symptoms of the one or more synucleinopathies or one or more TDP-43 proteinopathies for more than 2 hours after administration to the subject. In embodiments, methods of treating one or more synucleinopathies or one or more TDP-43 proteinopathies are provided which include administering to a subject in need thereof a pharmaceutical composition including Formula (I), or a pharmaceutically acceptable salt thereof, wherein the composition provides improvement in one or more symptoms of the one or more synucleinopathies or one or more TDP-43 proteinopathies for more than 3 hours after administration to the subject. In embodiments, methods of treating one or more synucleinopathies or one or more TDP-43 proteinopathies are provided which include administering to a subject in need thereof a pharmaceutical composition including Formula (I), or a pharmaceutically acceptable salt thereof, wherein the composition provides improvement in one or more symptoms of the one or more synucleinopathies or one or more TDP-43 proteinopathies for more than 4 hours after administration to the subject. In embodiments, methods of treating one or more synucleinopathies or one or more TDP-43 proteinopathies are provided which include administering to a subject in need thereof a pharmaceutical composition including Formula (I), or a pharmaceutically acceptable salt thereof, wherein the composition provides improvement in one or more symptoms of the one or more synucleinopathies or one or more TDP-43 proteinopathies for more than 6 hours after administration to the subject. In embodiments, methods of treating one or more synucleinopathies or one or more TDP-43 proteinopathies are provided which include administering to a subject in need thereof a pharmaceutical composition including Formula (I), or a pharmaceutically acceptable salt thereof, wherein the composition provides improvement in one or more symptoms of the one or more synucleinopathies or one or more TDP-43 proteinopathies for more than 8, 10, 12, 14, 16, 18, 20, 22 or 24 hours after administration to the subject. In embodiments, improvement in at least one symptom for 12 hours after administration of the pharmaceutical composition to the subject is provided in accordance with the present disclosure. In embodiments, the pharmaceutical compositions provide improvement in next day functioning of the subject. For example, the pharmaceutical compositions may provide improvement in one or more symptoms of one or more synucleinopathies or one or more TDP-43 proteinopathies for more than about, e.g., 2 hours, 4 hours, 6 hours, 8 hours, 10 hours, 12 hours, 14 hours, 16 hours, 18 hours, 20 hours, 22 hours or 24 hours after administration and waking from a night of sleep.

[0370] In embodiments, provided herein are methods of treating one or more synucleinopathies or one or more TDP-43 proteinopathies including administering to a subject in need thereof a pharmaceutical composition including Formula (I), or a pharmaceutically acceptable salt thereof, in an effective amount, after an early sign of one or more symptoms of one or more synucleinopathies or one or more TDP-43 proteinopathies is detected to reduce or prevent further symptoms. In embodiments, a continuing regimen of administration of active agents herein according to Formula (I), or a pharmaceutically acceptable salt thereof, is effective to reduce or prevent occurrence of symptoms associated with one or more synucleinopathies or one or more TDP-43 proteinopathies.

[0371] In embodiments, the methods described herein are effective to reduce, delay, or prevent one or more other clinical symptoms of one or more synucleinopathies or one or more TDP-43

proteinopathies. For example, the effect of a composition including Formula (I), or a pharmaceutically acceptable salt thereof, on a particular symptom, pharmacologic, or physiologic indicator can be compared to an untreated subject, or the condition of the subject prior to treatment. In embodiments, the symptom, pharmacologic, and/or physiologic indicator is measured in a subject prior to treatment, and again one or more times after treatment is initiated. In embodiments, the control is a reference level, or average determined based on measuring the symptom, pharmacologic, or physiologic indicator in one or more subjects that do not have the disease or condition to be treated (e.g., healthy subjects). In embodiments, the effect of the treatment is compared to a conventional treatment that is known the art.

[0372] Effective treatment of one or more synucleinopathies or one or more TDP-43 proteinopathies herein may be established by showing reduction in the frequency or severity of symptoms (e.g., more than, e.g., 10%, 20%, 30% 40%, 50% or more) after a period of time compared with baseline. For example, after a baseline period of 1 month, the subjects may be randomly allocated Formula (I), or a pharmaceutically acceptable salt thereof, or placebo as add-on therapy to standard therapies, during a double-blind period of, e.g., 2 months.

[0373] In embodiments, primary outcome measurements may include the percentage of responders on Formula (I), or a pharmaceutically acceptable salt thereof, and on placebo, defined as having experienced at least a 10% to 50% or more reduction of symptoms during the second month of the double-blind period compared with baseline.

[0374] Cognitive impairment (aka impairment of cognition) may be associated with subjects having one or more synucleinopathies or one or more TDP-43 proteinopathies. Cognitive impairment may be measured against normal cognitive function, which refers to the normal physiologic activity of the brain, including, but not limited to, one or more of the following: mental stability, memory/recall abilities, problem solving abilities, reasoning abilities, thinking abilities, judging abilities, ability to discriminate or make choices, capacity for learning, ease of learning, perception, intuition, attention, and awareness, as measured by any criteria suitable in the art.

[0375] Cognitive impairment also includes deficits in mental activities that are mild or that otherwise do not significantly interfere with daily life. Mild cognitive impairment (MCI) is an example of such a condition. A subject with mild cognitive impairment may display symptoms of dementia as described above (e.g., difficulties with language or memory) but the severity of these symptoms is such that a diagnosis of dementia may not be appropriate.

[0376] One skilled in the art will appreciate that there are numerous human and animal models that may be used to evaluate and compare the relative safety and efficacy of compounds described herein for the treatment of cognitive impairment associated with one or more synucleinopathies or one or more TDP-43 proteinopathies. In humans, cognitive function may be measured, for example and without limitation, by the clinical global impression of change scale (CGI); the Mini Mental State Exam (MMSE) (aka the Folstein Test); the Neuropsychiatric Inventory (NPI); the Clinical Dementia Rating Scale (CDR); the Cambridge Neuropsychological Test Automated Battery (CANTAB), the Sandoz Clinical Assessment-Geriatric (SCAG) scale, the Benton Visual Retention Test (BVRT), Montreal Cognitive Assessment (MoCA) or Digit Symbol Substitution Test (DSST).

[0377] In animal model systems, cognitive function may be measured in various conventional ways known in the art, including using a Morris Water Navigation Task, Barnes maze, radial arm maze task, T maze and the like. Other tests known in the art may also be used to assess cognitive function, such as novel object recognition and odor recognition tasks.

[0378] Cognitive function may also be measured using imaging techniques such as Positron Emission Tomography (PET), functional magnetic resonance imaging (fMRI), Single Photon Emission Computed Tomography (SPECT), or any other imaging technique that allows one to measure brain function. In animals, cognitive function may also be measured with electrophysiological techniques.

[0379] In embodiments, Formula (I), or a pharmaceutically acceptable salt thereof, is administered

via a pharmaceutical composition for treatment of one or more synucleinopathies or one or more TDP-43 proteinopathies. Pharmaceutical compositions herein encompass dosage forms. Dosage forms herein encompass unit doses. In embodiments, as discussed below, various dosage forms including conventional formulations and modified release formulations can be administered one or more times daily. Any suitable route of administration may be utilized, e.g., enteral or parenteral, including oral, rectal, nasal, pulmonary, vaginal, sublingual, transdermal, intravenous, intraarterial, intramuscular, intraperitoneal and subcutaneous routes. As mentioned previously, suitable dosage forms include tablets, capsules, oral liquids, powders, aerosols, transdermal modalities such as topical liquids, patches, creams and ointments, parenteral formulations and suppositories.

[0380] In embodiments, provided herein are methods of treating one or more synucleinopathies or one or more TDP-43 proteinopathies including administering to a subject in need thereof Formula (I), or a pharmaceutically acceptable salt thereof, which provides an in vivo plasma profile of Formula (I), or a pharmaceutically acceptable salt thereof, wherein the in vivo plasma profile of Formula (I), or a pharmaceutically acceptable salt thereof, in the subject 10 hours after administration of Formula (I), or a pharmaceutically acceptable salt thereof, is reduced by more than 50% and the method provides improvement in the subject for more than 10, 12, 14, 16, 18, 20, 22 or 24 hours after administration.

[0381] In embodiments, provided herein are methods of treating one or more synucleinopathies or one or more TDP-43 proteinopathies including administering to a subject in need thereof Formula (I), or a pharmaceutically acceptable salt thereof, which provides an in vivo plasma profile, wherein the in vivo plasma profile of Formula (I), or a pharmaceutically acceptable salt thereof, in the subject 10 hours after administration of Formula (I), or a pharmaceutically acceptable salt thereof, is reduced by more than 55% and the method provides improvement in the subject for more than 10, 12, 14, 16, 18, 20, 22 or 24 hours after administration. In embodiments, provided herein are methods of treating one or more synucleinopathies or one or more TDP-43 proteinopathies including administering to a subject in need thereof Formula (I), or a pharmaceutically acceptable salt thereof, which provides an in vivo plasma profile, wherein the in vivo plasma profile of Formula (I), or a pharmaceutically acceptable salt thereof, in the subject 10 hours after administration of Formula (I), or a pharmaceutically acceptable salt thereof, is reduced by more than 55% and the method provides improvement in the subject for more than 10, 12, 14, 16, 18, 20, 22 or 24 hours after administration.

[0382] In embodiments, provided herein are methods of treating one or more synucleinopathies or one or more TDP-43 proteinopathies including administering to a subject in need thereof Formula (I), or a pharmaceutically acceptable salt thereof, which provides an in vivo plasma profile, wherein the in vivo plasma profile of Formula (I), or a pharmaceutically acceptable salt thereof, in the subject 10 hours after administration of Formula (I), or a pharmaceutically acceptable salt thereof, is reduced by more than 60% and the method provides improvement in the subject for more than 10, 12, 14, 16, 18, 20, 22 or 24 hours after administration. In embodiments, provided herein are methods of treating one or more synucleinopathies or one or more TDP-43 proteinopathies including administering to a subject in need thereof Formula (I), or a pharmaceutically acceptable salt thereof, which provides an in vivo plasma profile, wherein the in vivo plasma profile of Formula (I), or a pharmaceutically acceptable salt thereof, in the subject 10 hours after administration of Formula (I), or a pharmaceutically acceptable salt thereof, is reduced by more than 60% and the method provides improvement in the subject for more than 10, 12, 14, 16, 18, 20, 22 or 24 hours after administration, in the subject for more than 10, 12, 14, 16, 18, 20, 22 or 24 hours after administration.

[0383] In embodiments, provided herein are methods of treating one or more synucleinopathies or one or more TDP-43 proteinopathies including administering to a subject in need thereof Formula (I), or a pharmaceutically acceptable salt thereof, which provides an in vivo plasma profile, wherein the in vivo plasma profile of Formula (I), or a pharmaceutically acceptable salt thereof, in

the subject 10 hours after administration of Formula (I), or a pharmaceutically acceptable salt thereof, is reduced by more than 65% and the method provides improvement in the subject for more than 10, 12, 14, 16, 18, 20, 22 or 24 hours after administration. In embodiments, provided herein are methods of treating one or more synucleinopathies or one or more TDP-43 proteinopathies including administering to a subject in need thereof Formula (I), or a pharmaceutically acceptable salt thereof, which provides an in vivo plasma profile, wherein the in vivo plasma profile of Formula (I), or a pharmaceutically acceptable salt thereof, in the subject 10 hours after administration of (Formula (I), or a pharmaceutically acceptable salt thereof, is reduced by more than 65% and the method provides improvement in the subject for more than 10, 12, 14, 16, 18, 20, 22 or 24 hours after administration.

[0384] In embodiments, provided herein are methods of treating one or more synucleinopathies or one or more TDP-43 proteinopathies wherein the amount of Formula (I), or a pharmaceutically acceptable salt thereof, within the subject about 4 hours after administration of the pharmaceutical composition is less than about 75% of the administered dose. In embodiments, provided herein are methods wherein the amount of Formula (I), or a pharmaceutically acceptable salt thereof, within the subject about, e.g., 6 hours, 8 hours, 10 hours, 12 hours, 15 hours, or 20 hours after administration of the pharmaceutical composition is less than about 75%.

[0385] In embodiments, provided herein are methods of treating one or more synucleinopathies or one or more TDP-43 proteinopathies wherein the amount of Formula (I), or a pharmaceutically acceptable salt thereof, within the subject about 4 hours after administration of the pharmaceutical composition is less than about 80% of the administered dose. In embodiments, provided herein are methods wherein the amount of Formula (I), or a pharmaceutically acceptable salt thereof, within the subject about, e.g., 6 hours, 8 hours, 10 hours, 12 hours, 15 hours, or 20 hours after administration of the pharmaceutical composition is less than about 80% of the administered dose.

[0386] In embodiments, provided herein are methods of treating one or more synucleinopathies or one or more TDP-43 proteinopathies wherein the amount of Formula (I), or a pharmaceutically acceptable salt thereof, within the subject about 4 hours after administration of the pharmaceutical composition is between about 65% to about 85% of the administered dose. In embodiments, the amount of Formula (I), or a pharmaceutically acceptable salt thereof, within the subject after about, e.g., 6 hours, 8 hours, 10 hours, 12 hours, 15 hours, or 20 hours after administration of the pharmaceutical composition is between about 65% to about 85% of the administered dose.

[0387] In embodiments, as mentioned previously, pharmaceutical compositions herein may be provided with conventional release or modified release profiles. Pharmaceutical compositions may be prepared using a pharmaceutically acceptable “carrier” or “excipient” composed of materials that are considered safe and effective. The “carrier” includes all components present in the pharmaceutical formulation other than the active ingredient or ingredients. The term “carrier” includes, but is not limited to, diluents, binders, lubricants, disintegrants, fillers, and coating compositions. Those with skill in the art are familiar with such pharmaceutical carriers and methods of compounding pharmaceutical compositions using such carriers.

[0388] In embodiments, pharmaceutical compositions herein are modified release dosage forms which provide modified release profiles. Modified release profiles may exhibit immediate release, delayed release, or extended release profiles. Conventional (or unmodified) release oral dosage forms such as tablets, capsules, suppositories, syrups, solutions and suspensions typically release medications into the mouth, stomach or intestines as the tablet, capsule shell or suppository dissolves, or, in the case of syrups, solutions and suspensions, when they are swallowed. The pattern of drug release from modified release (MR) dosage forms is deliberately changed from that of a conventional dosage form to achieve a desired therapeutic objective and/or better patient compliance. Types of MR drug products include orally disintegrating dosage forms (ODDFs) which provide immediate release, extended release dosage forms, delayed release dosage forms (e.g., enteric coated), and pulsatile release dosage forms.

[0389] An ODDF is a solid dosage form containing a medicinal substance or active ingredient which disintegrates rapidly, usually within a matter of seconds when placed upon the tongue. The disintegration time for ODDFs generally range from one or two seconds to about a minute. ODDFs are designed to disintegrate or dissolve rapidly on contact with saliva. This mode of administration can be beneficial to people who may have problems swallowing tablets whether it be from physical infirmity or psychiatric in nature. Subjects with one or more synucleinopathies or one or more TDP-43 proteinopathies may exhibit such behavior. ODDF's can provide rapid delivery of medication to the blood stream through mucosa resulting in a rapid onset of action. Examples of ODDFs include orally disintegrating tablets, capsules and rapidly dissolving films and wafers.

[0390] Extended release dosage forms (ERDFs) have extended release profiles and are those that allow a reduction in dosing frequency as compared to that presented by a conventional dosage form, e.g., a solution or unmodified release dosage form. ERDFs provide a sustained duration of action of a drug. Suitable formulations which provide extended release profiles are well-known in the art. For example, coated slow release beads or granules ("beads" and "granules" are used interchangeably herein) in which Formula (I), or a pharmaceutically acceptable salt thereof, is applied to beads, e.g., confectioners nonpareil beads, and then coated with conventional release retarding materials such as waxes, enteric coatings and the like. In embodiments, beads can be formed in which Formula (I), or a pharmaceutically acceptable salt thereof, is mixed with a material to provide a mass from which the drug leaches out. In embodiments, the beads may be engineered to provide different rates of release by varying characteristics of the coating or mass, e.g., thickness, porosity, using different materials, etc. Beads having different rates of release may be combined into a single dosage form to provide variable or continuous release. The beads can be contained in capsules or compressed into tablets.

[0391] In embodiments, modified dosage forms herein incorporate delayed release dosage forms having delayed release profiles. Delayed release dosage forms can include delayed release tablets or delayed release capsules. A delayed release tablet is a solid dosage form which releases a drug (or drugs) such as Formula (I), or a pharmaceutically acceptable salt thereof, at a time other than promptly after administration. A delayed release capsule is a solid dosage form in which the drug is enclosed within either a hard or soft soluble container made from a suitable form of gelatin, and which releases a drug (or drugs) at a time other than promptly after administration. For example, enteric-coated tablets, capsules, particles and beads are well-known examples of delayed release dosage forms. Enteric coated tablets, capsules and particles and beads pass through the stomach and release the drug in the intestine. In embodiments, a delayed release tablet is a solid dosage form containing a conglomerate of medicinal particles that releases a drug (or drugs) at a time other than promptly after administration. In embodiments, the conglomerate of medicinal particles are covered with a coating which delays release of the drug. In embodiments, a delayed release capsule is a solid dosage form containing a conglomerate of medicinal particles that releases a drug (or drugs) at a time other than promptly after administration. In embodiments, the conglomerate of medicinal particles is covered with a coating which delays release of the drug.

[0392] Delayed release dosage forms are known to those skilled in the art. For example, coated delayed release beads or granules in which one Formula (I), or a pharmaceutically acceptable salt thereof, is applied to beads, e.g., confectioners nonpareil beads, and then coated with conventional release delaying materials such as waxes, enteric coatings and the like. In embodiments, beads can be formed in which Formula (I), or a pharmaceutically acceptable salt thereof, is mixed with a material to provide a mass from which the drug leaches out. In embodiments, the beads may be engineered to provide different rates of release by varying characteristics of the coating or mass, e.g., thickness, porosity, using different materials, etc. In embodiments, enteric coated granules of Formula (I), or a pharmaceutically acceptable salt thereof, can be contained in an enterically coated capsule or tablet which releases the granules in the small intestine. In embodiments, the granules have a coating which remains intact until the coated granules reach at least the ileum and thereafter

provide a delayed release of the drug in the colon. Suitable enteric coating materials are well known in the art, e.g., Eudragit® coatings such methacrylic acid and methyl methacrylate polymers and others. The granules can be contained in capsules or compressed into tablets.

[0393] In embodiments, Formula (I), or a pharmaceutically acceptable salt thereof, is incorporated into porous inert carriers that provide delayed release profiles. In embodiments, the porous inert carriers incorporate channels or passages from which the drug diffuses into surrounding fluids. In embodiments, Formula (I), or a pharmaceutically acceptable salt thereof, is incorporated into an ion-exchange resin to provide a delayed release profile. Delayed action may result from a predetermined rate of release of the drug from the resin when the drug-resin complex contacts gastrointestinal fluids and the ionic constituents dissolved therein. In embodiments, membranes are utilized to control rate of release from drug containing reservoirs. In embodiments, liquid preparations may also be utilized to provide a delayed release profile. For example, a liquid preparation consisting of solid particles dispersed throughout a liquid phase in which the particles are not soluble. The suspension is formulated to allow at least a reduction in dosing frequency as compared to that drug presented as a conventional dosage form (e.g., as a solution or a prompt drug-releasing, conventional solid dosage form). For example, a suspension of ion-exchange resin constituents or microbeads.

[0394] In embodiments, pharmaceutical compositions described herein are suitable for parenteral administration, including, e.g., intramuscular (i.m.), intravenous (i.v.), subcutaneous (s.c.), intraperitoneal (i.p.), or intrathecal (i.t.). Parenteral compositions must be sterile for administration by injection, infusion or implantation into the body and may be packaged in either single-dose or multi-dose containers. In embodiments, liquid pharmaceutical compositions for parenteral administration to a subject include Formula (I), or a pharmaceutically acceptable salt thereof, in any of the respective amounts described above. In embodiments, the pharmaceutical compositions for parenteral administration are formulated as a total volume of about, e.g., 10 ml, 20 ml, 25 ml, 50 ml, 100 ml, 200 ml, 250 ml, or 500 ml. In embodiments, the compositions are contained in a bag, a glass vial, a plastic vial, or a bottle.

[0395] In embodiments, pharmaceutical compositions for parenteral administration include respective amounts described above for Formula (I), or a pharmaceutically acceptable salt thereof. In embodiments, pharmaceutical compositions for parenteral administration include about 0.05 mg to about 500 mg Formula (I), or a pharmaceutically acceptable salt thereof. In embodiments, pharmaceutical compositions for parenteral administration to a subject include Formula (I), or a pharmaceutically acceptable salt thereof, at a respective concentration of about 0.005 mg/ml to about 500 mg/ml. In embodiments, the pharmaceutical composition for parenteral administration includes Formula (I), or a pharmaceutically acceptable salt thereof, at a respective concentration of, e.g., about 0.05 mg/ml to about 50 mg/ml, about 0.1 mg/ml to about 50 mg/ml, about 0.1 mg/ml to about 10 mg/ml, about 0.05 mg/ml to about 25 mg/ml, about 0.05 mg/ml to about 10 mg/ml, about 0.05 mg/ml to about 5 mg/ml, or about 0.05 mg/ml to about 1 mg/ml. In embodiments, the pharmaceutical composition for parenteral administration includes Formula (I), or a pharmaceutically acceptable salt thereof, at a respective concentration of, e.g., about 0.05 mg/ml to about 15 mg/ml, about 0.5 mg/ml to about 10 mg/ml, about 0.25 mg/ml to about 5 mg/ml, about 0.5 mg/ml to about 7 mg/ml, about 1 mg/ml to about 10 mg/ml, about 5 mg/ml to about 10 mg/ml, or about 5 mg/ml to about 15 mg/ml.

[0396] In embodiments, a pharmaceutical composition for parenteral administration is provided wherein the pharmaceutical composition is stable for at least six months. In embodiments, the pharmaceutical compositions for parenteral administration exhibit no more than about 5% decrease in Formula (I), or a pharmaceutically acceptable salt thereof, e.g., 3 months or 6 months. In embodiments, the amount of Formula (I), or a pharmaceutically acceptable salt thereof, degrades at no more than about, e.g., 2.5%, 1%, 0.5% or 0.1%. In embodiments, the degradation is less than about, e.g., 5%, 2.5%, 1%, 0.5%, 0.25%, 0.1%, for at least six months.

[0397] In embodiments, pharmaceutical compositions for parenteral administration are provided wherein the pharmaceutical composition remains soluble. In embodiments, pharmaceutical compositions for parenteral administration are provided that are stable, soluble, local site compatible and/or ready-to-use. In embodiments, the pharmaceutical compositions herein are ready-to-use for direct administration to a subject in need thereof.

[0398] The pharmaceutical compositions for parenteral administration provided herein may include one or more excipients, e.g., solvents, solubility enhancers, suspending agents, buffering agents, isotonicity agents, stabilizers or antimicrobial preservatives. When used, the excipients of the parenteral compositions will not adversely affect the stability, bioavailability, safety, and/or efficacy of Formula (I), or a pharmaceutically acceptable salt thereof, used in the composition. Thus, parenteral compositions are provided wherein there is no incompatibility between any of the components of the dosage form.

[0399] In embodiments, parenteral compositions including (Formula (I), or a pharmaceutically acceptable salt thereof, include a stabilizing amount of at least one excipient. For example, excipients may be selected from the group consisting of buffering agents, solubilizing agents, tonicity agents, antioxidants, chelating agents, antimicrobial agents, and preservative. One skilled in the art will appreciate that an excipient may have more than one function and be classified in one or more defined group.

[0400] In embodiments, parenteral compositions including Formula (I), or a pharmaceutically acceptable salt thereof, and an excipient wherein the excipient is present at a weight percent (w/v) of less than about, e.g., 10%, 5%, 2.5%, 1%, or 0.5%. In embodiments, the excipient is present at a weight percent between about, e.g., 1.0% to 10%, 10% to 25%, 15% to 35%, 0.5% to 5%, 0.001% to 1%, 0.01% to 1%, 0.1% to 1%, or 0.5% to 1%. In embodiments, the excipient is present at a weight percent between about, e.g., 0.001% to 1%, 0.01% to 1%, 1.0% to 5%, 10% to 15%, or 1% to 15%.

[0401] In embodiments, parenteral compositions may be administered as needed, e.g., once, twice, thrice or four or more times daily, or continuously depending on the subject's needs.

[0402] In embodiments, parenteral compositions of Formula (I), or a pharmaceutically acceptable salt thereof, are provided, wherein the pH of the composition is between about 4.0 to about 8.0. In embodiments, the pH of the compositions is between, e.g., about 5.0 to about 8.0, about 6.0 to about 8.0, about 6.5 to about 8.0. In embodiments, the pH of the compositions is between, e.g., about 6.5 to about 7.5, about 7.0 to about 7.8, about 7.2 to about 7.8, or about 7.3 to about 7.6. In embodiments, the pH of the aqueous solution is, e.g., about 6.8, about 7.0, about 7.2, about 7.4, about 7.6, about 7.7, about 7.8, about 8.0, about 8.2, about 8.4, or about 8.6.

[0403] In embodiments, provided herein are methods of treating one or more synucleinopathies or one or more TDP-43 proteinopathies including administering to a subject in need thereof a pharmaceutical composition including Formula (I), or a pharmaceutically acceptable salt thereof, in a respective amount described herein, wherein the composition provides an in vivo plasma profile having a C.sub.max less than about 800 ng/ml. In embodiments, the composition provides improvement for more than 6 hours after administration to the subject.

[0404] In embodiments, pharmaceutical compositions including Formula (I), or a pharmaceutically acceptable salt thereof, provide an in vivo plasma profile of Formula (I), or a pharmaceutically acceptable salt thereof, having a C.sub.max less than about, e.g., 2000 ng/ml, 1000 ng/ml, 850 ng/ml, 800 ng/ml, 750 ng/ml, 700 ng/ml, 650 ng/ml, 600 ng/ml, 550 ng/ml, 450 ng/ml, 400 ng/ml, 350 ng/ml, or 300 ng/ml and wherein the composition provides improvement of next day functioning of the subject. In embodiments, the pharmaceutical composition provides an in vivo plasma profile of Formula (I), or a pharmaceutically acceptable salt thereof, having a C.sub.max less than about, e.g., 250 ng/ml, 200 ng/ml, 150 ng/ml, or 100 ng/ml and wherein the composition provides improvement of next day functioning of the subject. In embodiments, the pharmaceutical composition provides improvement in one or more symptoms of one or more synucleinopathies or

one or more TDP-43 proteinopathies herein for more than 6 hours after administration.

[0405] In embodiments, provided herein are methods of treating one or more synucleinopathies or one or more TDP-43 proteinopathies including administering to a subject in need thereof a pharmaceutical composition containing Formula (I), or a pharmaceutically acceptable salt thereof, wherein the composition provides a consistent in vivo plasma profile having a $AUC_{sub.0-\infty}$ of less than about 900 ng.Math.hr/ml. In embodiments, the pharmaceutical composition provides improvement in next day functioning of the subject. In embodiments, the compositions provide an in vivo plasma profile of Formula (I), or a pharmaceutically acceptable salt thereof, having a $AUC_{sub.0-\infty}$ of less than about, e.g., 850 ng.Math.hr/ml, 800 ng.Math.hr/ml, 750 ng.Math.hr/ml, or 700 ng.Math.hr/ml and wherein the pharmaceutical composition provides improvement of next day functioning of the subject. In embodiments, the composition provides improvement in one or more symptoms of one or more synucleinopathies or one or more TDP-43 proteinopathies herein for more than 6 hours after administration.

[0406] In embodiments, provided herein are methods of treating one or more synucleinopathies or one or more TDP-43 proteinopathies including administering to a subject in need thereof a pharmaceutical composition comprising Formula (I), or a pharmaceutically acceptable salt thereof, wherein the pharmaceutical composition provides an in vivo plasma profile of Formula (I), or a pharmaceutically acceptable salt thereof, having a $AUC_{sub.0-\infty}$ of less than about, e.g., 650 ng.Math.hr/ml, 600 ng.Math.hr/ml, 550 ng.Math.hr/ml, 500 ng.Math.hr/ml, or 450 ng.Math.hr/ml. In embodiments, the composition provides an in vivo plasma profile of Formula (I), or a pharmaceutically acceptable salt thereof, having a $AUC_{sub.0-\infty}$ of less than about, e.g., 400 ng.Math.hr/ml, 350 ng.Math.hr/ml, 300 ng.Math.hr/ml, 250 ng.Math.hr/ml, or 200 ng.Math.hr/ml. In embodiments, the pharmaceutical composition provides an in vivo plasma profile of Formula (I), or a pharmaceutically acceptable salt thereof, having a $AUC_{sub.0-\infty}$ of less than about, e.g., 150 ng.Math.hr/ml, 100 ng.Math.hr/ml, 75 ng.Math.hr/ml, or 50 ng.Math.hr/ml. In embodiments, the pharmaceutical composition provides improvement of next day functioning of the subject after administration for more than, e.g., 4 hours, 6 hours, 8 hours, 10 hours, or 12 hours, after administration of the composition to the subject.

[0407] In embodiments, the $T_{sub.max}$ provided by a pharmaceutical composition containing Formula (I), or a pharmaceutically acceptable salt thereof, is less than 3 hours. In embodiments, the $T_{sub.max}$ provided by the pharmaceutical composition containing Formula (I), or a pharmaceutically acceptable salt thereof, is less than 2.5 hours. In embodiments, the $T_{sub.max}$ provided by the pharmaceutical composition containing Formula (I), or a pharmaceutically acceptable salt thereof, is less than 2 hours. In embodiments, the $T_{sub.max}$ provided by the pharmaceutical composition containing Formula (I), or a pharmaceutically acceptable salt thereof, is less than 1.5 hours. In embodiments, the $T_{sub.max}$ provided by the pharmaceutical composition containing Formula (I), or a pharmaceutically acceptable salt thereof, is less than 1 hour. In embodiments, the $T_{sub.max}$ provided by the pharmaceutical composition containing Formula (I), or a pharmaceutically acceptable salt thereof, is less than 0.5 hour. In embodiments, the $T_{sub.max}$ provided by the pharmaceutical composition containing Formula (I), or a pharmaceutically acceptable salt thereof, is less than 0.25 hour.

[0408] In embodiments, the pharmaceutical composition containing Formula (I), or a pharmaceutically acceptable salt thereof, provides a dissolution of at least about 80% within the first 20 minutes of administration to a subject in need thereof. In embodiments, the pharmaceutical composition containing Formula (I), or a pharmaceutically acceptable salt thereof, provides a dissolution of at least about, e.g., 85%, 90% or 95% within the first 20 minutes of administration to a subject in need thereof. In embodiments, the pharmaceutical composition containing Formula (I), or a pharmaceutically acceptable salt thereof, provides a dissolution of at least 80% within the first 10 minutes of administration to a subject in need thereof.

[0409] In embodiments, provided herein are methods of treating one or more synucleinopathies or

one or more TDP-43 proteinopathies including administering to a subject in need thereof a first pharmaceutical dosage including a sub-therapeutic amount of Formula (I), or a pharmaceutically acceptable salt thereof. In embodiments, treating one or more synucleinopathies or one or more TDP-43 proteinopathies includes administering to a subject in need thereof a pharmaceutical composition containing Formula (I), or a pharmaceutically acceptable salt thereof, in a sub-therapeutic amount, wherein the composition provides improvement in one or more symptoms of the disorder for more than 6 hours after administration.

[0410] A sub-therapeutic dosage is an amount of Formula (I), or a pharmaceutically acceptable salt thereof, that is less than the amount typically required for a therapeutic effect. In embodiments, a sub-therapeutic dosage is an amount of Formula (I), or a pharmaceutically acceptable salt thereof, that alone may not provide improvement in at least one symptom of one or more synucleinopathies or one or more TDP-43 proteinopathies but is sufficient to maintain such improvement. In embodiments, the methods provide administering a first pharmaceutical composition containing an effective amount of Formula (I), or a pharmaceutically acceptable salt thereof, that provides improvement in at least one symptom of one or more synucleinopathies or one or more TDP-43 proteinopathies and a second composition containing a subtherapeutic amount of Formula (I), or a pharmaceutically acceptable salt thereof, that maintains the improvement. In embodiments, after administration of the first pharmaceutical composition, the second pharmaceutical composition may provide a synergistic effect to improve at least one symptom of one or more synucleinopathies or one or more TDP-43 proteinopathies.

[0411] In embodiments, provided herein are methods of treating one or more synucleinopathies or one or more TDP-43 proteinopathies including administering to a subject in need thereof a first pharmaceutical composition including a first pharmaceutical dosage of Formula (I), or a pharmaceutically acceptable salt thereof, wherein the first pharmaceutical dosage provides improvement for more than 6 hours after administration, and a second pharmaceutical composition including a sub-therapeutic dosage of Formula (I), or a pharmaceutically acceptable salt thereof.

[0412] In embodiments, the first or the second pharmaceutical composition are provided to the subject once in the evening and once in the morning. In embodiments, the total amount of Formula (I), or a pharmaceutically acceptable salt thereof, administered to a subject in a 24-hour period is any of the respective amounts described herein.

[0413] In embodiments, the first and/or the second pharmaceutical compositions may be provided with conventional release or modified release profiles. The first and second pharmaceutical compositions may be provided at the same time or separated by an interval of time, e.g., 1 hour, 2 hours, 3 hours, 4 hours, 5 hours, 6 hours, 12 hours, etc. In embodiments, the first and the second pharmaceutical compositions may be provided with different drug release profiles to create a two-phase release profile. For example, the first pharmaceutical composition may be provided with an immediate release profile, e.g., ODDF, parenteral, etc., and the second pharmaceutical composition may provide an extended release profile. In embodiments, one or both of the first and second pharmaceutical compositions may be provided with an extended release or delayed release profile. Such compositions may be provided as pulsatile formulations, multilayer tablets or capsules containing tablets, beads, granules, etc. In embodiments, the first pharmaceutical composition is an immediate release composition. In embodiments, the second pharmaceutical composition is an immediate release composition. In embodiments, the first and second pharmaceutical compositions are provided as separate immediate release compositions, e.g., film, tablets or capsules. In embodiments the first and second pharmaceutical compositions are provided 12 hours apart.

[0414] It should be understood that respective dosage amounts of (Formula (I), or a pharmaceutically acceptable salt thereof, that are provided herein are applicable to all the dosage forms described herein including conventional dosage forms, modified dosage forms, the first and second pharmaceutical compositions, as well as the parenteral formulations described herein. Those skilled in the art will determine appropriate amounts depending on criteria such as dosage

form, route of administration, subject tolerance, efficacy, therapeutic goal and therapeutic benefit, among other pharmaceutically acceptable criteria.

[0415] Combination therapies utilizing Formula (I), or a pharmaceutically acceptable salt thereof, can include administration of the active agents together in the same admixture, or in separate admixtures. In embodiments, the pharmaceutical composition can include two, three, or more active agents. In embodiments, the combinations result in a more than additive effect on the treatment of the disease or disorder. Thus, treatment is provided for one or more synucleinopathies or one or more TDP-43 proteinopathies with a combination of agents that combined, may provide a synergistic effect that enhances efficacy.

[0416] Unless defined otherwise, all technical and scientific terms used herein have the same meanings as commonly understood by one of skill in the art to which the disclosure herein belongs.

[0417] The term “about” or “approximately” as used herein means within an acceptable error range for the particular value as determined by one of ordinary skill in the art, which will depend in part on how the value is measured or determined, i.e., the limitations of the measurement system. For example, “about” can mean within 3 or more than 3 standard deviations, per the practice in the art. Alternatively, “about” can mean a range of up to 20%, up to 10%, up to 5%, and/or up to 1% of a given value. Alternatively, particularly with respect to biological systems or processes, the term can mean within an order of magnitude, preferably within 5-fold, and more preferably within 2-fold, of a value.

[0418] “Improvement” refers to the treatment of one or more synucleinopathies such as Parkinson's disease, Parkinson's disease with dementia, dementia, Lewy body dementia, pure autonomic failure, and multiple system atrophy, or one or more TDP-43 proteinopathies such as amyotrophic lateral sclerosis and frontotemporal dementia, or dementia from any cause, measured relative to at least one symptom of the foregoing disorders.

[0419] “Improvement in next day functioning” or “wherein there is improvement in next day functioning” refers to improvement after waking from an overnight sleep period wherein the beneficial effect of administration of (Formula (I), or a pharmaceutically acceptable salt thereof, applies to at least one symptom of a syndrome or disorder herein and is discernable, either subjectively by a subject or objectively by an observer, for a period of time, e.g., 2 hours, 3 hours, 4 hours, 5 hours, 6 hours, 12 hours, 24 hours, etc. after waking.

[0420] “PK” refers to the pharmacokinetic profile. $C_{sub,max}$ is defined as the highest plasma drug concentration estimated during an experiment (ng/ml). $T_{sub,max}$ is defined as the time when $C_{sub,max}$ is estimated (min). $AUC_{sub,0-\infty}$ is the total area under the plasma drug concentration-time curve, from drug administration until the drug is eliminated (ng.Math.hr/ml or $\mu\text{g.Math.hr/ml}$). The area under the curve is governed by clearance. Clearance is defined as the volume of blood or plasma that is totally cleared of its content of drug per unit time (ml/min).

[0421] “Pharmaceutically acceptable” refers to molecular entities and compositions that are “generally regarded as safe”, e.g., that are physiologically tolerable and do not typically produce an allergic or similar untoward reaction, such as gastric upset and the like, when administered to a human. In embodiments, this term refers to molecular entities and compositions approved by a regulatory agency of the federal or a state government, as the GRAS list under section 204(s) and 409 of the Federal Food, Drug and Cosmetic Act, that is subject to premarket review and approval by the FDA or similar lists, the U.S. Pharmacopeia or another generally recognized pharmacopeia for use in animals, and more particularly in humans.

[0422] “Effective amount” or “therapeutically effective amount”, previously referred to, can also mean a dosage sufficient to alleviate one or more symptoms of a syndrome, disorder, disease, or condition being treated, or to otherwise provide a desired pharmacological and/or physiologic effect. “Effective amount” or “therapeutically effective amount” may be used interchangeably herein.

[0423] “Co-administered with”, “administered in combination with”, “a combination of”, “in

combination with” or “administered along with” may be used interchangeably and mean that two or more agents are administered in the course of therapy. The agents may be administered together at the same time or separately in spaced apart intervals. The agents may be administered in a single dosage form or in separate dosage forms.

[0424] “Subject in need thereof” may include individuals that have been diagnosed with one or more synucleinopathies such as Parkinson's disease, Parkinson's disease with dementia, dementia, Lewy body dementia, pure autonomic failure, and multiple system atrophy, or one or more TDP-43 proteinopathies such as amyotrophic lateral sclerosis and frontotemporal dementia, or dementia from any cause. The methods may be provided to any individual including, e.g., wherein the subject is a neonate, infant, a pediatric subject (6 months to 12 years), an adolescent subject (age 12-18 years) or an adult (over 18 years). Subjects include mammals. “Subject” and “patient” are used interchangeably herein.

[0425] “Prodrug” refers to a pharmacological substance (drug) that is administered to a subject in an inactive (or significantly less active) form. Once administered, the prodrug is metabolized in the body (in vivo) into a compound having the desired pharmacological activity.

[0426] “Analog” and “Derivative” may be used interchangeably and refer to a compound that possesses the same core as the parent compound, but may differ from the parent compound in bond order, the absence or presence of one or more atoms and/or groups of atoms, and combinations thereof. The derivative can differ from the parent compound, for example, in one or more substituents present on the core, which may include one or more atoms, functional groups, or substructures. In general, a derivative can be imagined to be formed, at least theoretically, from the parent compound via chemical and/or physical processes.

[0427] The term “pharmaceutically acceptable salt”, as used herein, refers to derivatives of the compounds defined herein, wherein the parent compound is modified by making acid or base salts thereof. Example of pharmaceutically acceptable salts include but are not limited to mineral or organic acid salts of basic residues such as amines; and alkali or organic salts of acidic residues such as carboxylic acids. The pharmaceutically acceptable salts include the conventional non-toxic salts or the quaternary ammonium salts of the parent compound formed, for example, from non-toxic inorganic or organic acids. Such conventional non-toxic salts include but are not limited to those derived from inorganic acids such as hydrochloric, hydrobromic, sulfuric, sulfamic, phosphoric, and nitric acids; and the salts prepared from organic acids such as acetic, propionic, succinic, glycolic, stearic, lactic, malic, tartaric, citric, ascorbic, pantoic, maleic, hydroxymaleic, phenylacetic, glutamic, benzoic, salicylic, sulfanilic, 2-acetoxybenzoic, fumaric, toluenesulfonic, naphthalenesulfonic, methanesulfonic, ethane disulfonic, oxalic, and isethionic salts. The pharmaceutically acceptable salts can be synthesized from the parent compound, which contains a basic or acidic moiety, by conventional chemical methods.

EXAMPLES

[0428] The examples provided herein are included solely for augmenting the disclosure herein and should not be considered to be limiting in any respect.

[0429] The following Examples establish: Compound A is capable of directly binding to KCC2, as measured using cellular thermal shift assay (CETSA). Compound A potently increases KCC2 activity without modifying its plasma membrane stability or the phosphorylation of key regulatory sites, as measured by patch-clamp recording and TI flux. This contrasts to other indirect potentiators of KCC2, which have been shown to increase expression levels on the plasma membrane, enhance its expression, increase transcription of the SLC12A5 gene, or modify its phosphorylation (Gagnon et al., Nat. Med. 19, 1524-1528 (2013); Lee et al., Brain: a journal of neurology 145, 950-963 (2022); Prael Iii et al., Front Cell Dev Biol 10, 912812 (2022); Tang et al., Sci. Transl. Med. 11, 10.1126 (2019)). Consistent with its role in limiting neuronal excitability, Compound A reduced neuronal Cl_{sup}– accumulation and the development of late recurrent discharges (LRDs) in acute brain slices exposed to 0-Mg. Compound A rapidly accumulated in the

brain when dosed IV, IP, or SC to concentrations ranging from 40-0.6 μM , dependent upon the route of administration. Importantly, as measured in the open field, Compound A appeared to have no gross effects on mouse behavior.

[0430] The Examples establish that in mice treated with Compound A and diazepam, neuronal cell death in the hippocampus was significantly reduced compared to controls treated with diazepam alone. This effect may be due to the ability of Compound A to limit hyperexcitability, but previous in vitro and in vivo studies have shown that reducing KCC2 expression levels, or transiently inhibiting it, are sufficient to induce neuronal apoptosis. Likewise, modifying KCC2 expression levels during development also impacts neuronal viability.

Example 1

Identification of Small Molecules that Activate and Directly Bind to KCC2

[0431] A multi-tiered high-throughput screening assay was used, leveraging a proprietary library of 1.3 million small molecule compounds from AstraZeneca (Cambridge, UK) to identify chemical entities capable of activating KCC2. A single point concentration of 30 μM was used to identify potassium flux using an established thallium (TI⁺) influx FLIPR assay as the primary read out in HEK-293 cells overexpressing KCC2 (Cardarelli et al., Nat. Med. 23, 1394-1396 (2017); Conway et al., J Biol Chem 292, 21253-21263 (2017); Lee et al., 2022, supra). Compounds that exhibited >20% potentiation of KCC2 activity with EC₅₀ of <30 μM were selected for further optimization. This approach resulted in the identified subseries of fused pyrimidine compounds as KCC2 activators, which are denoted as subseries A (SSA; FIG. 1A).

[0432] Neuronal Cultures. Hippocampal neurons were prepared from Sprague Dawley E18 embryos and plated at 450,000 per dish and maintained at 37° C. in a humidified 5% CO₂ incubator for 10-25 days before experimentation (Lee et al., Nat. Neurosci. 14, 736-743 (2011)).

[0433] Cell Culture and biotinylation. HEK-293 cells were purchased from ATCC (CRL-1573) and transfected with human KCC2 and GFP expression plasmids using Lipofectamine 2000. Cells were utilized for experimentation 48-72 h following transfection. Cells were biotinylated at 4° C. using NHS-Biotin. Following lysis and purification on immobilized avidin beads, cell surface and total fractions were subject to immunoblotting as outlined previously (Conway et al., 2017, supra).

[0434] FLIPR TI Flux assay. HEK-293 cells were washed with HBSS, and then incubated with loading buffer containing TI sensitive fluorescent dye (<https://www.moleculardevices.com>), 20 μM bumetanide 20 μM ouabain and 2.5 mM probenecid for 1 h at room temperature (RT) under control conditions or with drugs in the dark using a FlexStation 3 microplate reader (Molecular Devices) and imaged 525 nm for 30 min. Stimulus buffer was added containing 5 mM K⁺ and 0.5 mM Tl⁺ and cells were imaged every 1.5 s over a 190 second time course. The initial rate of Tl⁺ uptake (Relative Fluorescence Units (RFU) per second) was determined following addition of the stimulus buffer as outlined previously (Conway et al., 2017, supra) (Lee et al., 2022, supra).

Example 2

Identification of Compound A

[0435] Determining if compounds in SSA directly interact with KCC2. This was tested using a cellular thermal shift assay (CETSA), which measures the ability of a drug to modify the thermal stability of its target in whole cells (Kawatkar et al., ACS Chem. Biol. 14, 1913-1920 (2019); Martinez et al., Sci Rep 8, 9472 (2018)).

[0436] 2×10⁶ transfected HEK-293 cells were incubated with 30 μM drug or vehicle for 90 min at 37° C. and then heated from 37-58° C. for 5 min. Cells were rapidly chilled to 25° C. and NP40 was added to a final concentration of 0.5% prior to snap freezing in liquid nitrogen. Following 5 freeze/thaw cycles lysates were centrifuged at 12,000×g to remove denatured proteins and associated cell debris. The supernatant cell and tissue lysate samples, or immunoprecipitated KCC2 samples were separated by SDS-PAGE and subsequently immunoblotted with KCC2 antibodies (1:2000, Millipore #07-432), KCC2-S940, KCC2-pT1007 (1:2000, PhosphoSolutions, #1568 actin and GAPDH (1:3000, SantaCruz #sc-32233) as detailed in previous studies (Lee et al., 2011, supra).

(Conway et al., 2017, supra), and the level of soluble KCC2 at each temperature was normalized to that seen at 37° C. This data was then used to determine the temperature at which 50% of KCC2 was denatured.

[0437] Aliquots of HEK-293 cells expressing KCC2 were exposed to 30 μ M SSA, heated from 37-55° C. The remaining level of soluble native KCC2 at each temperature was measured by immunoblotting and normalized to the level seen at 37° C. which was given a value of 1. SSA-1 reduced the thermal stability of KCC2 compared to vehicle, which demonstrated target engagement. To gain insights into affinity, isothermal dose response analysis was performed at 50° C. revealing a K_{sub}.D of 395.6 $\pm$ 23.5 nM of SSA for KCC2 (FIG. 1B-D). SSA derivatives were further optimized for affinity, optimal drug like properties and predicted brain penetration by in vitro cellular uptake assays, which resulted in the identification of Compound A (FIG. 1E). Dose response measurements with Compound A were then made at 50° C. and this data was used to estimate its relative affinity for KCC2 (Kawatkar et al., 2019, supra) (Martinez et al., 2018, supra). To control for compounds that may indirectly modulate KCC2 activity, via modifying its phosphorylation (Kahle et al., Trends Neurosci. 36, 726-737 (2013); Moore et al., Proc. Natl. Acad. Sci. U.S.A. 115, 10166-10171 (2018); Silayeva et al., Proc. Natl. Acad. Sci. U.S.A. 112, 3523-3528 (2015); Smalley et al., Front. Mol. Neurosci. 13, 563091 (2020)), the effect of Compound A to modify the activity of a panel of 144 protein kinases (SelectScreen; Thermofisher) was studied. This panel included protein kinase C isoforms, OSR1, SPAK (STK39) and with-no-lysine kinases (WNK), all of which phosphorylate KCC2 on multiple residues to indirectly modify its activity (Kahle et al., 2013, supra; Smalley et al., 2020, supra). Compound A did not modify the activity of this kinase panel.

[0438] As measured by TI⁺ flux, Compound A exhibited an EC_{sub}.50 for KCC2 of 261.4 $\pm$ 22.2 nM and a maximum potentiation of 90% at a saturating concentration of 3 μ M using a 1% β -cyclodextrin (BCD) vehicle (FIG. 1F). Several studies have shown that the activity and plasma membrane accumulation of KCC2 is potentiated via phosphorylation of S940, a process mediated by the activity of protein kinase C and protein phosphatase 1 (Lee et al., 2011, supra; Lee et al., J Biol Chem 282, 29777-29784 (2007); Moore et al., Proc. Natl. Acad. Sci. U.S.A. 115, 10166-10171 (2018); Moore et al., Trends Neurosci. 40, 555-571 (2017); Silayeva et al., Proc. Natl. Acad. Sci. U.S.A. 112, 3523-3528 (2015)). The ability of Compound A to modulate the activity of a KCC2 construct was tested in which this key residue for phospho-dependent modulation of KCC2 was mutated to an alanine (S940A). Significantly, Compound A increased the activity of KCC2-S940A construct (FIG. 1G). Finally, the specificity of Compound A for other solute transporters was examined using the TI⁺ assay using HEK-293 cells expressing KCC3, KCC4, or NKCC1. Compared to KCC2, effects on KCC3, KCC4, and NKCC1 were only seen at concentrations more than 35-fold that of KCC2 (>100 μ M) (FIG. 1G).

[0439] These results suggest that Compound A binds directly to KCC2 and acts to increase its activity with minimal effects on other solute transporters that regulate neuronal Cl_{sup}.– accumulation.

Example 3

Compound A Potentiates KCC2 Activity without Modifying its Plasma Membrane Accumulation or Phosphorylation

[0440] To confirm the validity of our measurements using TI⁺ flux, KCC2 was expressed in HEK-293 cells with the α 1 subunit of the glycine receptor (GlyR α 1) which forms homomeric glycine-activated ion channels when expressed in this system. The gramicidin perforated patch clamp technique was used to measure the reversal potential for GlyR α 1-mediated currents (E_{sub}.GLY) in cells exposed to Compound A. The use of this technique, while limited in throughput, facilitates high resolution single-cell recordings without disturbing endogenous intracellular Cl_{sup}.– levels (Cardarelli et al., 2017, supra; Conway et al., 2017, supra). To calculate E_{sub}.GLY, cells were exposed to 50 μ M Gly and the polarity of Gly-induced currents was determined at different holding

potentials. Brief (15 min) incubation with either 0.3 μ M (approx. EC₅₀) or 3 μ M Compound A induced shifts of -15.6 ± 4.5 (p=0.022), and -32.2 ± 9.5 (p=0.001) mV in E_{sub.GLY} respectively, while vehicle was without effect (-0.8 ± 0.5 mv; p=0.512) (FIG. 2A-B). As determined using the Nernst equation, these negative shifts in E_{sub.GLY} by Compound A corresponded to reduced intracellular Cl_{sup.-} levels of FIG. 2B; Compound A; 5.2 ± 1.2 ; vehicle= 7.9 ± 1.2 mM; p=0.0104). [0441] The effects of Compound A on the plasma membrane stability of KCC2 expression was assessed using biotinylation, as described previously (Cardarelli et al., 2017, supra; Conway et al., 2017, supra). Compared to vehicle, 0.3 μ M or 3 μ M Compound A did not modify the level of KCC2 on the plasma membrane (FIG. 2C; $94.5 \pm 8.4\%$, p=0.745 and $104.7 \pm 8.4\%$ of control; p=0.00736 respectively). The reliability of the biotinylation procedure was assessed via immunoblotting with antibodies against the cytosolic protein actin. Actin was present in the total lysates but was absent from the surface fractions (FIG. 2C).

[0442] The activity of KCC2 is subject to dynamic modulation via phosphorylation of critical intracellular residues within its C-terminal cytoplasmic domain. Central to this process are serine 940 (S940) and threonine 1007 (T1007), which activate and inhibit KCC2 activity respectively (Moore et al., 2017, supra). Therefore, the effects of Compound A on the phosphorylation of these residues was examined using immunoblotting with characterized phospho-specific antibodies pS940 and pT1007, respectively. Compound A (3 μ M) did not significantly modify phosphorylation of S940 ($95.3 \pm 10.2\%$ of control; p=0.764), or T1007 ($108 \pm 10.2\%$ of control; p=0.649) (FIG. 2D).

[0443] The foregoing suggests that Compound A potentiates KCC2 activity without modifying either its plasma membrane accumulation or the phosphorylation of key regulatory sites.

Example 4

Compound A Reduces Neuronal Cl_{SUP.-} Accumulation In Vitro

[0444] To test if Compound A modifies neuronal Cl_{sup.-} levels gramicidin-perforated patch-clamp recording was used to measure the reversal potentials of GABA_{sub.A} receptor mediated currents (E_{sub.GABA}) in 18-20 Div hippocampal neurons (Lee et al., 2011, supra; Moore et al., 2018, supra). At this developmental stage, GABA_{sub.AR} receptor activation leads to hyperpolarization, a phenomenon critically dependent upon KCC2 (Kontou et al., J Biol Chem, 100364 (2021)).

[0445] Neuronal Patch clamp assays. Neuronal cell culture recordings were performed in bath saline at 34° C. For gramicidin perforated-patch experiments, pipettes contained (in mM): 140 KCl and 10 HEPES, pH 7.4 KOH. For whole-cell experiments, pipettes contained (in mM): 115 K-meth-SO_{sub.4}, 30 KCl, 2 Mg-ATP, 4 Na-ATP, 0.4 Na-GTP, and 10 HEPES, pH 7.4 KOH. Bath saline contained the following (in mM): 140 NaCl, 2.5 KCl, 2.5 CaCl_{sub.2}, 2.5 MgCl_{sub.2}, 10 HEPES, and 11 glucose, pH 7.4 NaOH (Kontou et al., 2021, supra). All compound solutions were applied to cells using a three-barreled 700 μ m pipe positioned just above the cell (Warner Instruments). Compound A was applied in 1 mg/mL 2-Hydroxypropyl- β -cyclodextrin, and its effects were compared to this vehicle alone. 10 mV voltage-ramp protocols over 1-s periods were used to determine the reversal potentials of the leak-subtracted muscimol-activated GABA_{sub.AR} currents. All voltages from whole-cell experiments were corrected offline using a calculated liquid junction potential value (13.6 mV) in Clampex (Molecular Devices). For gramicidin perforated-patch experiments, intracellular Cl_{sup.-} values were back-calculated using measured E_{sub.GABA} values and the Nernst equation:

$$[00001] E_{\text{ion}} = \frac{RT}{zF} \ln \frac{[\text{ion}]_o}{[\text{ion}]_i}$$

where E is the ion's reversal potential, R is the universal gas constant, T is temperature in kelvins, F is Faraday's constant, z is the charge of the ion, [chloride]_{sub.i} is the intracellular chloride concentration and [chloride]_{sub.o} is the extracellular ion concentration.

[0446] Measurements were performed in the presence of bumetanide (10 μ M) and tetrodotoxin

(TTX; 300 nM) to limit the contributions of NKCC1 and activity dependent shifts in chloride levels, respectively (Lee et al., 2011, supra). Cultures were exposed to Compound A or vehicle for 15 min and voltage ramps were used to determine changes in the reversal potential of GABA_{sub}.AR receptor currents over time (FIG. 3A). These data were then used to determine shifts subsequently, using [Cl_{sup}.-] levels. At 15 min 300 nM Compound A (an approximately EC_{sub}.50 concentration) significantly reduced E_{sub}.GABA from -75 ± 3.1 to -85.1 ± 4.2 mV (FIG. 3B; $p=0.0104$), a net -9.1 ± 2.2 mV negative shift, reflecting a 2.2 ± 0.6 mM reduction in Cl_{sup}.- (FIG. 3C; $p=0.0153$). In contrast, vehicle did not significantly modify E_{sub}.GABA (FIG. 3B; $0=-77.2 \pm 3.3$, $15=-78.0 \pm 4.3$ mV respectively, $p=0.580$) or chloride levels.

[0447] To measure the effects of Compound A on KCC2 activity under more dynamic conditions cultures in Compound A were incubated for 1 h and subjected to whole cell patch-clamp recording to artificially impose a 32 mM Cl_{sup}.- load on neurons via the patch pipette (Lee et al., 2022, supra; Moore et al., 2018, supra). 5 min after break-in, an initial E_{sub}.GABA value was determined and the effects of a subsequent 5 min incubation with VU0463271 was assessed. Compound A-treated cells exhibited lower basal E_{sub}.GABA values compared to controls (FIG. 3C; FIG. 3D -59.3 ± 2.1 and -47.4 ± 2.3 mV, respectively, $p=0.025$). Likewise, VU0463271 induced shifts in E_{sub}.GABA were larger for cultures treated with Compound A (FIG. 3C; FIG. 3D 17.5 ± 4.6 and 5.4 ± 4.2 mV, respectively, $p=0.009$).

[0448] Thus, Compound A reduces neuronal Cl_{sup}.- levels and slows the development of hyperexcitability in an ex vivo system.

Example 5

Compound A Rapidly Accumulates in the Brain In Vivo

[0449] PK studies. Mice were injected intraperitoneally (IP), intravenously (IV), or subcutaneously (SC), with Compound A and 5% BCD in a maximum volume of 300 μ l. Plasma and brain samples were rapidly frozen on dry ice. Tissue extracts were treated with Acetonitrile and Compound A levels were quantified using LC-MS/MS (Integrated Analytical Solutions, Inc, Berkeley, CA 94710; www.ianalytical.net).

[0450] To examine the ability of Compound A to cross the blood brain barrier, mice were injected with a single 25 mg/kg IV bolus; accumulation in the brain was evident within 30 minutes min in the brain and plasma to 7.6 ± 4.4 and 41.5 ± 11.5 and 11.2 μ M respectively (FIG. 4A). Likewise, a 50 mg/kg IP injection resulted in the accumulation of Compound A in the brain to 2.67 ± 0.21 and 4.1 ± 0.13 μ M 2 and 4 hr later (FIG. 4B). Drug accumulation in the brain was examined following SC injection (FIG. 4C; 50 mg/kg). Accumulation was detected at 30 min and reached a maximal concentration of 675.75 ± 80.5 nM at 4 hr, a level that was maintained 8 h later (FIG. 4C).

[0451] Given the ability of Compound A to accumulate in the brain, it was examined to see if it also causes any gross effects on animal behavior. Mice were injected with Compound A (SC; 50 mg/kg), or vehicle and their behavior was compared in the open field test (Tretter et al., Proc. Natl. Acad. Sci. U.S.A. 106, 20039-20044 (2009); Vien et al., Frontiers in molecular neuroscience 15, 817996 (2022)). Compound A did not modify the total distance traveled during the test trial (FIG. 4D; Compound A= 6134.2 ± 235.3 cm, and vehicle= 6453.7 ± 215.4 cm; $p=0.253$, $n=11$ mice). Likewise, time spent in the center zone was comparable (FIG. 4D; Compound A= 18.4 ± 2.1 , and vehicle 16.6 ± 2.5 % of total time; $p=0.658$, $n=11$ mice).

[0452] Collectively, these results suggest that Compound A accumulates in the brain when dosed IV, IP or SC, and does not lead to any stereotyped effects on behavior.

[0453] It should be understood that the examples and embodiments provided herein are exemplary examples and embodiments. Those skilled in the art will envision various modifications of the examples and embodiments that are consistent with the scope of the disclosure herein. Such modifications are intended to be encompassed by the claims.

Claims

1. A method of treating a synucleinopathy comprising administering a compound according to Formula (I), ##STR00037## or a pharmaceutically acceptable salt thereof, wherein: R^{sup.1} is selected from C_{sub.2-6}alkyl; C_{sub.2-6}alkenyl; C_{sub.2-6}alkynyl; C_{sub.2-6}alkoxy; C_{sub.2-6}alkenyloxy; C_{sub.2-6}alkynyloxy; C_{sub.3-7}cycloalkyl; —O—C_{sub.3-7}cycloalkyl; C_{sub.6-10}aryl; —O—(CH_{sub.2})_{sub.m}—C_{sub.6-10}aryl; 6 membered heteroaryl; and thiophenyl; wherein alkyl, alkenyl, alkynyl, alkoxy, alkenyloxy, alkynyloxy and cycloalkyl are optionally substituted with 1, 2 or 3 substituents selected from —F and —CF_{sub.3} and wherein aryl and heteroaryl are optionally substituted with 1 or 2 substituents selected from -halo, —C_{sub.1-3}alkyl, —C_{sub.1-8}alkoxy and —C_{sub.2-8}alkynyloxy wherein —C_{sub.1-3}alkyl, —C_{sub.1-8}alkoxy and —C_{sub.2-8}alkynyloxy are optionally substituted with 1, 2, or 3 substituents selected from —F, —CF_{sub.3}, —NHC(O)O—C_{sub.1-6}alkyl or two substituents together with the carbon to which they are attached form diazirinyl; R^{sup.2} is selected from —H; -halo; and —C_{sub.1-3}alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and —CF_{sub.3}; A is selected from ##STR00038## or a N-oxide thereof; R^{sup.3} is selected from —H; —C_{sub.1-6}alkyl; —C_{sub.2-6}alkenyl; —C_{sub.2-6}alkynyl; C_{sub.3-7}cycloalkyl; and a 5 or 6 membered heterocycloalkyl; wherein the alkyl, alkenyl, alkynyl, cycloalkyl or heterocycloalkyl are optionally substituted by 1, 2 or 3 groups selected from —F, —CF_{sub.3}, —C_{sub.1-3}alkyl optionally substituted by 1 or 2 substituents selected from —F, —CF_{sub.3}, —C(O)NR^{sup.8}R^{sup.9} and —NR^{sup.8}R^{sup.9}; R^{sup.4a} and R^{sup.4b} are each independently selected from —H and —C_{sub.1-3}alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and CF_{sub.3}; R^{sup.4c} and R^{sup.4d} are each independently selected from —H and —C_{sub.1-3}alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and CF_{sub.3}, or R^{sup.4c} and R^{sup.4d} together with the carbon to which they are attached represent carbonyl; R^{sup.5a}, R^{sup.5b}, R^{sup.5c} and R^{sup.5d} are each independently selected from —H and —C_{sub.1-3}alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and CF_{sub.3}; R^{sup.6} is selected from —H; -halo; —NH_{sub.2}; —CN; —C_{sub.1-3}alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and CF_{sub.3}; —C_{sub.1-3}alkoxy optionally substituted with 1, 2 or 3 substituents selected from —F and —CF_{sub.3}; —C(O)O—C_{sub.1-3}alkyl; —C(O)NR^{sup.8}R^{sup.9}; —C(O)OH; and —NHC(O)—C_{sub.1-3}alkyl; R^{sup.7} is selected from NR^{sup.10}R^{sup.11}; a 5 to 7 membered monocyclic heterocycloalkyl; and a 5 or 6 membered monocyclic heteroaryl; wherein the heterocycloalkyl and heteroaryl are optionally substituted with 1, 2 or 3 groups selected from —CN; —C_{sub.1-6}alkyl optionally substituted with 1, 2 or 3 substituents selected from —F, —CF_{sub.3} and —OH; —C_{sub.1-3}alkoxy optionally substituted with 1, 2 or 3 substituents selected from —F and CF_{sub.3}; —C(O)OH; —C_{sub.1-3}alkylene-NHC(O)C_{sub.1-6}alkyl; —C_{sub.1-3}alkylene-NHC(O)OC_{sub.1-6}alkyl; C_{sub.3-5}cycloalkyl; or the heterocycloalkyl is optionally substituted with two substituents on the same ring carbon which together with the carbon atom to which they are attached form a 5 to 7 membered monocyclic heterocycloalkyl; and wherein when R^{sup.7} is morpholinyl and R^{sup.1} is unsubstituted phenyl, R^{sup.2} is not —H; R^{sup.8} and R^{sup.9} are each independently selected from —H and —C_{sub.1-6}alkyl; R^{sup.10} is —C_{sub.1-6}alkyl; R^{sup.11} is selected from —C_{sub.1-6}alkyl optionally substituted with 1 or 2 substituents selected from —F and —C_{sub.1-3}alkoxy; and —(CH_{sub.2})_{sub.n}R^{sup.12}; R^{sup.12} is a 5 or 6 membered heteroaryl, a 3 to 5 membered cycloalkyl or a 3 to 6 membered heterocycloalkyl; m is 0 or 1; and n is 1, 2 or 3, to a subject diagnosed with the synucleinopathy in an amount of from about 0.01 mg to about 1500 mg.
2. The method of treating a synucleinopathy according to claim 1, wherein the total amount of a compound according to Formula (I), or a pharmaceutically acceptable salt thereof, administered to the subject in a twenty-four hour period is between about 1 mg and about 1000 mg.

3. The method of treating a synucleinopathy according to claim 1, wherein the synucleinopathy is selected from the group consisting of Parkinson's disease, Parkinson's disease with dementia, dementia, Lewy body dementia, pure autonomic failure, and multiple system atrophy.
4. The method of treating a synucleinopathy according to claim 1, wherein the subject has been diagnosed with Parkinson's disease and the method provides improvement in at least one symptom selected from the group consisting of tremors in the resting muscles, rigidity, slowness of movement, impaired balance, a shuffling gait, behavioral changes, cognitive impairment, tremor in hands, arms, legs, jaw, or head, muscle stiffness, slowness of movement, impaired balance and coordination, depression, difficulty swallowing, difficulty chewing, difficulty speaking, urinary problems, constipation, reduced or lack of facial expression and animation, parkinsonian gait, leaning forward, taking small, quick steps, and reduced arm swinging.
5. The method of treating a synucleinopathy according to claim 1, wherein the subject has been diagnosed with Parkinson's disease with dementia and the method provides improvement in at least one symptom selected from the group consisting of tremors in the resting muscles, rigidity, slowness of movement, impaired balance, a shuffling gait, behavioral changes, cognitive impairment, tremor in hands, arms, legs, jaw, or head, muscle stiffness, slowness of movement, impaired balance and coordination, depression, difficulty swallowing, difficulty chewing, difficulty speaking, urinary problems, constipation, reduced or lack of facial expression and animation, parkinsonian gait, leaning forward, taking small, quick steps, reduced arm swinging, and dementia.
6. The method of treating a synucleinopathy according to claim 1, wherein the subject has been diagnosed with Lewy body dementia and the method provides improvement in at least one symptom selected from the group consisting of visual hallucinations, seeing things that are not present, nonvisual hallucinations, hearing or smelling things that are not present, cognitive problems including unpredictable changes in concentration, attention, alertness, and wakefulness, disorganized, unclear, or illogical ideas, loss of thinking abilities, memory loss, poor judgment, confusion about time and place, difficulty with language and numbers, ataxia, muscle rigidity or stiffness, shuffling walk, slow movement, frozen stance, tremor, shaking, balance problems and repeated falls, stooped posture, loss of coordination, smaller handwriting, reduced facial expression, difficulty swallowing, weak voice, insomnia, REM sleep behavior disorder, restless leg syndrome, depression, apathy, anxiety, agitation, aggression, paranoia, diminished regulation of body temperature, diminished regulation of blood pressure, dizziness, syncope, sensitivity to heat and cold, sexual dysfunction, urinary incontinence, constipation, and poor sense of smell.
7. The method of treating a synucleinopathy according to claim 1, wherein the subject has been diagnosed with pure autonomic failure and the method provides improvement in at least one symptom selected from the group consisting of orthostatic hypotension, dizziness, faintness, syncope, light-headedness, visual disturbances, neck pain, breathing difficulty, abnormal sweating, anhidrosis, hypohidrosis, heat intolerance, night sweats, poor gastric motility, fatigue, weakness, sexual dysfunction, impotence, sleep disorders, constipation, frequent urination, incontinence, and frequent urinary infections.
8. The method of treating a synucleinopathy according to claim 1, wherein the subject has been diagnosed with multiple system atrophy and the method provides improvement in at least one symptom selected from the group consisting of slowness of movement, tremor, rigidity, stiffness, clumsiness, coordination problems, impaired speech, hoarseness, orthostatic hypotension, bladder control problems, incontinence, urinary retention, muscle contractures, abnormal posture, bending of the neck, involuntary sighing, insomnia, emotional problems, breathing problems, sleep apnea, irregular heart rhythms, tremor, problems of balance, autonomic nervous system dysfunction, difficulty swallowing, difficulty speaking, and abnormal eye movements.
9. The method of treating a synucleinopathy according to claim 1, wherein the compound according to Formula (I), or a pharmaceutically acceptable salt thereof, is Compound A:
- ##STR00039##

10. The method of treating a synucleinopathy according to claim 1, wherein the compound according to Formula (I), or a pharmaceutically acceptable salt thereof, is Compound B:
##STR00040##

11. The method of treating a synucleinopathy according to claim 1, wherein the compound according to Formula (I), or a pharmaceutically acceptable salt thereof, is Compound C:
##STR00041##

12. The method of treating a synucleinopathy according to claim 1, wherein the compound according to Formula (I), or a pharmaceutically acceptable salt thereof, is Compound D:
##STR00042##

13. The method of treating a synucleinopathy according to claim 1, wherein the compound according to Formula (I), or a pharmaceutically acceptable salt thereof, is Compound E:
##STR00043##

14. The method of treating a synucleinopathy according to claim 1, wherein the compound according to Formula (I), or a pharmaceutically acceptable salt thereof, is Compound F:
##STR00044##

15. The method of treating a synucleinopathy according to claim 1, wherein the compound according to Formula (I), or a pharmaceutically acceptable salt thereof, is Compound G:
##STR00045##

16. The method of treating a synucleinopathy according to claim 1, wherein the compound according to Formula (I), or a pharmaceutically acceptable salt thereof, is Compound H:
##STR00046##

17. The method of treating a synucleinopathy according to claim 1, wherein the compound according to Formula (I), or a pharmaceutically acceptable salt thereof, is administered enterally.

18. The method of treating a synucleinopathy according to claim 17, wherein the compound according to Formula (I), or a pharmaceutically acceptable salt thereof, is administered orally, sublingually, buccally, transdermally or rectally.

19. The method of treating a synucleinopathy according to claim 1, wherein the compound according to Formula (I), or a pharmaceutically acceptable salt thereof, is administered parenterally.

20. The method of treating a synucleinopathy according to claim 1, wherein the compound according to Formula (I), or a pharmaceutically acceptable salt thereof, is administered in the form of a liquid for parenteral use, a tablet, a capsule, a caplet, a pill, an oral liquid, a lozenge, a film, a powder, an aerosol, or a patch.

21. The method of treating a synucleinopathy according to claim 1, wherein the subject is a human subject.

22. A method of treating a TD-43 proteinopathy comprising administering a compound according to Formula (I), ##STR00047## or a pharmaceutically acceptable salt thereof, wherein: R^{sup.1} is selected from C_{sub.2-6}alkyl; C_{sub.2-6}alkenyl; C_{sub.2-6}alkynyl; C_{sub.2-6}alkoxy; C_{sub.2-6}alkenyloxy; C_{sub.2-6}alkynyloxy; C_{sub.3-7}cycloalkyl; —O—C_{sub.3-7}cycloalkyl; C_{sub.6-10}aryl; —O—(CH_{sub.2})_{sub.m}—C_{sub.6-10}aryl; 6 membered heteroaryl; and thiophenyl; wherein alkyl, alkenyl, alkynyl, alkoxy, alkenyloxy, alkynyloxy and cycloalkyl are optionally substituted with 1, 2 or 3 substituents selected from —F and —CF_{sub.3} and wherein aryl and heteroaryl are optionally substituted with 1 or 2 substituents selected from -halo, —C_{sub.1-3}alkyl, —C_{sub.1-8}alkoxy and —C_{sub.2-8}alkynyloxy wherein —C_{sub.1-3}alkyl, —C_{sub.1-8}alkoxy and —C_{sub.2-8}alkynyloxy are optionally substituted with 1, 2, or 3 substituents selected from —F, —CF_{sub.3}, —NHC(O)O—C_{sub.1-6}alkyl or two substituents together with the carbon to which they are attached form diazirinyl; R^{sup.2} is selected from —H; -halo; and —C_{sub.1-3}alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and —CF_{sub.3}; A is selected from ##STR00048## or a N-oxide thereof; R^{sup.3} is selected from —H; —C_{sub.1-6}alkyl; —C_{sub.2-6}alkenyl; —C_{sub.2-6}alkynyl; C_{sub.3-7}cycloalkyl; and a 5 or 6 membered

heterocycloalkyl; wherein the alkyl, alkenyl, alkynyl, cycloalkyl or heterocycloalkyl are optionally substituted by 1, 2 or 3 groups selected from —F, —CF.sub.3, —C.sub.1-3alkyl optionally substituted by 1 or 2 substituents selected from —F, —CF.sub.3, —C(O)NR.sup.8R.sup.9 and —NR.sup.8R.sup.9; R.sup.4a and R.sup.4b are each independently selected from —H and —C.sub.1-3 alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and CF.sub.3; R.sup.4c and R.sup.4d are each independently selected from —H and —C.sub.1-3 alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and CF.sub.3, or R.sup.4c and R.sup.4d together with the carbon to which they are attached represent carbonyl; R.sup.5a, R.sup.5b, R.sup.5c and R.sup.5d are each independently selected from —H and —C.sub.1-3 alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and CF.sub.3; R.sup.6 is selected from —H; -halo; —NH.sub.2; —CN; —C.sub.1-3alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and CF.sub.3; —C.sub.1-3alkoxy optionally substituted with 1, 2 or 3 substituents selected from —F and —CF.sub.3; —C(O)O—C.sub.1-3alkyl; —C(O)NR.sup.8R.sup.9; —C(O)OH; and —NHC(O)—C.sub.1-3 alkyl; R.sup.7 is selected from NR.sup.10R.sup.11; a 5 to 7 membered monocyclic heterocycloalkyl; and a 5 or 6 membered monocyclic heteroaryl; wherein the heterocycloalkyl and heteroaryl are optionally substituted with 1, 2 or 3 groups selected from —CN; —C.sub.1-6alkyl optionally substituted with 1, 2 or 3 substituents selected from —F, —CF.sub.3 and —OH; —C.sub.1-3alkoxy optionally substituted with 1, 2 or 3 substituents selected from —F and CF.sub.3; —C(O)OH; —C.sub.1-3alkylene-NHC(O)C.sub.1-6alkyl; —C.sub.1-3alkylene-NHC(O)OC.sub.1-6alkyl; C.sub.3-5cycloalkyl; or the heterocycloalkyl is optionally substituted with two substituents on the same ring carbon which together with the carbon atom to which they are attached form a 5 to 7 membered monocyclic heterocycloalkyl; and wherein when R.sup.7 is morpholinyl and R.sup.1 is unsubstituted phenyl, R.sup.2 is not —H; R.sup.8 and R.sup.9 are each independently selected from —H and —C.sub.1-6 alkyl; R.sup.10 is —C.sub.1-6 alkyl; R.sup.11 is selected from —C.sub.1-6alkyl optionally substituted with 1 or 2 substituents selected from —F and —C.sub.1-3alkoxy; and —(CH.sub.2).sub.nR.sup.12; R.sup.12 is a 5 or 6 membered heteroaryl, a 3 to 5 membered cycloalkyl or a 3 to 6 membered heterocycloalkyl; m is 0 or 1; and n is 1, 2 or 3, to a subject diagnosed with the TD-43 proteinopathy in an amount of from about 0.01 mg to about 1500 mg.

23. A method of treating dementia comprising administering a compound according to Formula (I), ##STR00049## or a pharmaceutically acceptable salt thereof, wherein: R.sup.1 is selected from C.sub.2-6alkyl; C.sub.2-6alkenyl; C.sub.2-6alkynyl; C.sub.2-6alkoxy; C.sub.2-6alkenyloxy; C.sub.2-6alkynyloxy; C.sub.3-7cycloalkyl; —O—C.sub.3-7cycloalkyl; C.sub.6-10aryl; —O—(CH.sub.2).sub.m—C.sub.6-10aryl; 6 membered heteroaryl; and thiophenyl; wherein alkyl, alkenyl, alkynyl, alkoxy, alkenyloxy, alkynyloxy and cycloalkyl are optionally substituted with 1, 2 or 3 substituents selected from —F and —CF.sub.3 and wherein aryl and heteroaryl are optionally substituted with 1 or 2 substituents selected from -halo, —C.sub.1-3alkyl, —C.sub.1-8alkoxy and —C.sub.2-8alkynyloxy wherein —C.sub.1-3alkyl, —C.sub.1-8alkoxy and —C.sub.2-8alkynyloxy are optionally substituted with 1, 2, or 3 substituents selected from —F, —CF.sub.3, —NHC(O)O—C.sub.1-6alkyl or two substituents together with the carbon to which they are attached form diazirinyl; R.sup.2 is selected from —H; -halo; and —C.sub.1-3alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and —CF.sub.3; A is selected from ##STR00050## R.sup.3 is selected from —H; —C.sub.1-6alkyl; —C.sub.2-6alkenyl; —C.sub.2-6alkynyl; C.sub.3-7cycloalkyl; and a 5 or 6 membered heterocycloalkyl; wherein the alkyl, alkenyl, alkynyl, cycloalkyl or heterocycloalkyl are optionally substituted by 1, 2 or 3 groups selected from —F, —CF.sub.3, —C.sub.1-3alkyl optionally substituted by 1 or 2 substituents selected from —F, —CF.sub.3, —C(O)NR.sup.8R.sup.9 and —NR.sup.8R.sup.9; R.sup.4a and R.sup.4b are each independently selected from —H and —C.sub.1-3 alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and CF.sub.3; R.sup.4c and R.sup.4d are each independently selected from —H and —C.sub.1-3 alkyl optionally substituted with 1, 2 or 3 substituents selected

from —F and CF.sub.3, or R.sup.4c and R.sup.4d together with the carbon to which they are attached represent carbonyl; R.sup.5a, R.sup.5b, R.sup.5c and R.sup.5d are each independently selected from —H and —C.sub.1-3 alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and CF.sub.3; R.sup.6 is selected from —H; -halo; —NH.sub.2; —CN; —C.sub.1-3alkyl optionally substituted with 1, 2 or 3 substituents selected from —F and CF.sub.3; —C.sub.1-3alkoxy optionally substituted with 1, 2 or 3 substituents selected from —F and —CF.sub.3; —C(O)O—C.sub.1-3alkyl; —C(O)NR.sup.8R.sup.9; —C(O)OH; and —NHC(O)—C.sub.1-3 alkyl; R.sup.7 is selected from NR.sup.10R.sup.11; a 5 to 7 membered monocyclic heterocycloalkyl; and a 5 or 6 membered monocyclic heteroaryl; wherein the heterocycloalkyl and heteroaryl are optionally substituted with 1, 2 or 3 groups selected from —CN; —C.sub.1-6alkyl optionally substituted with 1, 2 or 3 substituents selected from —F, —CF.sub.3 and —OH; —C.sub.1-3alkoxy optionally substituted with 1, 2 or 3 substituents selected from —F and CF.sub.3; —C(O)OH; —C.sub.1-3alkylene-NHC(O)C.sub.1-6alkyl; —C.sub.1-3alkylene-NHC(O)OC.sub.1-6alkyl; C.sub.3-5cycloalkyl; or the heterocycloalkyl is optionally substituted with two substituents on the same ring carbon which together with the carbon atom to which they are attached form a 5 to 7 membered monocyclic heterocycloalkyl; and wherein when R.sup.7 is morpholinyl and R.sup.1 is unsubstituted phenyl, R.sup.2 is not —H; R.sup.8 and R.sup.9 are each independently selected from —H and —C.sub.1-6 alkyl; R.sup.10 is —C.sub.1-6alkyl; R.sup.11 is selected from —C.sub.1-6alkyl optionally substituted with 1 or 2 substituents selected from —F and —C.sub.1-3alkoxy; and —(CH.sub.2).sub.nR.sup.12; R.sup.12 is a 5 or 6 membered heteroaryl, a 3 to 5 membered cycloalkyl or a 3 to 6 membered heterocycloalkyl; m is 0 or 1; and n is 1, 2 or 3, to a subject diagnosed with dementia in an amount of from about 0.01 mg to about 1500 mg.
